

UNICEF SBCC Cells Programme Evaluation for 2017-21

For the states of Assam, Madhya Pradesh, Odisha, Telangana, West Bengal and Uttar Pradesh, India.

Final report

Date: March 4, 2023



Name of the Evaluation Project:	UNICEF SBCC (Social and Behaviour Change Communication) Cells Programme Evaluation for 6 states
Timeframe of the evaluation	2017 – 2021
Duration of the Evaluation Assignment	May 2022 – February 2023
Location of the evaluated object	National Scope with a focus on the following Indian States – Assam, Madhya Pradesh, Odisha, Telangana, Uttar Pradesh, and West Bengal
Names and/or organization(s) of the evaluator(s)	PricewaterhouseCoopers India (PwC)
Name of organization commissioning the evaluation	UNICEF India

To,

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Specialist, Social & Behavior Change,

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UNICEF India Country Office

4 March 2023

Subject: Submission of the Final Evaluation Report-third agreed upon deliverable for the assignment titled Evaluation of UNICEF's SBCC Cells Programme from 2017-2021

Dear Ms. Malhotra,

We sincerely thank you for giving us an opportunity to undertake the assignment on 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'.

As per the scope of the assignment outlined in the Terms of Reference (Contract number: 43347541), we are submitting Final Evaluation Report - third agreed upon deliverable .

We would be happy to provide any clarifications or additional information necessary to support this report. We can be reached on the mentioned contact details or on my email id, kaustabh.basu@pwc.com

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Yours sincerely,

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1. List of Acronyms

ADM	Additional District Magistrate
ANM	Auxiliary Nurse Midwife
ASHA	Accredited Social Health Activist
AWW	Anganwadi Worker
BCC	Behaviour Change Communication
BCM	Block Community Mobilizer
BEE	Block Education and Extension Officer
BPM	Block Programme Manager
BRIDGE	Boosting Routine Immunization Demand Generation
C4D	Communication for Development
CBO	Community Based Organization
CMO	Chief Medical Officer
CDMO	Chief District Medical Officer
CEO	Chief Executive Officer
CHAI	Clinton Health Access Initiative
CoE	Centre for Excellence
CSO	Civil Society Organisations
DCM	District Community Mobiliser
DHS(FW)	Directorate of Health Services (Family Welfare)
DIO	District Immunization Officer
DM	District Magistrate
DWCD	Department of Women and Child Development
FGD	Focus Group Discussion
FLW	Frontline worker
GAVI	Global Alliance for Vaccines and Immunization
GI	Group Interview
GoI	Government of India
HSS	Health Systems Strengthening
ICDS	Integrated Child Development Services
IEC	Information Education Communication
IPC	Inter-personal communication
MoHFW	Ministry of Health and Family Welfare
MOIC	Medical Officer in Charge
MPW	Multi-Purpose Health Worker
MT	Master Trainers
NGO	Non-Governmental Organization
NIRD&PR	National Institute for Rural Development and Panchayati Raj
NHM	National Health Mission
NRHM	National Rural Health Mission
PHC	Primary Health Centre
PHEO	Public Health Extension Officer
PwC	PricewaterhouseCoopers
PWD	Person with Disability
RI	Routine Immunisation
RMNCH+A	Reproductive, Maternal, New-born, Child, and Adolescent
SBCC	Social and Behaviour Change Communication
SDG	Sustainable Development Goals
SEPIO	State Extended-Programme-on-Immunization Officer (State EPI Officer or SIO)
SIHFW	State Institute for Health and Family Welfare
SRINO	State RI Nodal Officer
ToC	Theory of Change
TOR	Terms of Reference
UN	United Nations
UNEG	United Nations Evaluation Group

UNICEF	United Nations Children's Fund
UNICEF ICO	UNICEF India Country Office
WHO	World Health Organization

2. Acknowledgement

This report provides an independent, evidence-based evaluation of the SBCC Cells in six states in India (2017-2021). The UNICEF India Country Office (ICO) engaged PricewaterhouseCoopers India (PwC) to undertake the evaluation. As a part of this evaluation extensive consultations were undertaken to gather insights and inputs from a wide range of stakeholders, including government and implementing partners at the national, state and district level.

The evaluation team from PwC acknowledges the contribution and support from all those who contributed in the evaluation. We received continuous support and guidance from UNICEF ICO in all the stages of the evaluation. Their support was valuable for determining the evaluation design and in organizing interviews with the key personnel. Additionally, we received their support to access state specific information/ documents. We would like to express our gratitude and appreciation to the following individuals from UNICEF team Mr. Atishay Mathur, Research & Evaluation Specialist, Dr. Nidhi Pasi, Research & Evaluation Specialist, Mr. Siddharta Shreshta, Chief SBC, Ms. Alka Malhotra, SBC Specialist, Ms. Cheshta Gulati, SBC Consultant, and ERG members Catalina Salazar Silva, Dr. Vishal Singh, Associate Professor, Communication, SIHFW-Rajasthan, Dr. Kalas Baviskar, DDHS, IEC, Maharashtra for their extensive support. Their detailed comments on the inception report and draft evaluation report have helped to strengthen the evaluation report. Our special gratitude to all focal persons from the UNICEF field offices for facilitating the organization of the key informant interviews and focus group discussions for the evaluation. We express our profound gratitude to the members of the Evaluation Reference Group for providing the evaluation team with feedback and support throughout the evaluation period. Lastly, we express our gratitude to all participants in the evaluation for providing their valuable input and time for the interviews and the discussions. Their insights and suggestions have enriched the report.

The evaluation was undertaken by the consortium consisting of Sangeeta Tikyani (Team Lead), Sujata Mullick, Alivia Biswas, Soumi Kundu, Gitanjali, Srija Basu, Antara Chaudhary, Ankur Mittal, Rahul Varyani, Tamal Bhattacharjee, Chandan Kumar Singh, Johnny Edward and Amrita Gupta. Additional research was carried out by Nishigandha Joshi. Support team included PwC state teams who helped in translation and editing of the report. The overall evaluation direction, guidance and quality assurance was provided by Kaustabh Basu.

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3. Executive summary

Background

Information, Education, and Communication (IEC) is a process of adequately supporting information and education to people to advance healthy behaviors which are influenced by several factors such as the local context, family, and the community – factors that are crucial to an individual's growth¹.

The concept of Social and Behavior Change Communication (SBCC) which is an evidence-based, consultative process of addressing social attitudes, knowledge and practices has evolved from earlier models of IEC done through identifying, analyzing and providing communities and individuals with relevant information through well-defined strategies. SBCC aims to make changes in knowledge, attitudes, and practices and has emerged as a more comprehensive approach than IEC. While IEC involves use of wide range of channels and material to pass on fact based instructional messages to the individual or community, SBCC acknowledges underlying socio-economic factors that affect individual behavior and, uses robust approach to facilitate improved social behavior and mobilize targeted populations for change. Various countries and international organizations across the globe have begun investing in SBCC activities to empower individuals and communities, and lower structural barriers that hinder people from adopting positive practices and societies from becoming more equitable, inclusive, cohesive and peaceful.²

Over last two decades, India's growth has contributed phenomenally to global human development. However, despite making considerable progress over the years, it continues to face challenges with reproductive health, child health and nutrition.³ SBCC interventions have increasingly been seen as crucial contributor towards addressing social and cultural barriers in health programmes and enhancing positive behavior towards health and wellbeing of oneself and community, as a whole. In this light, UNICEF has been working actively along with Ministry of Health and Family Welfare (MoHFW) and state governments to strengthen the health system institutional structures for SBCC. Behavior change communication has been inculcated as a part of the larger Health Systems Strengthening program approach.

In 2013, a national behavior change communication (BCC) strategy for RI was developed with UNICEF's support. UNICEF also extended its support to GoI towards institutionalization of SBCC Cells and transformed communication focus from IEC to demand generation through behavioral change. In 2014, Global Alliance for Vaccines and Immunization (GAVI) provided support to this by revamping and revitalizing existing IEC/BCC institutions; or setting up new ones within the state institutions. The SBCC Cells were equipped with trained human resources and budgets that directly supported the State Immunization Division and played a significant role in planning and operationalization of communication interventions for flagship interventions such as Mission Indra Dhanush and new vaccine introductions. Under the GAVI HSS 2 UNICEF is mandated to engage and facilitate the process of SBCC Cells with the sustainability considerations as stated in the programme document, which outlines that the cells must be fully state government owned and operated.

Objective:

With the GAVI programme gradually drawing to a close, this UNICEF SBCC evaluation is intended to provide critical programmatic insights into what has worked, what has not worked and key learnings from the sample states included in the evaluation. Similarly, the findings of the evaluation are expected to support strengthening of SBCC cell functioning, and ongoing program initiatives aimed directly at maximizing utilization of public services. Based on the findings of the evaluation, UNICEF aims to advise the MoHFW and other public administrative bodies on the effectiveness and sustainability of institutionalizing SBCC Cells.

Methodology:

PricewaterhouseCoopers India (PwC) was engaged by the UNICEF India Country Office (ICO) to undertake this evaluation study. The geographic scope of the evaluation state specific (six states, namely, Odisha, Madhya Pradesh, West Bengal, Uttar Pradesh, Telangana, and Assam). A purposive sampling approach was used in district selection across sample states. Further, details on the rationale used for district selection has been included in the methodology section of the report (3.4.2). The SBCC Cells were assessed on the OECD DAC criteria to understand

² <https://www.unicef.org/social-and-behaviour-change>

³ <https://www.unicef.org/india/children-in-india>

the milestones reached by these cells in terms of relevance, effectiveness, efficiency, coherence, and sustainability considerations within the period of evaluation that is 2017-2021.

The evaluation followed the health systems strengthening approach which was also a part of the SBCC systems strengthening, and which included SBCC workforce, service delivery, financing, leadership, governance, information, and knowledge management. Gender and equity lens was adopted as cross-cutting themes across all evaluation objectives. The data collection for the evaluation was conducted during November 2022 to January 2023.

The evaluation was mainly conducted by the way of qualitative interviews and FGDs. Additionally, certain identified aspects were verified through observations such as availability of IEC material (Flyers, posters, media clips on social media), display of IEC material at the SBCC cell, and health centers etc. Similarly, aspects related to routine practices were assessed through observation and record reviews.

Key findings

The main findings are described for each evaluation criterion and structured around the key evaluation questions that guided the evaluation.

Relevance: The SBCC Cells aligned with the national and state priorities and other programmes, however the degree of alignment varied from model to model. The cells-initiated activities enhanced the reach and adherence of the programmes and interventions at grass root level. Even during the pandemic cells responded pro-actively and developed strategies, designed materials and planned activities to ensure right information is provided to people in efficient and effective manner.

Effectiveness: SBCC cells have been only partially successful in achieving their desired goals and objectives. Lack of systematic structure, scarcity of financial and human resources, limited scope of creativity negatively impacted the performance of the cells. Although the cells were established to lead the communication change interventions but majority of them ended up being supportive unit to the state's IEC activities.

Efficiency: SBCC cells across the states (except Telangana) did not have a say in allocation of financial resources for the SBCC activities. This limits the cells' efficiency in delivery due to lack of adequate funding for monitoring, research and evidence generation and other essential aspects of SBCC. Due to various reasons like vacant posts, retired / transferred personnel, lack of dedicated communication cadre etc. coupled with lack of dedicated financial allocations leads to reduced efficiency of cells to function within planned timelines.

Coherence: In most of the states, the cells had good coordination and convergence with state government. Cells are used to consistently provide government support for implementation of communication strategies at state and district level. However, intersectoral convergence was missing due to which cell were not able to achieve its goals and objectives in effective and efficient manner. It was also observed that cells failed to utilize the potential of other development partners and NGOs in creating a difference and initiating social behavioral change.

Sustainability: The SBCC cells work is well integrated into the programming of the departments they are housed under. However, they have a considerable scope of scaling up and integration in the public health system of the country and states. The government (at the state and district-level) have been reported to have limited capacity to adequately carry out the activities of the SBCC Cells independently at present due to limited technical knowledge / expertise and dedicated human resource and financial allocations. Collaborative efforts of all stakeholders would enhance the pool of resources and increase the reach of the cells and help in its transition.

Conclusion: The SBCC cells program was initiated to help state government departments to transition from IEC to SBCC. Though this transition is not yet complete for any of the six states evaluated in this study, the inception of the cells has got the ball rolling in the right direction. The state and district level officials are now familiar with the term 'SBCC' and are aware of its basic conceptual definition and how it differs from IEC. They are also aware of the novel methods being used in SBCC such as miking, interpersonal communication, leveraging key influencers, etc. The SBCC program has contributed to health systems strengthening in these six states by bringing about a change in the way health, ICDS and PRI officials approach and engage with community members.

In conclusion, it can be said that the SBCC cells program **kickstarted the transition of communications activities from IEC to SBCC**. The program's contribution has been significant and in many of the states been the first SBCC intervention initiated. Though the progress has been slow over the past programmatic cycle, it is important to understand that to bring about a change not just in methods but in the approach, understanding and attitude towards communications is a mammoth task. The very principles of SBCC can be applied here to see the slow progress not as a sign of inefficiency and ineffectiveness of the program, but rather as a **work in progress status for the cells**. The cells require guidance and support for at least one more programmatic cycle whilst they figure out their transition strategy to move from being UNICEF supported to completely government owned. **We strongly suggest UNICEF to continue its investment** and support in the program during this period.

Recommendations:

The recommendations have been formulated based on the findings which were analyzed through primary and secondary data. The recommendations are divided between those which are common to all the cells across all 3 categories and those which are specific to each category, and are formulated specific to the two main stakeholders, i.e., the government and UNICEF. Broadly, the recommendations are focused on addressing the need for establishment of district-level cells in all the six states, continued need of technical assistance and aid in handover process from UNICEF, capacity building and skill development of health officials at the state, district and block level, and the need for increased agency for cells to lead SBCC programming and have a greater say in budget and timeline planning. We also recommend, a similar evaluation to assess the progress of the transition of the state and decide future course of action at the end of the next programmatic cycle of five years.

Recommendation	Priority and Time frame	SBC SS Indicator	Responsible organization
State should develop a dedicated cell at state level for SBCC activities	High 6 – 12 months	Leadership and Governance	Department under which cell is housed in states
UNICEF to prioritize key areas for provision of technical expertise	High 6 – 12 months	Workforce and Service Delivery	UNICEF
Ensuring dedicated financial resources for SBCC cells	High 6 – 12 months	Finance	Department under which cell is housed and UNICEF
Adopt a systematic approach to human resource management	High 12 – 18 months	Workforce	Department under which cell is housed and UNICEF
Ensuring monitoring and evaluation of SBCC cells	High 6 – 12 months	Leadership & Governance, Workforce, Service Delivery, Information	Department under which cell is housed and UNICEF
SBCC interventions should be more explicit and strategically integrated to gender equity	High 6 – 12 months	Across all Indicators	State Government with UNICEF's support
UNICEF to develop SOPs for SBCC cells at state and district level	Medium 12 – 18 months	Service Delivery	UNICEF
Adopt a systematic approach for evidence based SBCC planning	Medium 12 – 24 months	Information	UNICEF

Development of SBCC cell repository	Medium 12 – 24 months	Information	Department under which cell is housed in states
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4. Introduction

India emerged as world's fifth largest economy by Nominal GDP and third largest by PPP in the year 2022⁴. Along with development of extensive tangible infrastructures there has been emphasis on change and progress at grass root level. Social building blocks and interventions in the form of Sarva Shiksha Abhiyan for increasing access to education, midday meal for nutritious food to children, Mission Indra Dhanush for Routine Immunization (RI), comprehensive primary care through Health and Wellness Centers (HWCs) and other measures for ensuring maternal and child health have been implemented to ensure a community where people are healthy (not merely illness free) and take onus of their health and wellbeing. In health sector, India has made substantial progress over the past decades. It is evident from the fact that, life expectancy at birth in India has grown from 45 years in 1960s to 70 currently.⁵

India is presently in a state of demographic and epidemiological transition. It bears triple burden of diseases namely, the infectious diseases; the noncommunicable diseases (NCDs) linked with lifestyle changes; and emerging epidemics and pandemics⁶. Despite making considerable progress over the years, it continues to face challenges with reproductive health, child health and nutrition⁷. Indicators, like, Maternal Mortality Ratio in India being just below 100⁸, Infant Mortality Rate being 28 and Under 5 Mortality rate being 32⁹ suggest that India still has a long way to go to achieve SDG targets¹⁰. It is essential to understand that better health and wellbeing of individuals cannot be established only through development of curative health services. Various interrelated social and behavioral determinants of health that contribute towards morbidities and mortalities need to be taken into consideration as they pose tougher challenges to the public health system. In fact, the epidemiological transition is fueled by these social and behavioral determinants of health in addition to other risk factors.

Nutrition is a crucial factor for health and development. Similarly, it is fundamental for improvement in infant, child and maternal health. It also leads to stronger immune systems and contributes to safer pregnancy and childbirth and lowers the risk of non-communicable diseases¹¹. Over the years, India continues to face a double burden of nutritional problem. It can be seen through NFHS-5 data that, there is sub-optimal progress in health and nutritional status in the population. In 2019–21, only 11% of breast-feeding children aged 6–23 months (about 2 years) receive an adequate diet, which was 8.3% in 2015-16 (NFHS 4). On the other hand, among the non-breastfeeding children the number has gone down to 12.7%, from 14.3% in 2015-16. When we consider children aged under 5 years, 35.5% are stunted (height-for-age), 32.1% are underweight (weight-for-age) and 19.3% are wasted (weight-for-height). Additionally, percentage of anemic children among children aged 6-59 months (about 5 years) has increased from 59% to 67% between 2015-16 to 2019-21. Malnutrition and anemia in young age sustains throughout the life span of individuals, especially women. Among the women aged between 15-19 years, 59% are anemic and among pregnant women aged 15-49 years 52.2% are anemic. On the other hand, obesity is also increasing its prevalence in India. According to NFHS – 5 data, 24% women and 23% of men in India are overweight or obese. Additionally, 57% women and 48% men have high risk waist-to-hip ratio¹². The double burden of high prevalence of undernourishment and increasing prevalence of obesity in a country reiterates the need for social and behavior change interventions.

The Sustainable Development Goals (SDGs), in particular, SDG 3 – Good health and well-being, and SDG 5 – Gender Equality cannot be achieved without bringing in behavioural change at all levels, combined with structural policy implementation. While practitioners of SBCC in India at state and district levels are tackling various challenges related to health (maternal, infant, and child mortality, etc.), gender biases and equity and participatory approaches of practitioners, SBCC representatives on field and within communities, they can engage more effectively for partnerships through greater resource utilization and wider impact of SBCC interventions. The SBCC program falls within the ambit of SDG 3, which focuses on the following goals:

- Reduce the mortality rate from noncommunicable diseases.

4 <https://www.imf.org/external/datamapper/NGDPD@WEO/IND>

5 <https://data.worldbank.org/indicator/SP.DYN.LE00.IN?locations=IN>

6 Narain JP. Public Health Challenges in India: Seizing the Opportunities. Indian J Community Med. 2016 Apr-Jun;41(2):85-8. doi:

10.4103/0970-0218.177507. PMID: 27051080; PMCID: PMC4799645.

7 Paul VK, Sachdev HS, Mavalankar D, Ramachandran P, Sankar MJ, Bhandari N, Sreenivas V, Sundararaman T, Govil D, Osrin D, Kirkwood B. Reproductive health, and child health and nutrition in India: meeting the challenge. Lancet. 2011 Jan 22;377(9762): 332-49. doi: 10.1016/S0140-6736(10)61492-4. Epub 2011 Jan 10. PMID: 21227494; PMCID: PMC3341742.

8 <https://pib.gov.in/PressReleaseIframePage.aspx?PRID=1879912>

9 <https://pib.gov.in/PressReleaseIframePage.aspx?PRID=1861710>

10 <https://www.un.org/sustainabledevelopment/health/>

11 https://www.who.int/health-topics/nutrition#tab=tab_3

12 http://rchiips.org/nfhs/NFHS-5_FCTS/India.pdf

- Reduce communicable diseases. Improve mental health and well-being and prevent substance abuse.
- Create and improve health systems and access to essential medicines and vaccines.
- Improve healthcare access for vulnerable populations, including women, children, and the elderly.
- Mitigate the impact of health emergencies such as pandemics and natural disasters.

The integration of SBCC programming in health programming directly addresses points three, four and five which are creating and improving health systems and access to essential medicines and vaccines, improving healthcare access for vulnerable populations, including women, children, and the elderly, and mitigating the impact of health emergencies such as pandemics and natural disasters. Further, through ensuring wider access to health messaging, the SBCC program also is working towards achievement of points one and two. Further, some important specific targets of SDG 3 include reducing maternal and child mortality, tackling infectious diseases, and ensuring universal access to sexual and reproductive health and rights which are also a primary focus of the SBCC program.

There has been well established iterative relationship between education and health. Literature shows a positive association between education attainment and health outcomes¹³. Education is therefore an important social determinant of health. Over the years, the country has significantly improved access to education. However, a substantial proportion of the population lacks formal education which negatively impacts their health and wellbeing seeking behavior. According to NFHS 5 (2019–21) data, in India, 72% (69% in 2015-16) of the female population, who are aged 6 years and above have ever attended school. This number reduces to 41% the indicator 10 or more years of schooling is considered. Similarly, it was seen that only 50% of men completed 10 or more years of schooling. Lack of proper education can lead to lower acceptability and adherence towards preventive interventions and promotive health practices. It is evident from the fact that, only 58% of mothers in India had at least 4 antenatal care visits and only 44% of women consumed iron folic acid for 100 days (about 3 and a half months). Such health-related behaviors from families continue post birth of child as well. Today, even after extensive implementation of RI interventions, close to one – fourth of children aged 12-23 months (about 2 years) are not fully vaccinated.¹⁴ UNESCO's Global Education Monitoring Report highlights that the attainment of higher levels of education among mothers is positively related to children's nutrition and vaccination rates, which contributes to decrease in preventable child deaths, maternal mortality¹⁵.

Along with education, lower demand of health and wellbeing services prevail due to certain supply and demand side barriers. People find visiting health facilities or Anganwadi for preventive and promotive health services difficult and time consuming. As preventive and promotive health measures do not provide immediate outcomes, underutilization of these services is observed. The statistics related to ante natal visits and vaccination provide evidence for the same. Additionally, even when services are brought at doorsteps communication barriers between service provider and beneficiary can curtail its acceptance. According to NFHS 5 data, only 24% of non-user females accepted that health workers had ever talked to them about family planning and its methods¹⁶. The challenges listed above are crucial determinants towards ensuring the health and wellbeing of individuals and society. On one hand, lack of education and awareness, distance from health facility, social beliefs and myths, attitude towards health workers, commutation and communication barriers negatively impact health seeking behavior of people leading to poor adherence and acceptability of preventive and promotive initiatives whereas on the other hand, malnutrition, lack of WASH facilities increase the susceptibility of the people towards numerous infections and ailments.

Although, across the country infrastructure is being built and updated (1.5 lakh HWCs operationalized till December 2022¹⁷) and health workforce have been deployed across the country to ensure consistent service delivery and innovative interventions implemented and over the years, governments at Union and state level have also invested financial and human resources for Information, Education and Communication (IEC) activities, but governments also need to focus on skill development and training of existing health workforce to enhance their inter-personal and communication skills. Additionally, public health system of India requires further recruitment of health workers for reduced workload of existing personnel and ensure better productivity. According to rural health statistics 2020 -21, out of the total requirements, 20.6% posts are vacant and there is shortfall of 30.8% of HW

13 Raghupathi, V., Raghupathi, W. The influence of education on health: an empirical assessment of OECD countries for the period 1995–2015. Arch Public Health 78, 20 (2020). <https://doi.org/10.1186/s13690-020-00402-5>

14 http://rchiips.org/nfhs/NFHS-5_FCTS/India.pdf

15 <https://en.unesco.org/themes/education-health-and-well-being#:~:text=A%20good%20quality%20education%20is,be%20well%20nourished%20and%20healthy>

16 http://rchiips.org/nfhs/NFHS-5_FCTS/India.pdf

17 <https://pib.gov.in/PressReleasePage.aspx?PRID=1887497>

(F)/ANM at PHC level¹⁸. The need of the hour is not only to undertake initiatives to deliver better quality services in an effective manner by resolving supply side constraints but also encourage people to access services by initiatives to tackle demand side constraints.

Effective and efficient communication pushes individuals and communities to adapt to progressive health and hygiene supportive behaviors. Under the Swachh Bharat Mission, inter – personal communication initiatives contributed significantly to IEC coverage under Swachh Bharat. The SBM was successful in delivering the message of safe sanitation practices to hundreds of millions of Indians through a unique ecosystem approach. This approach to catalyzing mission focused IEC activities led to a paradigm shift in how India approaches awareness and behavior change across sectors¹⁹. According to NFHS 5 data, in 2019-21, 70% of the population were living in households that use an improved sanitation facility and 77.3% of the women aged 15-24 years use hygienic methods of protection during their menstrual period. When compared with NFHS 4 data, the situation has certainly improved from 49% and 58% respectively. Enhancement of positive behavior towards cleanliness, sanitation may be attributed to effective communication by various stakeholders to ensure people initiate social and behavior changes related to health and hygiene as seen in Swachh Bharat campaign which has enhanced focus on SBC component²⁰. Similarly, there has been enhanced focus on SBC using a 360-degree approach to promote safe and hygienic menstrual health practices in recent years which might have resulted in improvement of said indicator over the years²¹.

In beginning of 2020, the world began its combat against COVID 19. Over the course of the three years, policymakers and health systems witnessed variations in infection and mortality rates regional, state, national and internal levels which pushed for dynamic and adaptive public health and social measures. The impact of COVID-19 has overwhelmed some of the most robust healthcare systems.²² This pandemic was much more than a health crisis. It was a communication, information, education, and behavioral crisis. It was identified that behaviors will drive this crisis, determine its trajectory and possess the power to mitigate the risk of the pandemic. Health systems across globe evolved and SBCC activities were initiated to curtail the impact of pandemic. The Indian government also considered Risk Communication and Community Engagement (RCCE) to be 'an integral part of a public health emergency response at every level' and had a detailed action plan for its use during the COVID 19 pandemic.²³ SBCC activities initiated by various stakeholders in India led to adoption of positive health and wellbeing practices which contributed towards effective and efficient management of the pandemic.

4.1. Background and context of intervention

Social and Behavior Change Communication (SBCC) becomes a center point of demand generation for health systems strengthening which in turn uses communication to change behaviors including service utilization and promote social change by positively influencing knowledge, attitudes, and social norms. UNICEF's engagement with the Ministry of Health and Family Welfare (MoHFW) and state governments aimed to strengthen the health system institutional structures for SBCC. In 2013, Global Alliance for Vaccines and Immunization (GAVI) committed a support of USD 107 million for the first phase of its Health Systems Strengthening program in India. This fund supported the government's intensified efforts focusing on overall health system improvements. Among others, the increase in demand for RI through a national behavior change communication (BCC) strategy was a priority area. This RI communication strategy for the Ministry of Health and Family Welfare was developed in 2013 with UNICEF's support. Behavior change communication was part of the larger HSS1 approach. UNICEF also extended its support to GoI towards institutionalization of SBCC Cells and transformed communication focus from IEC to demand generation through behavioral change. Incorporation of Social and Behavior Change Communication (SBCC) methods was envisaged to act as a fulcrum across stages of programming and planning, implementation, monitoring, evaluation, and evidence generation. In 2014, under GAVI HSS 1, effort was channeled towards revamping and revitalizing exiting IEC/BCC institutions; or setting up new ones within the state institutions if no set-ups for communication existed. By 2016, MoHFW had a comprehensive SBCC strategy in place to guide state and national programs on health and nutrition. Methods included in this strategy were IPC, community mobilization, mass media, and ICT.

18 https://main.mohfw.gov.in/sites/default/files/rhs20-21_1.pdf

19 https://jalshakti-ddws.gov.in/sites/default/files/201906_SBM_IEC_Whitepaper.pdf

20 https://jalshakti-ddws.gov.in/sites/default/files/201906_SBM_IEC_Whitepaper.pdf

21 <https://nhm.gov.in/index1.php?lang=1&level=3&sublinkid=1021&lid=391>

22 <https://www.euro.who.int/en/health-topics/health-emergencies/coronavirus-covid-19/news/news/2020/4/reconfiguring-health-systems-vital-to-tackling-covid-19>

23 https://www.nhp.gov.in/rcce_pg#:~:text=Risk%20communication%20and%20community%20engagement,in%20response%20to%20many%20questions.

The SBCC Cells were equipped with trained human resources and budgets that directly supported the State Immunization Division and played a significant role in planning and operationalization of communication interventions for flagship interventions such as Mission Indra Dhanush and new vaccine introductions. The intent of the SBCC Cell was to strengthen programme planning and implementation through evidence based SBCC focused approaches. It is important to note that few states of India such as Odisha, West Bengal and Madhya Pradesh already had institutionalized IEC/BCC/SBCC cells even before the allocation of GAVI HSS1 fund in 2014. With funding from GAVI HSS 1, and subsequently from HSS 2, the focus was to systematically integrate SBCC in programme conceptualization. SBCC was positioned as a catalyst to augment the results of RI programming at the state and district level. The states of Assam, Jharkhand, Madhya Pradesh, and Odisha were the fore runners in setting up and operationalizing the cells. Adopting a bottom-up approach, selected states such as West Bengal, Uttar Pradesh, Odisha, and Madhya Pradesh focused on institutionalizing the SBCC Cells at the district level that supported resource mobilization from different yet connected line departments. On the other hand, the SBCC cell set up in Andhra Pradesh serves the three states of Andhra Pradesh, Telangana and Karnataka and focuses on themes beyond RMNCH+A and RI such as RCCE and ODF. It is known as the Communication Resource Unit (CRU) in these states.

Overall, the model was flexible in meeting the need and context for the respective states and focused on building the capacity of officials such that (1) focus is shifted from IEC to BCC; (2) initiatives or communication campaigns for demand generation are evidence driven and scientifically designed to support measurability/ monitoring; (3) increased attention is laid on standardization of communication initiatives while being cognizant of the contextual norms; and (4) building evidence and sharing knowledge on the approach.

UNICEF followed two approaches of setting up SBCC Cells:

1. Strengthening the existing IEC Bureau within state machinery (e.g., Bihar, Chhattisgarh, Gujarat, Maharashtra, Uttar Pradesh, West Bengal, Assam, Jharkhand, Madhya Pradesh). This was done through the provision of extensive technical support in SBC knowledge and skills using several interventions and through the placement of skilled SBC professionals in the existing IEC unit structures.
2. Setting up SBCC Cells in state institutions that are best suited for setting up centers for communication e.g., Communication Resource Unit in Andhra Pradesh and Centre of Excellence in Odisha.

Most Cells were housed within the Directorate of Health Services; herein the key focus of the Cells was on health programmes including RI and MCH. SBCC Cells hosted by allied departments such as ICDS have focused on wider health or social determinants including WASH, child protection, child marriage, education, etc.

4.2. Understanding of the SBCC cells in the evaluation from secondary literature review

In India's SBCC models, communication has been instrumental in advancing health-related outcomes such as RMNCH+A, and Routine Immunization (RI) coverage, greater awareness of women and maternal child health, including nutrition, and vaccination, and WASH. States such as Assam, Odisha, and Madhya Pradesh have pioneered the establishment and operationalizing the cells. Whereas states like Uttar Pradesh, Odisha, West Bengal, as also Madhya Pradesh aimed at institutionalizing the SBCC Cells at the district level for greater mobilization of resources. States such as Telangana has mobilized efforts beyond the given needs of RMNCH+A and RI such as ODF and RCCE and established the Communication Resource Unit (CRU). In view of this, the organizational structure of the cell in each state differ to a large extent as elucidated below ^{*38}

The SBCC cell in Assam is housed under the health department and is institutionalized at the state level. The state has two SBCC cells at the state level both by National Health Mission (NHM) and the Directorate of Health Services, Family Welfare (DHFSW), with some common SBCC activities undertaken by both NHM and DHFSW such as polio, etc. Assam was one among the first states to have established a BCC cell in 2007 brought under NRHM for bringing together all health communications at one place. The second SBCC cell under the DHFSW focuses on communication related to Routine Immunization (RI) to address and eliminate the surrounding misconceptions that have so far constrained the state from attaining full immunization coverage. The nodal officers of the cell are DG DHS(FW), NHM. The communication strategies were revised in 2014 to address issues around demand generation for health service utilization to meet the NHM objectives using a more comprehensive

³⁸ Source: Presentations by state SBCC cells
SBCC Reports Case State study

approach to communication by utilizing all feasible modes of media to reach communities across all socio-economic profiles.

In Odisha, the State Institute of Health and Family Welfare has been a nodal organization for executing SBCC activities at the state and district level after its establishment in 1994. SIHFW was formulated because a gap was identified along the lines of - building capacities of managers at the state and the district level and frontline workers. To this end, a Centre of Excellence was set up inside the SIHFW. The Departments of Health and Family Welfare, Women and Child Development, Rural Development, OPEPA and OSACS were collaborators in this effort. It was envisaged that this would enhance technical understanding of the staff and would assist formulation of communication strategy and its effective implementation. The COE was further envisioned to design and develop quality material on behavior change communication, forge partnerships with recognized institutions to deliver capsule training on BCC, to provide research support on communication programs of the state and to provide technical support in procuring services related to communication material and management. The CoE has three major components - (a) a program management unit, (b) a material and design management unit and (c) a finance management unit. Integrated BCC cells are present in each of the 31 districts of Odisha.

Uttar Pradesh was among the first few states to establish state level SBCC cell, in 2014. The cell was introduced into the public health system of UP with the support from UNICEF and was designed to act as a state-level body that would guide the district communication cadre to undertake BCC activities. The nodal officer for the cell is CMOs. UNICEF works mainly in five priority districts – Balarampur, Mirzapur, Shravasti, Jaunpur, and Sonbhadra. Balarampur and Mirzapur are two districts where the SBCC cells have been set up and supported by UNICEF. Over the years, with the support of Technical Support Group for Behavior Change Communication (TSG-BCC) under the Health Partner's Forum (HPF) involving representatives from the GoUP, NHM, UNICEF, Save the Children, MAMTA, TSU, Plan India, BBC Media, and other development partners is responsible for developing a comprehensive strategy/plan and its implementation. In the past few years, the state has witnessed several efforts focused towards reducing the overall IMR, MMR and increasing Institutional Deliveries, Immunization, Early Breast Feeding, MR campaign, COVID-19 (prevention and vaccination) etc.

The state of Madhya Pradesh had functional IEC bureau under Department of Health and Family Welfare. The SBCC state plan was incorporated as a division in the state IEC cell. The SBCC Cell focuses on communications related to immunization, nutrition, and healthcare. Technical assistance for the same is given by the SBCC cell and implementation is undertaken by IEC. The nodal person for cell at state level are Director (IAS officer), Deputy Director (medical officer), Media Education and Information Officer (MEIO) and SBCC coordinator. The thematic areas of the cell include addressing the existing inequality in health-related challenges, women's nutrition, and vaccination using mediums like mass media, social media, print media, folk music using local artists have been used as tools of communication for successful IEC/SBCC campaigns. The SBCC Cells are also present at the district level and fairly actively engaging with the government, development partners and communities on behavior change related to nutrition, MCH, etc. District level SBCC cells are present in Guna and Shivpuri districts in Madhya Pradesh.

The SBCC cell in Telangana - Communication Resource Unit (CRU) was established by National Institute for Rural Development (NIRD) and Panchayat Raj (PR) in collaboration with UNICEF to address gaps in government departments' capacities to design and implement BCC strategies. Telangana has opted to establish only one dedicated SBCC cell at the state level and restrained from developing district level cells in the state. It aims to provide an institutional mechanism for strategic communication support to various government departments and national flagship programmes. It caters to strategic communication needs through consultations, technical support and capacity building. The nodal officers for CRU include Professor and Head and 2 SBCC Coordinators. Thematic areas of CRU are tribal development, women and child development, education, rural wash, Panchayati raj and rural development and health.

The West Bengal SBCC cell has been institutionalized under Department of Women and Child Development (DWCD). The nodal officers include secretary in consultation with MIC, District Magistrate, Additional District Magistrate, Deputy Magistrate, District Nodal Officer. Cell emphasizes on participatory planning for improving utilization of IEC/SBCC budgeting thematic areas of sanitation, Health, child marriage, nutrition, education and violence against children. The cell gathers evidences and focuses on increasing institutional delivery and increase RI in hard-to-reach areas, achieved ODF, increase school attendance, engaging faith leaders to prevent child marriage. SBCC cells are present in districts Purulia, 24 South Parganas, Murshidabad, Dakshin Dinajpur and Malda, where they are institutionalized within respective District Magistrate's office.

Challenges in SBCC Strategy Implementation

Social and Behavioural Change Communication (SBCC) is a key aspect of awareness generation and creating demand for the available health and welfare services. UNICEF is working actively along with state government towards building capacities of Frontline Health Workers (FHW) to deliver SBCC programs in rural areas by the needs of vulnerable communities. However, implementation of SBCC components have seen various challenges over the years.

SBCC programs require wide scale participation by stakeholders across different sectors like various government departments, development partners, civil society, local governance bodies, media and entertainment. However, it is seen that the engagement of all relevant stakeholders in the development of SBCC strategy has been non uniform and inadequate across states which acts as a hindrance to convergent action. Literature suggests that SBCC strategies can make desired behavior change when resources across various sectors are successfully leveraged³⁹.

A situation analysis of behavior change communication (BCC) and health promotion activities within the existing national health programs suggests that, reported scarcity of funds and human resources to support appropriate BCC and health promotion activities at the community and health center level. In addition, fund allocation and financial planning is still heavily focused on printing of IEC material⁴⁰. Similarly, a study highlights physical/material gaps, administrative issues and training requirements of the staff in addition to resource challenges⁴¹. Another suggests that availability of IEC materials at ground level was a challenge despite ample availability of the material due to local logistic issues⁴².

Additionally, there is a lack of structured reporting formats to capture the programme activities leading to challenges in documentation. Similarly, the monitoring and evaluation of the interventions was limited on account of lack of trained staff and resources to conduct and manage M&E⁴³.

Given this discourse, following are five critical factors considered during the evaluation:

- 1) Engagement of all relevant departments in the development of SBCC strategy
- 2) Level of awareness of SBCC strategy across relevant departments and officials
- 3) Resource procurement, management and utilization
- 4) Availability of funds, rationale of allocation and budget utilization

4.3. Objective and scope of the evaluation

The purpose of the evaluation is to understand the SBCC Cells' performance towards maximizing utilization of allocated resources and augmenting the results of the health systems strengthening initiatives and other cross sectoral themes of social and health determinants at the state and district level. The evaluation is intended to further provide critical programmatic insights into what has worked and what has not worked and why. The findings will further strengthen behavior change initiatives aimed directly at maximizing utilization of public services. This evaluation is intended to act as a guiding document for UNICEF to decide its future engagement and investment in the SBCC program.

The programme will be evaluated for the period 2017-2021. The cells will be evaluated on the five parameters of the HSS framework namely, leadership and governance, workforce, service delivery, finance, and information, against five OECD DAC evaluation criteria of relevance, efficiency, effectiveness, coherence, and sustainability. Gender and equity lens will be cross cutting in reviewing the Cells' performance and milestones on the aforementioned evaluation criteria.

The geographical scope of the evaluation is across six states namely, Madhya Pradesh, Uttar Pradesh, West Bengal, Assam, Telangana, and Odisha. The objective of the evaluation is to understand the 'why' and the 'how' of the success or failure of the initiative and provide evidence-based recommendations on making the functioning

³⁹ Dalberg WASH Institute. "An assessment of the reach and value of IEC activities under Swachh Bharat Mission (Grameen)." (2019).

⁴⁰ Thakur JS, Jaswal N, Grover A. Is Focus on Prevention Missing in National Health Programs? A Situation Analysis of IEC/BCC/Health Promotion Activities in a District Setting of Punjab and Haryana. *Indian J Community Med.* 2017 Jan-Mar;42(1):30-36. doi: 10.4103/0970-0218.199795. PMID: 28331251; PMCID: PMC5349000

⁴¹ Ugargol, Allen Prabhaker, A Study of Gaps, Weaknesses and Opportunities in the IEC Services of the Directorate of Health and Family Welfare Services in a Southern State in India (May 3, 2012). Available at SSRN: <https://ssrn.com/abstract=2079166>

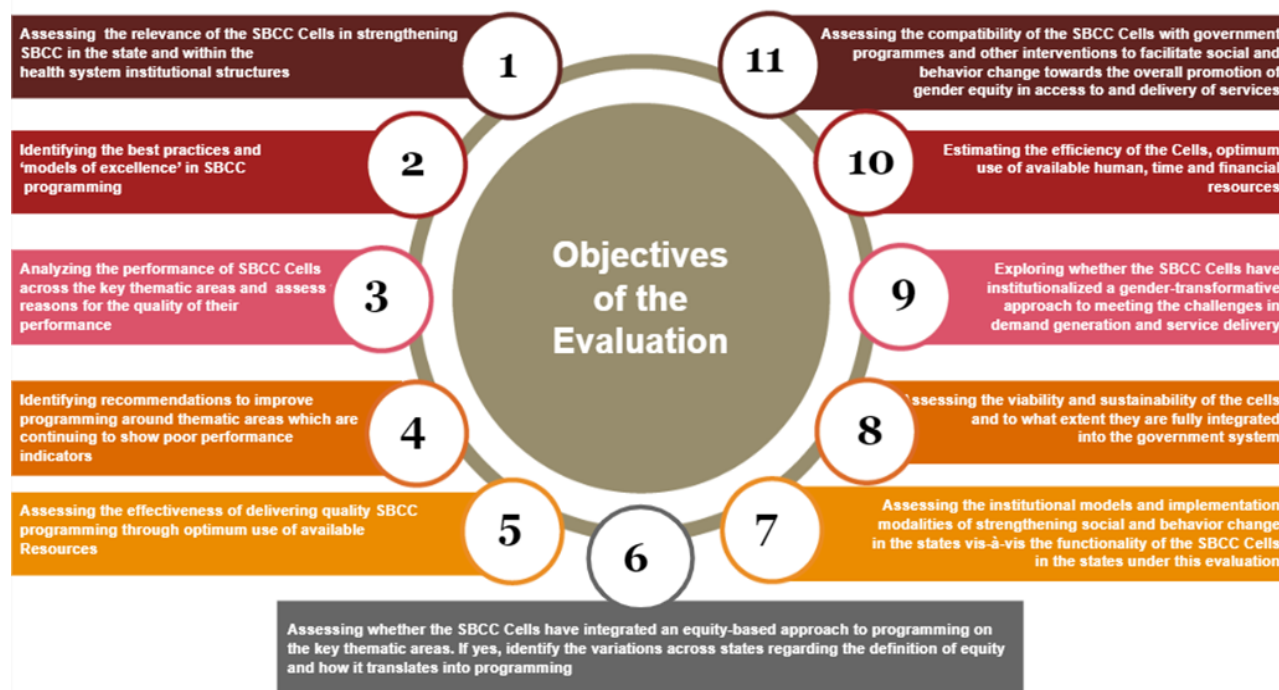
⁴² Pattanshetty, Sanjay M. "SWOT analysis of IEC implementation in RMNCH+ A program in Udupi district." *International Journal of Community Medicine and Public Health* 5.5 (2018): 2109.

⁴³ Thakur JS, Jaswal N, Grover A. Is Focus on Prevention Missing in National Health Programs? A Situation Analysis of IEC/BCC/Health Promotion Activities in a District Setting of Punjab and Haryana. *Indian J Community Med.* 2017 Jan-Mar;42(1):30-36. doi: 10.4103/0970-0218.199795. PMID: 28331251; PMCID: PMC5349000.

of SBCC Cells more effective and efficient. Additionally, the evaluation would investigate how the Cells adapted and supported the respective state governments in their COVID response. Figure 1 Below provides an illustrative representation of the objectives.

The present chapter laid the foundation on highlighting the context for the assignment and included the rationale, scope, and objectives of the same. The following chapter would elaborate the approach and methodology to be adopted for the overall assignment, of which a detailed layout of few of the segments from the chapter may be referred in the annexures.

Figure 1: Objectives of the evaluation



5. Methodology

Evaluation criteria and questions

For this evaluation, we adapted the Organization for Economic Co-operation, and Development Assistance Committee (OECD-DAC) evaluation criteria of relevance, effectiveness, efficiency, coherence, and sustainability and contextualized it to use as a guide for the evaluation. These criteria provided a normative framework which served as basis for evaluation. We developed key evaluation questions (KEQs) to further elaborate each evaluation criteria. Further, a consultative and iterative process was adopted with the ICO and state teams, to re-sequence, club and recategorize the KEQs (Table 1)

Figure 2: Evaluation Criteria

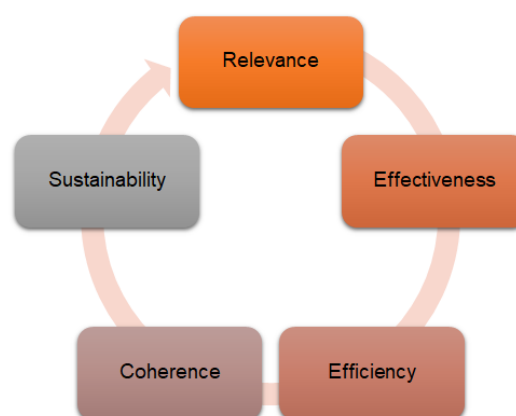


Table 1: Categorization of key evaluation questions as per evaluation criteria

Evaluation Criteria	Key Evaluation Questions (KEQs)
Relevance	- How relevant and appropriate are the SBCC Cells to the flagship programs, and priorities of the national and state governments? - How relevant, appropriate, and aligned is the design of the SBCC Cell to the State's established systems and institutions of service delivery? - To what extent does the SBCC Cell respond to the needs of community members and community representatives (FLWs, trainees etc.) - How adaptable is the SBCC Cell to evolving concerns/areas of focus/evidence for realignment (including COVID)
Effectiveness	- To what extent has setting up of SBCC Cells strengthened government systems in bringing about more effective and evidence-based communication initiatives? - To what extent is evidence-based programming effective in identifying and addressing the challenges to demand generation for RI, health, nutrition, and WASH services? - To what extent have the training equipped stakeholders to plan, implement, monitor behavioral change, and demand generation initiatives? - How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives? - To what extent have the SBCC Cells improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates? - Have the SBCC Cells successfully fulfilled their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships? - How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system?
Efficiency	- Did the SBCC Cells efficiently deliver/implement their activities and function within the planned/allocated monetary resources? - Did the SBCC Cells efficiently deliver/implement their activities and function within the planned/allocated human resources and timelines? - Did the SBCC Cells sufficiently deliver/implement their activities and function within the planned/allocated timelines?

Evaluation Criteria	Key Evaluation Questions (KEQs)
Sustainability	- To what extent have the different state departments integrated SBCC programming into its planning and implementation of initiatives? - Does the government (at the state and district-level) have the capacity to successfully carry out the activities of the SBCC Cells independently? - Do the Cells and their implementation structures (at the state and district-level) have adequate financial allocations and partnerships to successfully carry out the activities of the SBCC Cells independently? - What are some of the recommendations to further strengthen the functioning of SBCC Cells to improve health systems delivery at the state/district/block/village levels?
Coherence	- How do the activities/interventions of the SBCC Cells interlink and synergize with the existing/other programme/initiatives by other relevant actors (NGOs and development partners and other sector players)? - How do the SBCC Cells interventions align/synergize/fit with other interventions being carried out by the state for improving health systems?

5.1. Conceptual framework: Theory of change

Based on document review, consultation with UNICEF ICO and UNICEF SBC specialists across the states under focus for this study, a retrospective Theory of Change (ToC) has been developed for the SBCC Cells. The activities of the Cell are categorised under the five pillars of Health System Strengthening including: (1) Leadership and governance; (2) Workforce; (3) Service Delivery; (4) Finance; and (5) Information. The overall intent of the SBCC Cell emerged to provide support for improvement of demand generation and accelerate uptake of government schemes that can positively impact a child's wellbeing through enhanced and equitable access to government sponsored services/schemes for health, nutrition, education, water, sanitation, and protection. Accordingly, the outcomes for this model are: (1) Sustainable government systems created for SBCC programming; (2) Improved service delivery* and outreach of government programmes; (3) Improved uptake and beneficiary participation towards government sponsored programmes/schemes; and (4) Positive policy environment created for SBCC programming across sectoral development initiatives.

To attain this intent, the **pathways of change** are designed as following:

- If the leadership is sensitised on the need for behavioural change, then increased number of programmes will adopt SBCC at the design and result stage
- If SBCC Cell is institutionalised under the government set-up then SBCC-based programming will be more sustainable, accountable, and transparent
- If SBCC relevant content is integrated in programme planning and capacity development, then beneficiary identification, targeting, coverage and outreach will improve
- If IPC capability of functionaries is strengthened, then their convincing power and quality of service delivery will improve
- If the desired behaviour is established, then communities will actively participate in monitoring service delivery and thereby demand for improved service quality

Overarching assumptions: Availability of a nodal govt department/institutional set up to host the SBCC Cell; adequate resource mobilization by nodal department/institution in supporting competent HR and financial budgets; availability of a network of community mobilizers to support awareness and demand for services.

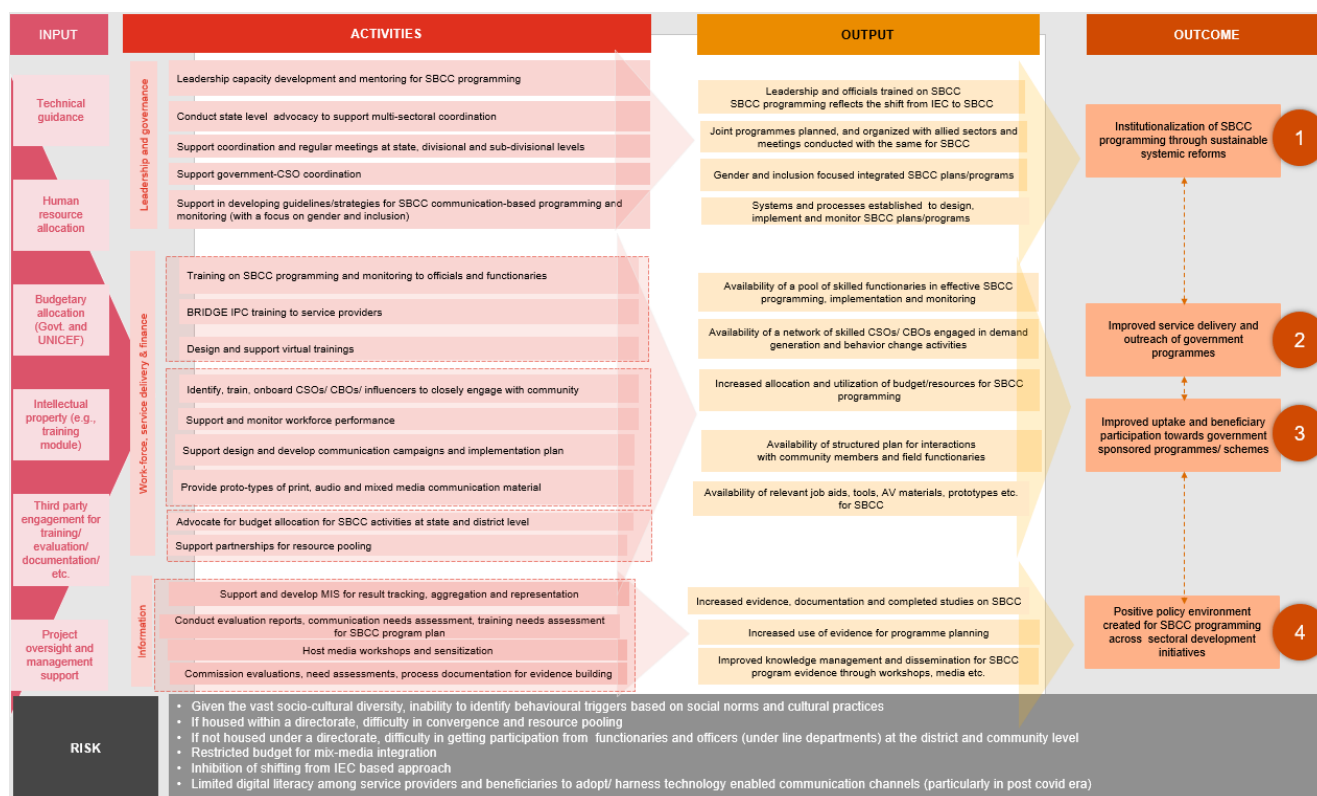
Risks:

- Given the vast socio-cultural diversity, inability to identify behavioural triggers based on social norms and cultural practices
- If housed within a Directorate, difficulty in convergence and resource pooling
- If not housed under a Directorate, difficulty in getting participation from functionaries and officers (under other line departments) at the district and community level
- Restricted budget for mix-media integration
- Inhibition of shifting from IEC based approach

- Limited digital literacy amongst service providers and community representatives to adopt/harness technology enabled communication channels (particularly in post COVID era)

In collating the information, it was noted that the SBCC Cells under review have distinctive characteristics, which although similar, are not comparable. According to the nature of institution and the thematic area of focus, the states were clubbed under three categories (1) SBCC Cells housed under the health department and based at the state level – Assam; (2) SBCC Cells housed under the health department, based at state and district level - Odisha, Madhya Pradesh, Uttar Pradesh; and (3) SBCC Cells housed outside the Health Department: West Bengal (with district level cells), Telangana (without district level cells). Accordingly, the team had developed (1) an overarching ToC capturing the broad intent and core areas of focus or change; and (2) three supporting ToCs as per the categorization of the states. This section provides an overarching ToC which will be finalized in the next phase (data collection) in detailed consultation with the states and ICO. Figure 2 below highlights the contents of the overarching ToC developed.

Figure 2: Overarching Theory of Change



5.2. Evaluation design

The evaluation focused on key building blocks in the structure, design, and operation of SBCC Cells. The core themes included nature of workforce, mechanism of service delivery, financing, leadership and design and execution of governance and information and knowledge management. Along with the OECD DAC evaluation criteria, the evaluation also included the domain of gender and social equity. In the detailed evaluation framework (refer to Annexure 1), we have mapped indicators with the KEQs as per all the overall evaluation criteria.

5.3. Sampling design

i. Selection of states

The assessment was planned across **six states** in India across the country's regions: Assam, West Bengal, Odisha, Jharkhand, Telangana and Madhya Pradesh. The states were selected purposively considering UNICEF's categorization of high burden states (Madhya Pradesh), tribal (Odisha, North-East), good governance (Telangana). Geographical variance in the eastern region (North-East (Assam), East (Odisha, West Bengal), Central (Madhya Pradesh and Jharkhand) and South (Telangana) was ensured, providing a more representative picture of the program. However, Jharkhand, a sample state for the evaluation, was later replaced with Uttar Pradesh. This change was made considering that the SBCC Cell in Jharkhand has been handed over to the government. prior to the evaluation period. Hence, the evaluation of this cell would not enable assessment of UNICEF's role in supporting the cell during the evaluation period.

Evaluation in the states of Odisha, Madhya Pradesh, West Bengal, and Uttar Pradesh was done at two levels as there are two levels of SBCC Cells, at the state and district level. In the remaining two states of, Assam and Telangana, the SBCC Cells only function at the state level, hence the evaluation was carried out at the state level.

ii. Selection of districts/blocks

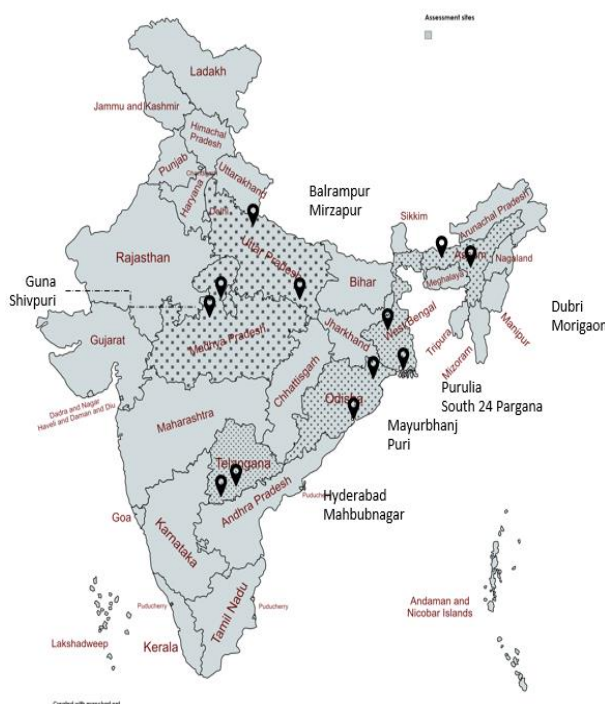
We understand that marginalized communities including scheduled castes, scheduled tribes, religious minorities, and persons with disabilities have been historically excluded and may be under-represented/covered/reported across development programmes intervention focus and results. While there have been efforts towards taking into consideration the need for greater intervention focus, and integration of such stakeholders within active planning and implementation of SBCC and related programming, there are further opportunities noted in this regard.⁴⁵ In this context, a stepwise methodology was used to ensure diversity of population in the selection of districts in the sample states.

Step 1: Within diversity of population, district level values of percentage of scheduled caste population, percentage of scheduled tribe population, percentage of population of religious minorities and total district level population of Persons with Disability was mapped across the sample states. The Census 2011⁴⁶ data was considered for the above-mentioned indicators.

Step 2: Two districts per sample state were selected based on values mapped across indicators mentioned above, elaborating the overall diversity of population⁴⁷. One district with a lower than state average value across the selected indicators and another with a higher than state average value across the selected indicators per state was selected. Further, distance from state capital was also considered in the process of district selection.

Table 2: Presentation of indicators mapped for selection of sample districts

Figure 3: Representative image of evaluation sites



⁴⁵ UNICEF Gender Results Report, 2020

⁴⁶ <https://censusindia.gov.in/census.website/>, Accessed on September 06th 2022

⁴⁷ It is to be noted that other indicators were considered to support mapping the proposed district selection across sample states. The indicators include availability of year of establishment of district level cells and year of implementation of first activity by SBCC Cell. However, there was limited availability of relevant information on these indicators across the secondary evidence for all sample states. In consultation with UNICEF, it was agreed to propose diversity of population as a potential indicator for mapping the district selection.

States	Districts	Distance from the state capital	Total District Population	% Scheduled Tribe	% Scheduled Caste	% Religious Minorities	Total population of Persons with Disability
Assam	Dhubri	243 km	1949258	0.3%	3.6%	80%	32718
	Morigaon	68.9 km	957,423	14.29 %	12.31	52%	17336
Madhya Pradesh	Shivpuri	284 km	1726050	13.2%	18.6%	4.35%	32943
	Guna	181km	1241519	15.4%	15.6%	5.46%	19257
Odisha	Puri	61 km	1,698,730	0.4%	19.1%	3.01%	53667
	Mayurbhanj	317 km	2,519,738	58.7%	7.3%	15.93%	68387
West Bengal	South 24 Parganas	62 km	8,161,961	1.2%	30.2%	36.53%	227129
	Puruliya	250 km	121,067	18.5%	19.4%	18.73%	61788
Uttar Pradesh	Mirzapur	286 km	2496970	0.8%	26.5%	8.03%	41840
	Balrampur	151 km	2,148,665	1.2%	12.9%	37.8%	30500
Telangana	Hyderabad	Proposed district same is the state capital	6,731,790	1.2%	6.3%	33.51%	177909
	Mahbubabad	215 km	4,053,028	9%	17.5%	8.82%	107782
Colour Indexes used							
			District with overall lower representation of values against indicators mapped				
			District with overall higher representation of values against indicators mapped				
			District closer to state capital				
			District farther from state capital				

Table 3: Evaluation sites

Region	State	Districts
North – East	Assam	Dubri, Morigaon*
Central	Madhya Pradesh	Shivpuri, Guna
East	Odisha	Puri, Mayurbhanj
East	West Bengal	South 24 Parganas, Purulia
North	Uttar Pradesh	Mirzapur, Balarampur
South	Telangana	Mahbubnagar, Hyderabad

Note: West Karbi, Anglong was one of the proposed evaluation districts in Assam initially. However, we had to change the district to Morigaon due to conflict in the district. The district was chosen as it was next in the list as per the criteria followed for selection of study sites.

We followed the steps given below to arrive at sample size for the evaluation

Step 1- Stakeholder mapping: We undertook stakeholder mapping across national, state, district, sub -district, and community level. This exercise's intent was to systematically capture the variant group of stakeholders engaged in supporting and implementing the activities of the Cells to be included in the evaluation. These stakeholders were categorized and clubbed under 16 sub -headings across various levels. Mapping was done vis-à-vis document review and in discussion with the respective UNICEF state /SBC officers and specialists.

Step 2- Selection of sample size in consultation with the respective UNICEF state /SBC officers and specialists. For this evaluation, respondents were selected purposively considering the area of enquiry.

The table below provides an overview of the stakeholder categorization and listing. It indicates the representative stakeholders who hold their respective positions across the sample states. It must be noted that these positions were not uniform across the states and variations were noted across the range of stakeholders engaged. With reference to the overall categorization of stakeholders, state specific stakeholder included in the study have been provided in the annexure.

Table 4: Distribution of stakeholder interviewed by type

Level	#	Stakeholder Category	Type of stakeholders
National	1.	UNICEF India Country Office	Chief SBC, SBC Specialist
State	2.	Nodal Leadership, SBCC Cell	Director IEC Bureau/ DG Directorate of Medical and Health Services/etc.
	3.	Functionaries of the SBCC Cell	RMNCH+A Officers/ Media officers/ Deputy Media officer/ MEIO/ General Manager IEC (NHM PMU)/ etc.
			State communication officer/ State Planning and monitoring officers/ Content development officers/ State IEC Officer
	4.	UNICEF Consultant (loaned staff)	Technical Consultant
	5.	Leadership, Nodal government department	Mission Director NHM/ Director General/ Director Dept of HFS/ Director ICDS/ etc.
	6.	Collaborating government official	Govt. officials of the nodal govt department including, SEPIO/ IO
	7.	Allied department' officials	Director or Officers from WCD/ Culture/ PRI/ RD/ SC& ST dept.
	8.	UNICEF state officials	SBC Specialist
	9.	Government officials responsible for the setting up of SBCC Cells (if different from current officials)	Ex-officials of Nodal government department/ SBCC Cell head
	10.	UNICEF officials responsible for the setting up of SBCC Cells (if different from current officials)	Ex-UNICEF state SBC Specialists/ Officers who were in the state when the Cell was established
	11.	Development agencies with which the SBCC Cell has collaborated	International Development Agencies/ allied UN agencies
National/ state level NGOs/ CSO/ Medical college			
District	12.	Head of District SBCC Cell/ district SBCC set-up	District Magistrate/ CDMO/ CEO Zila Panchayat
2 districts to be covered per state	13.	Officials supporting communication planning at district level	DHEIO/ DPHCO/ DMC/ DMEIO/ DIO/ Key Resource Person (KRP) from collaboration departments
	14.	SBC Cadre at district level (Master Trainers)	(District level representatives who have been engaged in SBCC related training and dissemination since last 6 months) ⁴⁸
Block	15.	SBCC Cadre at sub-district level	(BCM/ BEE/ BPHEO)
		(Master Trainers)	(Representatives who have been engaged in SBCC related training and dissemination since last 6 months)
	16.	Trainees of SBCC cells	FLWs (ANM/ASHA/AWWs/ ASHA Sahyogini/ CSO/ CBO
	17.	Community Members	Parents and caregivers

Table 5: Sample size covered per state

⁴⁸ The relevant MT designation for district level were discussed with UNICEF state SBC officials while the evaluators undertake the coordination related discussions in planning for state consultations as part of the data collection phase.

#	Name of the state	State level		Divisional Level		District level		Block Level		Total interactions	
		No of interactions in IR	No of interactions completed	No of interactions in IR	No of interactions completed	No of interactions in IR	No of interactions completed	No of interactions in IR	No of interaction completed	Total interactions mentioned in IR	Total interactions completed on field
1	Uttar Pradesh	7	4	2	1	3	4	8	6	20	15
2	Madhya Pradesh	14	11	NA	NA	16	9	5	7	26	26
3	Telangana	9	7	NA	NA	1	1	5	5	15	13
4	West Bengal	5	3	NA	NA	12	9	5	7	22	19
5	Assam	15	7	NA	NA	5	4	5	8	25	19
6	Odisha	14	19	NA	NA	7	6	8	8	30	33
7	National level	NA	NA	NA	NA	NA	NA	NA	NA	01	01
Total Interactions proposed in IR and completed on field										139	126

5.4. Data collection methods

The data was collected through qualitative interview guides/ FGD guide. Pilot testing of both the qualitative and FGD guides was conducted to assess the feasibility of data collection tools in Mandar district of Jharkhand in September- October 2022. After the successful completion of the pilot testing, a team consultation was arranged, and the tools were revised based on the observations from the pilot. The revisions included incorporating new areas of inquiry which emerged during the discussion, and feedback received on the questions framed during the pilot testing.

The tools were administered by a trained research team, oriented, and trained on the study methodology and technical components of interview/ FGD guides. The study team who conducted the interviews were public health professionals with prior experience of conducting such studies. On field training of the team for conducting the interviews was also carried out. Safety protocols were followed during the data collection to counter the spread of COVID-19 and protect all the individuals associated.

To ensure the quality of collected data, certain identified aspects were verified through observations such as availability of IEC material (Flyers, posters, media clips on social media), display of IEC material at the SBCC cell, and health centers etc. And those related to routine practices were assessed through observation and record reviews. Similarly, the team corroborated aspects on capacity building by SBCC cells by observation of training like ToT for master resource person and other capacity building sessions.

The questionnaire was administered in Hindi/ English based on respondents' preference. FGD guide was administered in local languages taking the help of translators. Translators were provided with an orientation on the study methodology and FGD guide beforehand. The team of researchers was trained to administer the questionnaire in the respective languages. The qualitative interviews were audio-recorded after getting the oral consent of the respondents. In case the consent for audio recordings was not obtained, manual notes were taken for the same. On average, the interviews took 30 minutes each. The interview was stopped when information saturation was reached i.e., no new information was emerging from the interaction. Data collection in each state took five days of time, by a team of 2-3 researchers. Data was collected during November 2022 to January 2023.

5.5. Analytical approaches

Desk review and secondary data analysis

A large volume and a comprehensive number of documents were reviewed in this evaluation including the program documents shared by the state SBCC cells including state case studies, power point presentations and communication material and reports publications by UNICEF including C4D progress reports. Detailed list of documents is provided in the appendix A.3.

The secondary data was subjected to content analysis to get insights on aspects such as structure of SBCC cells, its functional and thematic areas and key challenges in functioning.

Qualitative interviews and focused group discussions

The evaluation team considered the design of the programme, its organic evolution, its' state-specific context, variations, and core areas of focus while developing the data analysis approach.

Given that this study is primarily qualitative in nature, the data was analysed as follows:

Interviews and Focus groups discussion: Coding of the transcriptions was conducted using reflexive thematic analysis. Themes were mapped to various components of the Organization for Economic Co-operation, and Development Assistance Committee (OECD-DAC) evaluation criteria of relevance, effectiveness, efficiency, coherence, and sustainability. Coding framework was shared with the larger study team to form the consensus, and accordingly finalized. The study findings have used illustrative quotations in English where required. Coding and analysis of qualitative data was carried out using Microsoft Excel.

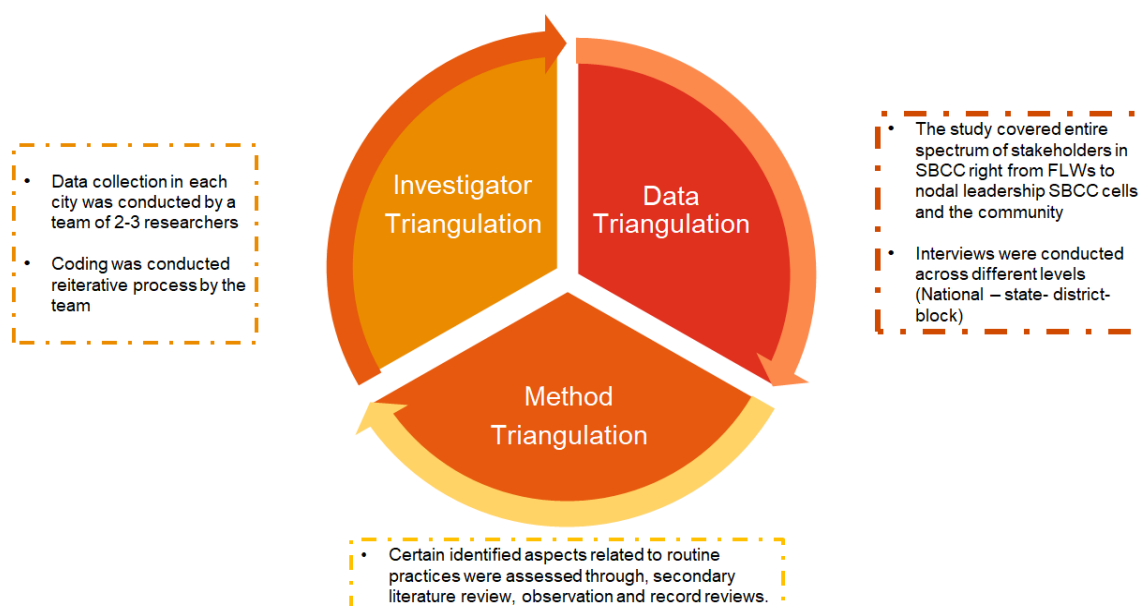
Field level Observation: The data for this is drawn from the inputs gathered across the parameters noted in the observation checklist. This information was added under relevant sections of state wise findings

Steps followed in analysis of qualitative data-

Step 1: Transcription of the recorded interviews/discussions into English were done manually. Particular care was taken during the transcription so that there is no loss of information during the process. To ensure quality of transcriptions, researchers and supervisors undertook random checks by listening to audio recordings.

Step 2: Development of coding framework and coding of qualitative data: Coding framework was developed as per the KEQs and SEQs to categorize responses using the information areas covered in the discussion guides. However, this list was augmented based on other codes that emerged from the transcripts during the actual coding process. After carefully reading the transcriptions, information from transcriptions was put into the relevant code

Triangulation of qualitative interview data and other secondary information was conducted to synthesize the evaluation findings and draw conclusions.



5.6. Limitations of the study

- The use of quantitative and numerical data points pertaining to health, behavioral and social indicators is limited within the scope of this evaluation. A link of causality and/or attributability cannot be formed between said datapoints and findings of the study. The scope of the study is limited to understanding contribution of SBCC cell in health system strengthening.
- We could not cover the target sample as we could not conduct few stakeholder consultations across state, district and block levels owing to unavailability of relevant stakeholders or prior commitments to other engagements or unresponsiveness to communications shared requesting for consultations. However, the response rate for the study was approximately 90 percent, which is acceptable in social science research.
- There was limited awareness of SBCC related activities among both trainers, trainees of SBCC and community members across a few sites. Further, the recently transferred recruitments had limited institutional memory on the evolution of the SBCC Cell in the state. Limited relevant insights were received on the nature of work done and challenges faced in doing so and evolution of SBCC cell through the interactions conducted with the above stakeholder groups.
- There is a lack of any prior in-depth, comprehensive baseline studies that could offer insights into all dimensions of SBCC programming, that could further be used or referred to during the evaluation. Hence, it was observed that the gaps in covering and strengthening SBCC programmes in certain areas in each state was largely attributed to the absence of on-ground research and profiling based on demographic and socio-economic status of the populations in the respective settlements.
- There was lack of institutional memory within the government related to SBCC cell including relevant documentation which might have resulted in recall bias. The evaluation team tried to include appropriate probes in the questionnaire to aid respondents' recall of activities. The team obtained relevant documents from the state SBCC team to fulfil the objectives of the evaluation.
- As there is no state level SBCC Cell in Uttar Pradesh, the stakeholders mapped at the state level were from different IEC cells across line departments with limited technical context and awareness of SBCC related activities. As a result, the input received from such consultations highlighted limited relevant insights on the same.
- In Assam, West Karbi, Anglong was one of the proposed evaluation districts initially. However, we had to change the district to Morigaon due to conflict in the district.
- The good practices that we have documented in the report have been compiled by information received from the respondents and we have not validated the same through field observations.

5.7. Ethics and UNEG Standards

Data collection team comprised of PwC staff including senior consultant and manager. All members of the team completed a foundational ethics training specifically UNICEF's course 'Introduction to Ethics in Evidence Generation' on Agora before conducting the evaluation.

The study team abided by 2021 UNICEF procedure on ethics and Evaluation and UNEG norms and standards for Evaluation. The team involved in primary data collection were oriented on protocols for ensuring compliance to ethics, gender sensitivity and equity principles in data collection and management. The data collection methods employed for the evaluation did not include any interaction with children or adolescents.

All stakeholders and evaluation participants identified for consultations and data collection, were duly informed on the study, its intent, how the input received is proposed to be used and limits of confidentiality. Mandatory oral consent was obtained from all the respondents before the interview / discussion, and any queries, concerns, or questions were addressed prior to the interview / discussion.

All respondents were informed about the voluntary nature of participation in the study and if they wish to withdraw after the consent, then they are free to do so. Each project team member engaged in the ensured adherence to non-disclosure of any data received or accessed in course of the evaluation.

The study team undertook measures to maintain the confidentiality of the respondents. During the process of synthesizing of findings personal identifiers were removed from the database and unique identification codes (UID) were used while analyzing the data. All interview data including audio files of recordings and transcriptions

were handled as per the research protocols and the project manager ensured that access to data was restricted and shared on a need-to-know and use basis.

Protocols for interactions and data collection including on-field conduct, particularly with women, and other vulnerable groups, were defined. Adherence to "do no harm" principle, respect for dignity and diversity, compliance with codes was ensured for the same. Further, team was trained and periodically oriented on the protocols. To maintain the quality of evaluation we ensured following points.

- Clarity of roles and reporting lines were well defined. Each person had specific responsibilities and clear job descriptions. Their outputs and deliverables directly contributed to the project's overall impact.
- Each team member had clear objectives at the start of each project year in discussion with the project leadership team. There was a protocol to update the TL (Team Leader) and project manager daily on field interactions and proceedings. The performance of the core team was assessed on a monthly basis. Training and mentoring were ensured if any member faced challenge in delivering quality results.
- The TL and the subject matter experts were engaged at state level interactions. Spot quality control checks were in place to address grievance if any during the evaluation on field.

6. Findings

The SBCC Cell Evaluation covered six states namely, Uttar Pradesh, Madhya Pradesh, Assam, West Bengal, Odisha, and Telangana. The findings have been triangulated from various sources of data such as secondary literature review, UNICEF state team presentations and primary data collection across all six states.

Each state's Cell structure can be categorised broadly as those which are housed within the health department, those which are outside of the purview of the health department, those which are with state level institutional setup and those with the state as well as district level institutional setup.

Category	State
Category 1: SBCC cells housed under the health dept. and based at the state level	Assam
Category 2: SBCC cells housed under the health department, based at state and district level	Uttar Pradesh, Odisha and Madhya Pradesh
Category 3: SBCC cells housed outside the Health Department	West Bengal (with district cells) and Telangana (no district cells)

The findings are presented across these categories and have been mapped against the key evaluation questions across the five OECD-DAC criteria of Relevance, Effectiveness, Efficiency, Sustainability and Coherence. The evaluation includes an overall cross-cutting theme of gender and equity embedded within the adopted methodology and are presented in the findings accordingly. To establish a holistic understanding about the evaluation criteria, each of these have been elaborated below:

6.1. Relevance

Relevance as an evaluation parameter aims to analyse the extent to which the intervention objectives and design respond to beneficiaries, global, country, and partner/institution needs, policies, and priorities, and continue to do so if circumstances change. To evaluate the level of relevance for the SBCC program, state level officials, UNICEF consultants (loaned staff) and district level officials of SBCC cells were asked to comment upon the program's alignment to national and state level flagship programs, usage of established systems of delivery and the cells' response to community needs. Similarly, to assess the adaptability of SBCC cells to COVID 19 response by assessing modified approach to SBCC activities during the pandemic.

KEQ 1: How relevant and appropriate are the SBCC Cells to the flagship programs, and priorities of the national and state governments?

Category 1

There is a relevance of the SBCC Cells to the flagship programs particularly as they are under the National Health Mission (NHM) and with the Directorate of Health Services, Family Welfare (DHSFW). The integrated SBCC activities are aimed at addressing health topics and behaviors for improved health and development outcomes. Mission Indradhanush has been a major action-oriented programme of the Cell to strengthen immunization coverage in priority districts which have less than 50 per cent unvaccinated or partially vaccinated children. Mission Tejaswee was aimed at reducing maternal, neonatal and child mortality. Over the years, special immunization drives have targeted children and expectant mothers to increase the immunization coverage to 90 per cent. Although Assam made considerable improvements in Routine Immunization, currently, the Cell's focus is on urban immunization, as it was found that immunization coverage is lower in urban areas as compared to rural areas. The relevance of RI has helped to have wider reach of demand generation for RI activities. Largely, the Call to Action for Child Survival and Development initiative by the Govt. of India was concurrent with the Reproductive, Maternal, New-born Child Health +Adolescent (RMNCH+A) Strategy for addressing child survival and reduction of Infant Mortality Rate (IMR), and Maternal Mortality Ratio (MMR). SBCC communication comprised mass media spots, open book wall painting, folk media, community theatre, rallies by school children, dissemination of IPC (ICDS and health service providers, and short videos and ASHA radio programs on

Immunization which were some of the key strategies. During Covid-19, the importance of hygiene and sanitation were reiterated through key messages on handwashing, social distancing, etc. District task force meetings were organized for SBCC coordinators across all 33 districts of Assam. The BCC activities have been under the Child Health or Maternal Health are included in their respective sections. The SBCC cell under NHM receive financial support through the State's PIP, as the NHM PIP is the main source of funding. IEC-related planning has been the aggregate of the budget of IEC under the umbrella of the NHM at state level.

Category 2

SBCC cells in category two are housed under the health department and hence work well within the ambit of flagship programmes of this department. The cells act as a facilitator of communication activities for various health interventions and programs. For example, in UP the cell contributes to programs such as Mission Indra Dhanush, Dastak Sanchari and Measles and rubella awareness; in Odisha the cell contributes to programs for routine immunization and RMNCHA+; and in Madhya Pradesh the cell has been able to contribute to programs such as Beti Bachao Beti Padhao. The cells aid in developing strategic plans and materials for integrating SBCC programming in flagship health programs of the state and national level. For example, in UP to increase vaccine coverage under Mission Indra Dhanush, the cell was able to recognise the importance of interpersonal communication (IPC) to increase a trust building process between the community members and frontline health workers. The cell was then able to train the frontline health workers in the skills of IPC which aided the said workers in spreading awareness about the importance of vaccination and reduce vaccine hesitancy.

Further, since the cells monetary allocation comes from the respective flagship programs, their capacity to function and develop strategic plans is restricted to the aspects of said programs.

Category 3

In the case of category 3, the cells are housed outside of the health department under NIRDPR (CRU) and WCD (West Bengal cell). Hence, the cells' functioning does not align with core health flagship programs such as Mission Indra Dhanush; rather the cells' functioning aligns with supporting health programs related to nutrition, WASH and sanitation. For example, CRU in Telangana has contributed to flagship programs such as SBM, JJM and Poshan Abhiyan by developing communication strategies which focus on integrating SBCC into the planning and programming stages of these programs, providing training to master trainers who are able to further disseminate their learnings at the field level and contextualization of SBCC and IEC material by translating to local languages and changing of social examples and imagery to those which are locally relevant.

For the cell in West Bengal, the expected area of focus is ICDS for Nutrition and the four largest schemes viz., Kanyasree, Rupasree, Lakshmi Bhandar and Social Welfare schemes such as old age and widow pension. The cell contributes to these programs by developing differentiated district wise communications plans based on community needs. The expected role of the cell is that of a convergence agency connecting the dots required for communications around the focus areas of WCD such as nutrition.

"For CRU, WASH is one of its main mandates. In Telangana, all districts have IEC consultants under Swachh Bharat Mission. CRU conducts capacity development and training programs for these IEC consultants"

- Study Participant (Telangana)

KEQ 2: How relevant, appropriate and aligned is the design of the SBCC Cell to the State's established systems and institutions of service delivery?

Category 1

The Health Communication in Assam is implemented by the District Extension Media Officer (DEMO), (now the state Cold Chain officer under NHM). The other members include DHSFW at the district level, Block Community Mobilizer (BCM), NHM and Health Educator (HE)/Block Extension Educator, DHSFW at the blocks. ASHA and ASHA Supervisors are the main frontline cadre involved in informing communities on available health services, disseminating health messages and assisting in utilization of health services. In addition to immunization, service delivery at the state level have comprised various innovations towards reduction of mortality, cleft-lip, and distribution of iron tablets in school, etc such as through the Rural Health Practitioners (RHP). Furthermore, the state is looking at launching the Boat Ambulance and Clinic concept (a good practice from Bangladesh) as was shared by an SBCC officer participant, the Mobile Medical Unit (MMU), ASHA Radio Program, Sarathi-104 (Health Information Helpline & Complaint Redressal system), etc. However, on ground it was found that accessibility of these in remote and hard-to-reach areas such as the *Chars* – areas or lands which are surrounded by water,

were difficult, and still a major concern. The gaps in Reproductive Maternal Newborn Child and Adolescent Health (RMNCH+A), have been addressed and reduced although services and knowledge and awareness related to more fundamental issues of child/early marriage, teen pregnancy, and school dropout, need to be addressed and tackled for integration of social behavior change, and health concerns related to adolescents and women. This is particularly true of the two districts of Dhubri and Morigaon in the evaluation study where it was found that the aforementioned practices are highly prevalent in LODOR groups, largely in the economically vulnerable and hard-to-reach areas. These two districts are also the ones which have seen the least intersectoral convergence among the 33 districts during Covid-19. The cells in this category occasionally converge fundamentally in RI work, polio and vaccination, although collaborations with other departments are not yet strengthened, specifically to tackle issues of early marriage and adolescent SRH which need more attention and convergence.

Category 2

The cells in category two are highly aligned to the states' established systems and institutions of service delivery. Since the cells are housed under the health department, they are able to leverage and make use of established systems such as the differentiated cadres of frontline health workers. Further, the cells do not have a dedicated communications cadre and hence comprise of members who are government employees in the positions of health officials, hence leveraging the state's established human resource cadre within the health system. Even the cells' activities are planned in accordance with the established programming and timelines of the health department. Hence, the cells' functioning is not only aligned but can be said to be dependent on the states' established systems and institutions of service delivery.

Further, the cells in this category have positive convergence and collaborations with other relevant departments such as their respective DWCD. This allows for even greater access to the states' established systems and institutions of service delivery. For example, the cells in this category are able to leverage the Anganwadi system too under the DWCD, in addition to the established systems of the health department.

Category 3

The cells in category three have access to the state's established systems and institutions of service delivery of the respective department they are housed under. For eg, CRU has access to leverage the PRI cadre as it is housed under NIRDPR and the cell in West Bengal has access to the Angwadi system as it is house under DWCD. Further, the cells do not have a dedicated communications cadre and hence comprise of members who are government employees in the positions of ICDS and PRI officials, hence leveraging the state's established human resource cadre within the state's established systems.

For the cell in West Bengal, even the cells' activities are planned in accordance with the established programming and timelines of the DWCD.

CRU has an added advantage of being housed under a national institute which gives it access to the institutes' established systems and institutions of service delivery (SIRDs) at the state level too.

KEQ 3: To what extent does the SBCC Cell respond to the needs of the community members and community representatives?

Category 1

One of the major activities of the Cell's engagement in responding to the needs of communities and representatives has been in the RMNCH+A coverage, and to increase awareness amongst the mothers and communities about the need of ANC and post-partum health. Although, the key focus and approach has been on the (six) High Priority Districts, there is an overall gap in, and the need to cover all vulnerable and hard-to-reach areas across the state's 33 districts. It was noted that communication gaps exist in tea garden areas of indigenous populations, and char areas which do not yet have a flow of transport facilities. It was also found that there is a limited capacity of FLHWs to tackle issues of ineffectual communication. Most FLWS in the workforce is currently aging and are close to retirement, while there is concurrently, a shortage of FLHWs due to a lack of recruitment. The FLHWs need continued and advanced training in improving their interpersonal skills to engage better with communities. A direct impact of this scenario has led to the impact of demand for service utilization. There is a lack of detailed, lucid explanation of locally relevant SBCC material particularly for indigenous populations speaking dialects. Thus, there is a need to create a communication strategy with appropriate and relevant messages for specific populations in specific areas. During Covid-19, the SBCC officers have themselves travelled to sensitive districts for addressing vaccine hesitancy of communities, by self-demonstrating the criticality of vaccine on themselves. Further, SBCC Cells have reached out to religious leaders and other Faith Based Organizations through an orientation, such that the importance of covid-19 vaccination, and immunization,

and early marriage prevention can be addressed. However, not much has been addressed as wider SBCC activities need to be undertaken with coordinated efforts at the district and block levels which need intervention of SBCC Cells' officers. Although there is one SBCC coordinator in each district, there is a need for capacity building. At present, IEC materials are the usual brochures and pamphlets which the FLWs and community influencers use as guiding sources for information. The overarching health issues related to mothers and expectant mothers in the last few years have increased mainly with attention to vaccine, RI, measles, etc. However, system strengthening through sufficient budgeting, M&E etc for state-level plans for RI across 33 districts in the state are necessary. For example, IMR/MMR, child marriage, etc are social concerns that are not yet addressed with any preventive measures. Reportedly, there is a case of child (under-age) marriage at 32 percent in Assam. Departments can play a role in better integration and convergence, but there is a limitation and challenge on account of no converging powers. Jingles, banners are not relevant and sufficient as awareness needs to be generated through actual on-groundwork. FLWs need advanced training to be able to be more communicable with communities. Conclusively, the targeted health-related activities and communication have been carried out within the usual and prescribed tasks of the SBCC programmes, but crucial and urgent social concerns of women and adolescent SRH, teen pregnancy, safeguarding rights of girls (early marriage), and school dropout need urgent responsive and preventive measures need to be set to be able to address community needs and generate awareness.

"The cell's major focus is on routine immunization. We try to tackle vaccine hesitancy through methods such as developing differential strategies based on needs of the people in the districts."

- Study Participant (Assam)

Category 2

Since the cells function not only at the state level but also at a district level, they are in regular communication with community members and representatives such as frontline health workers. They are able to provide regular guidance and oversight of dissemination of health messaging at the community level. Though there is a major limitation in the cells ability to respond to the needs of community members and representatives due to resource and budget constraints. It was found that the cells' approach is top down as they function only on the mandate of the state government and have no flexibility to include findings from formal and informal needs assessments done at the community level. Since the cells are provided with monetary allocation through the budget for each respective program, they are given detailed instructions on how much to spend and are restricted to spend the fund only on items given on a state mandated list of IEC and SBCC materials.

"We work for all national health programs like RCH...Health system strengthening, and all is covered in integrated BCC cells. Focus is on RCH, immunization, NCD, school health etc."

- Study Participant (Odisha)

Category 3

CRU does not have a mandate to work at the community or implementation level. Hence, it does not directly contribute to responding to the needs of community members and representatives. Though it works closely with allied departments like WCD which have grassroot level presence and utilizes secondary data to analyse and understand the needs of community members while designing the SBCC programme for various programs under different departments.

Since the cells in West Bengal functions at a district level, they are in regular communication with community members and representatives such as frontline health workers. They are able to provide regular guidance and oversight of dissemination of health messaging at the community level. Further, the cells are able to contribute to responding to the needs of the community members by developing customised strategies for communications focused on local contexts and the needs of community members and representatives.

KEQ 4: How adaptable is the SBCC Cell to evolving concerns/areas of focus/evidence for realignment (including COVID)

Category 1

During Covid-19, the SBCC cells were active in rolling out vaccines, and most people in the cells were directly or indirectly involved in vaccine roll out programmes. As part of the Covid-19 communication activities, state cells covered populations under the 33 districts, wherein district coordinators visited sessions which were conducted, and gathered information through mobile phones. The district coordinators covered and conducted these interactive sessions to address logistical issues of vaccine accessibility of diverse populations groups. It was noted that the state was the first towards attaining effective vaccine management (EVM). However, as was found that at the state and district level, there was no direct Covid-management of medical activities of doctors; rather it was largely focused on administrative tasks. In terms of adaptability, post-Covid, social media has not had a substantial role, even though it is a pressing need with comprehensive awareness communication material for MR issues, and additionally, there is no information generation or awareness/coverage for RI through social media. Furthermore, no training has been conducted in the last 2 years for ASHA/Aanganwadi workers, while No BRIDGE training has been conducted post virtual training that was conducted during Covid. The lack of training of FLHWs has been a barrier in improving and advancing issues on communication and attitudes related to health both from a micro-level and macro level perspectives.

Category 2

As the cells are housed under the health department, and with COVID-19 being a public health issue, the cells were able to contribute to the COVID response for their respective states. The cells were able to conduct online training for frontline health workers when social distancing was recommended during the pandemic. Further, the cells were able to train frontline health workers to use novel means of communication such as telephone and WhatsApp to deliver health messaging to community members while maintaining physical distance.

The cells were also able to act as a repository for online IEC and SBCC material which was sent out to the frontline health workers and health officials to be spread amongst community members. The cells have been able to adapt to other evolving concerns too such as being introduced to an increased focus on ODF practices.

Category 3

The cells in this category reported to have flexibility to be able to adapt to evolving concerns and areas of focus. For example, CRU was able to contribute to designing and providing technical assistance in developing Covid-19 response not just for the states it serves (namely Andhra Pradesh, Telangana and Karnataka) but also for other states such as Sikkim. Largely, at the state level, the cells are able to adapt national level IEC material into regionally contextual material includes translation into local languages and changing of imagery. For West Bengal, at the district level, citizen-centric steps are being taken to have a wider reach and needs of the target beneficiaries. The districts level cells work on issues of institutional delivery, child marriage, adolescent health, malnutrition, menstrual hygiene etc. The cells have been very active during COVID 19 and mobilization during COVID vaccination. The district level cells were able to act as a convergence point for BMOHs and frontline health workers during the pandemic. SBCC cell aligns with the needs arising from time to time.

Conclusion

The cells in all three categories are gradually working towards the pathways of change indicated for the first and fourth longer term outcomes* in the theory of change namely, institutionalization of SBCC programming through sustainable systemic reforms, and positive policy environment created for SBCC programming across sectoral development initiatives. The outputs pertaining to relevance as an evaluation indicator are leadership and officials being trained on SBCC programming reflects the shift from IEC to SBCC and joint programmes being planned and organised with allied sectors and meetings conducted with the same for SBCC. The cells were found to be taking active initiative to train leadership and officials, and in developing joint efforts with other development actors. Although considerable progress is yet to be made.

(*Outcomes two and three, namely, improved service delivery and outreach of government programmes, and improved uptake and beneficiary participation towards government sponsored programmes/schemes are beyond the scope of this evaluation.)



My Village, My Home campaign



The Maternal and Child Health Integrated Program (MCHIP) adopted a community-level tool-based programme called My Village, My Home. The objective of this tool is to strengthen service delivery and RI coverage for communities, assess the high drop-out rates and to set up a system of tracking beneficiaries in districts, and to improve the quality of coverage. FLWs use this visual depiction of the immunization status of infants and make use of the community to monitor the immunization coverage of all infants in the locality. The tool is designed in a way that the information on the oldest infant in the community of the respective village is on the bottom row, while that of the younger infants are in the rows upward. The dates of vaccination are written or highlighted in colours in the respective boxes against the names. The advantage of the tool is that it has increased intersectoral collaboration and is a community-friendly tool that can be accessed easily by parents of children, and the ASHA, Anganwadi and ANM workers.

6.2. Effectiveness

Effectiveness as an evaluation parameter, helps in analysing the extent to which an initiative or intervention has been able to achieve its goals and objectives. It highlights the intervention's success in terms of producing the desired effects. It helps in understanding, whether the intervention has structural or implementational limitations. In the course of SBCC cell's evaluation, effectiveness was measured through, their role as institutions for evidence generation, strategic planning, monitoring, training, documentation and analysis, knowledge sharing, cell's contribution in health system strengthening and effectiveness of strategy, material and content developed by the cell.

We gauged the effectiveness of SBCC with respect to above mentioned categories by interviewing state and district level officials of SBCC cells and corroborated the same through interactions with master trainers, FLW, and community members and observation of training and workshops across the field.

KEQ 1: To what extent has setting up of SBCC Cells strengthened government systems in bringing about more effective and evidence-based communication initiatives

Category 1

The transition from IEC to SBCC although formally occurred between 2014-2015, in practice, the Cells are largely continuing as IEC bureau. Although, an evidence-based communication strategy was designed with support from UNICEF in Assam, there is a lack of evidence generation in terms of regular, updated documentation, particularly in the High Priority aspirational districts, detailed mapping of hard-to-reach areas, Monitoring and Evaluation (M&E) at district and block levels, and prioritizing IPC and evidence-building and documentation in tribal belts (hills), Char areas, and tea estates. While the state has worked effectively in evidence based RMNCH+A and RI strategies, there is a potential to further address LODOR areas through innovative communication material and strategies, and through in-depth studies for evidence generation. There was no reported participatory studies or strategy pertaining to M&E plans. The Special Immunization Drive (SID) for immunization coverage of dropout and left-out children was conducted using household display of color-coded stickers (Red, Yellow and Green) based on Immunization status were pasted on houses. The system of stickers ensured easy identification of houses for immunization coverage using community monitoring. Additionally, the HBNC Voucher System worked well and may be considered an evidence-based communication as it helped mothers and FLHWs to be updated on home visits which were documentary evidence. However, there was no mention or observation of the HBNC Voucher system on field. The SBCC Cell has received good support from UNICEF for work on RI, Training and Communication activities. The Har Ghar Dastak campaign has been an effective programme that reached out to LODOR areas through awareness generation, mobilization, and vaccination activities. To a great extent, it can be observed that the SBCC Cell has strengthened the government system specifically in the health-related development, i.e., Immunization and such, but needs to do more to collaborate with government's efforts on behavioral change at the ground.

Category 2

The cells in category two have not been able to focus on evidence generation to develop evidence-based communication initiatives or planning. Though, it is important to note that the national and state programs that the cells contribute to are planned and programmed with an evidence-based focus and are able to identify vulnerable sections of the community which require higher efforts to be equitably integrated in the service delivery process.

The members of the cells are government health officials who do not have the technical expertise to conduct evidence generation studies and engage in data collection. Further the funding for these cells comes through program budgets which only include enough funds for conducting communications activities such as printing of posters for the said program, and does not take into account other aspects of SBCC such as evidence generation, monitoring, etc.

Category 3

The cells participation in primary research is limited. Though within CRU, there was evidence of use of existing secondary data sources (from sources such as NFHS and DHS surveys) for evidence. Further, both cells were able to leverage data collected by WCD's through its interventions and use it as source of secondary literature. The cells leverage secondary data and evidence generated by line departments such as the WCD who are engaged in implementation activities on field to identify challenges related to demand generation of WASH, child protection and nutrition schemes and services. In this manner, the cells have been able to contribute to developing differentiated communications strategies based on evidence from secondary sources.

"CRU has not yet taken up this role of leading opportunities of providing primary evidence generation activities yet. Our way of working is reflective of the government and UNICEF's focus on product rather than process and hence evidence generation is not one of our mandates"

- Study Participant (Telangana)



Research oriented forward looking goals



CRU's (Telangana) main forward-looking goal is to establish itself as a research focused institution. It wishes to focus on primary data collection and drawing in depth behavioral analyses from the data collected. CRU is housed in NIRDPR and is under the leadership of the head of Center of Gender Studies, TISS Hyderabad. Under such a leadership, the cell staff is academically oriented and has a clear understanding of research methodologies. For example, compared to the other state cells, staff of CRU had a distinctly better understanding of the process of evidence generation and the type of resources required to conduct it.

KEQ 2: To what extent is evidence-based programming effective in identifying and addressing the challenges to demand generation for RI, health, nutrition, and WASH services?

Category 1

At this point, the effectiveness of evidence-based programming in RI, health, nutrition, and WASH services cannot be entirely ascertained for the cell as they have not engaged in evidence-generation activities (as was observed on ground during the evaluation).

Category 2

The effectiveness of evidence-based programming and identifying and addressing challenges in routine immunization, health, nutrition, and wash practices cannot be ascertained for the cells in this category as they

have not been able to engage in evidence generation activities and develop evidence-based programming. (As observed during the evaluation)

Category 3

The effectiveness of evidence-based programming and identifying and addressing challenges in routine immunization, health, nutrition, and wash practices cannot be fully ascertained for the cells in this category as they have not been able to engage in primary data collection to generate evidence and develop evidence-based programming. Though through the cells' success in being able to identify and address the challenges in nutrition and WASH services using secondary sources of evidence suggest a positive effect of evidence-based programming on these processes.

It is important to note that since the cells in this category are not housed under the health department, they have not contributed to developing evidence-based programming for identifying and addressing the challenges in RI and health services. (As observed during the evaluation)

KEQ 3: To what extent have the training equipped stakeholders to plan, implement, monitor behavioral change, and demand generation initiatives?

Category 1

It was gathered from the evaluation, that there are gaps at two levels in the capacity building of the Cells. One is, that there is a lack of vision to design SBCC interventions in terms of innovative design, programmes, and their implementation, and continued newer ways to contextualize and strategize local health communication interventions. Second, that a shortage of (suitably skilled) staff/Cell officers with competencies on account of a lack of organizational capacity within the Cell has hindered the possibilities of conducting and strengthening the capacity building and improve the quality and effectiveness of SBCC work. Vacant positions at key government positions at the State and district level SBCC cells have only stalled capacity strengthening. The Cell's existing members are only able to execute the tasks assigned and possible within their capacity and purview of their duties on a regular basis. However, capacity building for SBCC officers and FLHWs to increase their skills and knowledge through periodic trainings is lacking at this point. Even though FLHWs have received training on communication intermittently, there is a gap in scaling up their attitudes, knowledge, awareness, and skills to keep pace with the frequent and evolving health schemes and programmes. As it was found, currently, there are no BRIDGE training and people (including trainers) are not familiar with TARANG. FLHWs are not adequately skilled in conducting mid-media events and IPC, or even interact with communities on deeper and wider social and gender and equity concerns. These would require them to gain skill sets and there are gaps in varying degrees across the FLHWs workforce. As was found on the field, processes of feedback sharing, monitoring, and remedying measures to scale up by the Cells are missing. Therefore, as was observed, the Cells are not yet strengthened in providing refresher's training.

"More capacity building at district and block level needed. NGO's teams can be utilized at district level. Monetary incentive for community influencers can motivate them to bring in change at the community level"

- Study Participant (Assam)

Category 2:

The cells have been able to carry out training and capacity building to equip stakeholders to plan and implement behavioural change initiatives. Though the effectiveness of the training differs for each of the cells. For example, in Odisha the training included an interactive workshop to develop the capacity of the frontline health workers in methods of SBCC, whilst the training in UP merely was a presentation of the district's performance across health indicators and was delivered in a lecture mode.

Since the cells in this category are housed under the health department, they have easy access to a large cadre of trainees (frontline health workers) who can implement techniques of behaviour change at the field level. Training on monitoring has not happened as the cells in this category have not engaged with monitoring activities.

Category 3:

CRU conducts training for district level functionaries such as the IEC consultants within SBM on SBCC and demand generation initiatives. The effectiveness of the training translating into demand generation could not be commented on in absence of supporting research.

"It is difficult to draw correlation between our work and the impact at the implementation level. We do not conduct any scientific assessments or monitoring and evaluation activities to be able to make such claims"

- Study Participant (Telangana)

In West Bengal the SBCC cell staff have conducted training on SBCC principles such as how an individual can change their behavior while creating domino effects in their immediate environment like family, community, school etc. Similar training has been done for frontline workers like ANM, AWW, ASHA, PRI members, SHG groups and have been equipped to deliver key messages.

KEQ 4 : How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives?

Category 1

Most of the health communication strategies and materials for the state are guided/ developed/managed by the IEC/BCC unit related to thematic BCC activities, including a range of communication channels, and designing materials and programmes for vulnerable populations. Key activities carried out by the IEC division are drawn from thematic campaigns of NRHM, including message designing, and material development, and distribution of audio/video and print material/software. In addition to communication, IEC/BCC unit is also responsible for training on BCC/IPC (inter-personal communication). IEC/BCC unit undertakes several activities in the ongoing year as determined in the Annual Action Plans. State Institute of Health and Family Welfare (SIHFW) is entrusted to train in all other subjects/themes except BCC/IPC in the state. The SBCC material must be published in multiple languages considering the cultural and ethnic diversity. Mission Tejaswee on the prevention and control of anemia is an instance of effective communication strategy included awareness generation, advocacy, distribution of IFA tablets to expectant mothers and lactating mothers by ANM. It was found that despite such efforts, there is a lack of funding that is much needed for creating transformative SBCC communication material in minority areas. and LODOR. Conclusively, district level officers and trainers demand communication material for LODOR groups, it was observed that both lack of vision to have wider reach in LODOR combined with insufficient funds has prevented the Cells from disseminating effective communication materials. However, there was no documented evidence demonstrated on lack of fund, during the evaluation.

"Jingles, banners are not relevant and sufficient as awareness needs to be generated through actual on-groundwork"

- Study Participant (M P)

Category 2

The cells themselves do not develop a lot of communication material or content. They rely on the material provided by the health department which includes materials such as posters and banners. Sometimes material and content developed by other development agencies such as UNICEF, CHAI and UNDP are also used as part of dissemination of health messaging. For example, the 'RI wheel' developed by CHAI is found to be extremely useful by frontline health workers in explaining to new mothers the due date of the next vaccine for their infant. This can be attributed to the lack of funds and dedicated communications cadre including a designer in each cell. The cells' inability to develop material can be attributed to the lack of funds and dedicated communications cadre including a designer in each cell. The cells in this category are able to contribute to developing a communication strategy at the district level by giving inputs on how dissemination and implementation at the district should be charted out.

Category 3

Most of the IEC material has resulted in appropriate and targeted communication at the district level. For instance, the material sent by the national and state governments is made relevant to local contexts by translating the

content to local languages and dialects and changing the imagery and social examples to more culturally relevant ones. In the districts, not all SBCC agenda of targeted communication and driving social behaviour change is successful in the district priorities. The evidence-driven communication of SBCC programme often takes a back-seat and gradually engagement of issue-based communication design is moving towards an IEC-based approach. Though there is evidence of differentiated district for instance, during COVID, the SBCC cell gave developed a lot of material on how to communicate with communities and designed pamphlets and posters for dissemination.



Saas-Bahu Sammelan



The Saas Bahu Sammelan is a state health department's initiative (Madhya Pradesh) rolled out under the directive of the National Rural Health Mission (NRHM). The main objective of this initiative has been to create a sense of bonding and understanding between expectant and new mothers and mothers-in-law to address concerns and misconceptions and myths around child-bearing and post-pregnancy phases for new mothers and their child. Through regular meetings, awareness campaigns, and 'munaadis', the sammelan is conducted at the village level by Anganwadi workers to encourage pregnant and new mothers and their mothers-in-law to gather and participate in discussions on socially relevant topics, to engage in a healthy discussion in a safe space in address and ask and get clarifications on women's health, childbirth, personal hygiene, etc., in a community-based environment. A larger objective of this initiative is to protect the girl child.

KEQ 5: To what extent have the SBCC Cells improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates?

Category 1

The state's SBCC Cells are currently RMNCH+A and RI focused. However, within the SBCC, there is a need for improving coordination between the NHM and DHSFW whereby resources and SBCC activities can be shared and allocated between the health departments where the Cells can have equitable participation in development of the PIPs and pool in resources to IPC than on printing materials in order to make the transition to SBCC more effective, and to strengthen SBCC activities. There is better convergence at the block and district levels which are aligned to WASH and ICDS and engaged in SBCC health communication. Medical college faculties have monitored programmes with the support of UNICEF, although there was no further insight on the same. Although the NGOs work with the support of the NHM in remote and hilly tribal and other socio-economically vulnerable areas, it was found that collaborations and engagements with development agencies and NGOs are limited. As was noted, that boat clinics and community IEC is not yet comprehensively covered. NGOs such as the Voluntary Health Association of India (VHAI) engage in issues such as communicable diseases, Reproductive Maternal Child and Adolescent Health, immunization, food and nutrition, and community participation and ownership through innovative programmes (egs: SALT, KHOJ, Arunoday), the organization does not work very closely with the state Cell. The State cells partner with bilateral and donor agencies such as WHO, UNDP, UNICEF, and JSI. Additionally, the NHM BCC Cell collaborates with a few NGOs and CBOs for communication outreach activities. The DHSFW has had a partnership with the Community Medicine Department of Assam Medical College.

Category 2

The cells are housed under the health department which already has a large network of collaborations and partnerships in place which the cells can leverage to their benefit. Though the cells have been able to develop convergence with allied departments and development agencies to carry out implementation of SBCC based programming, there is potential to further strengthen convergence with allied departments which is often restricted by the mutual exclusivity of programs within these departments. The cells also have partnerships with NGOs with whom they collaborate for more effective implementation at the community level as NGOs usually have field level

presence. In UP, there was a one-off incidence of a local sugar factory contributing to the health department's Covid-19 response through its CSR funds.

Category 3

The cells in this category are housed under the NIRDPR and WCD. Both departments already have a large network of collaborations and partnerships in place which the cells can leverage to their benefit. Though the cells have been able to develop convergence with allied departments such as WCD for CRU and Department of Panchayats & Rural Development for the cell in West Bengal, and other development agencies to carry out implementation of SBCC based programming, there is potential to further strengthen convergence with other allied departments which is often restricted by the mutual exclusivity of programs within these departments. There is evidence of engagement of the cells with CSOs/NGOs during the pandemic at an implementation level but beyond the pandemic, there has been limited standing partnerships.

CRU has an added advantage of being housed under a national institute which gives it access to the institutes' established systems and institutions of service delivery (SIRDs) at the state level too.

KEQ 6: Have the SBCC Cells successfully fulfilled their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships?

Role	Category 1	Category 2	Category 3
Evidence Generation	Evidence generation has not been conducted by the cells in category one.	Evidence generation has not been conducted by the cells in category two.	Evidence-driven communication design of the SBCC program
Strategic Planning	The cell functions within the mandates and the allocated budget provided by the national and state health and family welfare department. This is a barrier to the cells' capacity to develop dedicated strategic plans and implement them.	Strategic planning for communications is majorly driven by the programmatic planning for each respective program. The approach is still top down in nature and hence, plans implemented on the field are unable to take into account needs of the community.	Budget cycle of SBCC affects the work wherein quarterly releases are delayed by months and it affects the planning and programming for West Bengal. CRU is able to develop strategic plans in communications for various departments that seek its technical expertise.
Monitoring	Lack of monitoring due to shortage of human resource and financial constraints	States are not engaged in monitoring impact due to a lack of a dedicated communication cadre and limited funds as it is labour and expertise intensive process.	No impact evaluation or monitoring has been conducted for the activities of the CRU. In WB, the SBCC cell is understaffed to be able to carry out any monitoring activities.
Training	There has been no active training or materials designed for FLWs. BRIDGE training has not been conducted for more than two years.	Cells have conducted capacity development and training workshops for health officials and frontline health workers	No training has been conducted in SBCC cell of West Bengal since 2020. Additionally, inappropriate budget affects training of existing staff

Documentation and analysis	No apparent documentation and analysis of the SBCC activities beyond the general administrative documentation	Odisha: Evidence of behaviour analysis conducted at the block level by studying community members' health seeking activities and social contexts. In MP and Up there is lack of documentation and no evidence of analysis.	No apparent documentation and analysis of the SBCC activities beyond the general administrative documentation
Knowledge sharing	No evidence of knowledge sharing was allied, collaborating departments and development agencies / NGOs was found in the state,	Knowledge sharing has not been priority though some evidence of knowledge sharing through collaborations with other developments and agencies on health and related attributes.	Knowledge sharing is a part of both the cells in this category. It is done through provision of technical expertise to various departments

KEQ 7: How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system?

HSS building blocks	Category 1	Category 2	Category 3
Leadership and Governance	The state leadership has been partially successful to capture the requirements of the communities in Assam, and this was evident through their contribution towards RI enhancement and Covid 19 management. However, at district level, due to scarcity of human resource and dependence on government health authorities for implementation, SBCC activities have not been covered widely and effectively. Additionally, the cell had limited scope of accountability and governance as elements of monitoring and evaluation have been limited in their structure.	The Cell being housed under health department, the leadership of state and district public health system were involved in the functioning of the cell. However, the degree of involvement varied from low in UP to high in MP and very high in Orissa. Cell Leadership played very active role in strategy designing and content development at state level. It was supported by effective and efficient implementation of these strategies at the district level by the district level leadership	CRU's leadership was actively involved in day-to-day functioning of the cell. It also plays a vital role in financial and strategic aspects of the functioning of the cell. Since there is only one state level cell, the leadership and governance structure is centralized. In WB, ownership and leadership of SBCC activities at district level from the government's side has kept the cell relevant and active. Cell workers, both at the district and block levels are well recognized by the relevant leadership who ensure that all stakeholders are working within a collaborative network. Further, the cell is housed within the Zilla Parishad under the leadership of the ADM and ICDS project officer.
Workforce	Consultants deployed by UNICEF have been actively involved in bridging the gap in communication related interventions in the state. The field visits, training and interactions of the cell's human resource with FLWs played a major role in community mobilization and enhanced coverage of the government interventions. However, limited exposure to hard-to-reach areas	The workforce engaged in SBCC activities in category two are health officers by designation who already have roles and responsibilities assigned to them. Their capacity to focus on communications activity is limited as priority is given to the governance and programmatic aspects of their professional role. Further, even at the field level, FHWs reported a similar dual burden of programmatic and	It is the only category, where in the workforce of the cell does not consist of health officials. The workforce in Telangana consists of direct members of the cell including the designer, and members of PRI for field level implementation. With NIRD&PR being a national level institute, the cell has access to all its state level counterparts (SIRD&PR across states). In West Bengal, district or block administration personnel

	continues to be limited in certain regions of the state.	communications activities. Hence, the need for a dedicated communications cadre at all three levels was strongly felt for better functioning of the cells in respect to human resource utilization	manage the activities of the cell. cell workers, both at the district and block levels work within a collaborative network.
Service Delivery	The Cell has been primarily focusing upon increasing the RI coverage in the state, and over the years due to its consistent efforts, intervention has gained momentum and vaccination coverage has enhanced across the state. However, the cell has not been able to strengthen the service delivery of promotive, preventive and curative measures towards good health. Collaborative efforts of ANMs, and Anganwadis and ASHA has enhanced the coverage of SBCC activities but the shortage of FLWs and the need for scaling up their training and skill sets is needed for wider community interactions	SBCC cells in category two are housed under the health department and hence work well within the ambit of flagship programs of this department. The cells act as a facilitator of communication activities for various health interventions and programs and develop strategic plans to bring about social change through communications. Though, the service delivery of the cells in category two is limited by a lack of adequate funding and absence of a dedicated communications cadre	CRU's service delivery is limited to provision of technical expertise to other departments and training of master trainers. It develops comprehensive strategic communications plans as per the need of the said departments. Further, CRU translates and contextualises IEC material sent by the central government. The roles and responsibilities of the cell in West Bengal is to act as a facilitator of communication activities for various interventions and programs under DWCD. Its targets for service delivery are dependent on the programmatic needs of the said interventions
Finance	The strategies of cell were inclined towards RI and hence, financial resources provided for the promotion of immunization interventions were being extensively utilized by the cell. The budget is managed through collaborative efforts of DMO, DCM, DEMO, etc. all work together on PIPs. Cell and system strengthening can be achieved through sufficient budgeting and availability of independent financial resources for the cell. There is thinning of budgets at the time of national review of the state PIPs, which leads to communication activities being placed under innovative schemes within their respective budget heads. There is a single-nodal agency where funds come in and which are then distributed from the source to other departments.	The cells in category two receive their monetary allocation from the respective program for which they have to carry out communications activity; meaning there is no dedicated budget for the cells. The cells are not allowed to use the funds as they seem fit to carry out SBCC activities rather the department gives out program wise mandates on which activities to spend how much on. The monitoring of expenditure of this fund is done through checking whether the expenditure target was reached for each item suggested by the department. Hence, the cell works strictly within these guidelines.	The cell in Telangana has adequate budget for SBCC activities and has a say in the yearly budget planning. However, it was mentioned that during Covid-19 response, it has not been able to utilize the available budget and the relevant funds have been returned to UNICEF. The allocated budget for the cell in West Bengal are drawn from existing IEC budgets of the respective departments like WCD, SBM, NHM and a certain section is funded by UNICEF. The budget is spent on certain activities like miking, nuked natak, folk arts etc.
Information	The cell is aligned with health department of the state government and hence, cell's priorities and sources of information are inclined towards national and state objectives. The cell utilizes the information and material provided by the health department. Local	The cells rely on the material provided by the health department which includes materials such as posters and banners to spread awareness and information relating to positive social and health practices amongst community members. Sometimes material	Various departments and programmes such as WCD and SBM have approached CRU for technical support in strategizing and developing communication materials. CRU aids in customizing communication materials of the central government by translating them

	contextual issues are highlighted in the communication strategies of the cell as flow of information largely follows top – down approach. However, much more socio-cultural contexts, language, and dialects must be assessed	and content developed by other development agencies such as UNICEF, CHAI and UNDP are also used as part of dissemination of health messaging. This can be attributed to the lack of funds and dedicated communications cadre including a designer in each cell	in local languages and adding context specific images and examples. In West Bengal, the cells are given need-based pamphlets by DWCD to distribute in district and block. Also, UNICEF's materials for communication like posters, pamphlets, key messages are used in health information dissemination.
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Conclusion

The cells across all three categories were found to be lacking in three out of the four short term outputs mentioned in the theory of change. The cells were not successfully able to establish systems and processes to design, implement and monitor SBCC plans/programs and use evidence for programme planning hence leading to limited evidence, documentation, and completed studies on SBCC. Though it was found that effective dissemination for SBCC program evidence through workshops, media, etc was being taken place. Hence, it can be concluded that within the evaluation criterion of effectiveness, the cells have a major scope of improvement to achieve the first and fourth longer term outcomes in the theory of change namely, institutionalization of SBCC programming through sustainable systemic reforms, and positive policy environment created for SBCC programming across sectoral development initiatives.

6.3. Efficiency

Efficiency measures the extent of resource utilization in the optimum, economical and timely manner. It provides justification about resources utilized in the achievement of intervention's goals. Evaluation of efficiency of an intervention includes critical evaluation of timeliness, economic and operational efficiency. Throughout the SBCC cell evaluation, programme was examined through cell's functioning within allocated resources and within planned timelines.

The efficiency of the cell was analysed through insights received by interviewing state, UNICEF consultants (loaned staff) and district level officials of SBCC cells.

KEQ 1: Did the SBCC cells efficiently deliver / implement their activities and function within the planned/ allocated monetary resources?

Category 1

This has been partially achieved. SBCC activities use funds from the Single Nodal Agency (SNA) and do not have dedicated funds presently, and thus Child Health or Maternal Health are included in their respective sections. As it was found that the SBCC Cell is not duly consulted during the PIP formulation, thus impacting communication activities vis-à-vis the budget. Although budget for FY 2020–2021 increased in Assam, it was mainly allocated for Anemia Free India strategy, a programme that is conducted through track and act for improved planning and allocations which is at 48 percent. Budgets have increased due better coverage of IFA supplementation and use of 'Hemoglobin' meters . It was difficult to ascertain from the evaluation if the budget was utilized anywhere other than RMNCH+A.

“System strengthening is necessary through sufficient budgeting, M&E, etc., for the state-level plan for RI across 33 districts in Assam. Under the new country program, the cell will prioritize nutrition and health too.”

- Study Participant (Assam)

Category 2

The cells in category two receive their monetary allocation from the respective program for which they have to carry out communications activity; meaning there is no dedicated budget for the cells. Since the state governments are still in the process of transitioning from IEC to SBCC, their understanding of estimated expenditure on communications activities is still guided by an IEC inventory of printing posters and banners with the recent inclusion of activities such as wall painting and mega phoning.

"We received about INR 2000 for a rally. It included everything. I made the banner myself and gave it to ASHA. I bought paper and all from my pocket. Funding is very less. We got a camera last year. We take IEC funds from programmes."

- Study Participant (Odisha)

For example, in Odisha even though the budget increased from INR 2 crores in 2011 to INR 200 crores in 2022, the number of programs and initiatives being run by the cell also increased and thus ensured that the deficit was still maintained. Hence, the cells' efficiency in delivery is limited by inadequate funding which does not account for many aspects of SBCC such as monitoring, research and evidence generation.

Further, the cells are not allowed to use the funds as they seem fit to carry out SBCC activities rather the department gives out a detailed instruction on which activities to spend how much on. The monitoring of expenditure of this fund is done through checking whether the expenditure target was reached for each item suggested by the department. Hence the cell must work strictly within these guidelines.

Category 3

The cell in West Bengal has inadequate financial allocation to efficiently deliver SBCC activities. For example, in the district of Purulia, only 20 schools could be selected for the WASH in school initiative due to limited budget availability. Further, inconsistencies in the budget cycle of SBCC affects the functioning of the cell wherein quarterly releases are delayed by months which affects the planning and programming. Though the SBCC consultant and budget available from UNICEF is a resource which the district cells are able to leverage in the functioning of the cell.

The cell in Telangana is able to deliver efficiently within the allocated monetary resources as it has a say in the annual budget planning which is done vis-à-vis its annual action plan. Hence, the cell is able to advocate for the budget it would require to efficiently carry out its activities for the year. However, it was mentioned that during Covid-19 response, it has not been able to utilise the available budget and the relevant funds have been returned to UNICEF.

KEQ 2: Did the SBCC Cells efficiently deliver/implement their activities and function within the planned/allocated human resources?

Category 1

This has been partially achieved, as there is shortage of FLWs at the district and block levels, specifically in remote and hard to reach areas, and additionally, the FLWs lack adequate and advance training in building their interpersonal skills. With a larger segment of FLWs in the aging category, and no new recruitment of FLWs currently, there is a wide gap in practicing the skill after training. This also has had an impact on the coverage of immunization which has not reached a hundred percent due to limited outreach and inaccessibility issues. The Cells although short-staffed, have been able to deliver the SBCC activities in a timely manner without delays, particularly prescribed tasks which are delivered by the Cell on a regular and periodic basis, and which are well within the scope of the duties of SBCC officers.

Category 2

The cells' allocation of human resources depends on state to state as the structure of each cell is different and other external factors such as vacancy in positions of health officials in this state also affects this allocation. For example, in Odisha there was only one resource person in the block level cells, whilst in UP there is an issue of vacancy in positions of health officials at district and block level. The capacity to focus on communications activity by these health officials at block, district and state levels is limited as priority is given to the governance and programmatic aspects of their professional role. The need for a dedicated communications cadre at all three levels was strongly felt for better functioning of the cells in respect to human resource utilization.

Category 3

The cells can efficiently deliver within the allocated human resources. CRU does not have a mandate to implement the communications strategies it has developed. For the processes of developing strategies and contextualizing IEC and SBCC material, it has adequate human resources and can deliver well within the allocated human resources. For example, having an allocated designer within CRU has proved effective in quickly turning around campaign material at a low cost as compared to hiring designers for every campaign. In West Bengal, effective convergence of block level SBCC staff with frontline workers and Panchayat workers has allowed the cell to deliver efficiently within the allocated human resources.

	Empowered frontline health workers for a robust health system	
Eruptions of laughter and loud chatter welcome you into Balrampur Anganwadi centre. Frontline health workers such as anganwadi workers, ASHA workers and ANMs have congregated in the centre for an interaction with the evaluators. The workers are confident, well informed and experienced in bringing about social behavioural change through health messaging and communications. Details of the myriad of health interventions they contribute to are at their fingertips. Their engagement is active, and they voice opinions and suggestions that they have already shared with their superiors. There is a lot of focus on empowering frontline health workers in Uttar Pradesh. The frontline health workers are equipped with the latest SBCC material available in the state, such as health educational videos (which are readily accessible in their own personal mobiles), pamphlets, posters and banners on topics such as COVID protocols, hand washing, routine immunization, etc. They also undergo regular training in communications and understanding health indicators in their local context. Most importantly, they are in direct line of communication with their superiors, which includes not just the ANMs but also other health officials such as the chief medical officer and health education officers. This sort of open communication has led to better access to resources for the frontline health workers and has harboured a sense of empowerment in the workers who feel that their voices and opinions.		

KEQ 3: Did the SBCC Cells sufficiently deliver/implement their activities and function within the planned/allocated timelines?

Category 1

Even though FLHWs have received training on communication intermittently, there is gap in scaling up their attitudes, knowledge, awareness, and skills to keep pace with the frequent and evolving health schemes and programmes. This also has had an impact on the coverage of immunization which has not reached a hundred percent due to limited outreach and inaccessibility issues. While the Cell works under tight timelines, particularly when there are new SBCC campaigns on RMNCH+A to be delivered or awareness to be shared. During Covid-19 and at other times of immunization activities, the Cell has worked beyond its capacity to be able to largely meet the allocated timelines.

Category 2

Since the cells are housed under the health department, they are mandated to carry out the communications activities for various health intervention and programs. The cells work on a challenging timeline as they must manage to fulfil their role in long term programs while also organizing various days celebrated by the government to bring about increased awareness regarding health practices and increase of uptake of health services such as Poshan Pakhwada, Swachh Bharat Mission day, international yoga day and world immunization week. This affects the efficiency of the cells' delivery as it results in reduced time and capacity of the cell to engage in other aspects of SBCC such as evidence generation, monitoring impact and conducting research on behaviour change.

Category 3

In West Bengal, the budget cycle of SBCC affects the cell's efficient delivery within planned timelines. Due to delays in quarterly releases by months, the cell is unable to carry out its activities in a timely manner. These delays affect the planning and programming of the cell and implementation of its activities. For CRU, whilst they are able to more or less deliver efficiently within the plan the timelines, the cell does work on a challenging timeline. The dual responsibility of organizing various days, like, SBM day, celebrated by the government along with regular SBCC activities, act as barriers for functioning within allocated timelines.

Conclusion

The short term output goals mentioned in the theory change which fall within the ambit of evaluating efficiency are namely availability of a pool of skilled functionaries in effective SBCC programming, monitoring and implementation and increased allocation and utilization of budget/resources for SBCC programming. Across the cells in all three categories, with the exception of the cell in Telangana (CRU), it was found that there is a need for a skilled and dedicated cadre of functionaries with expertise in communications. Further, the same five cells (with CRU as exception) also exhibited a need for increased budget to ensure efficient delivery of SBCC activities. This indicates that the cells have a major scope of improvement to achieve the first and fourth longer term outcomes in the theory of change, namely, institutionalization of SBCC programming through sustainable systemic reforms, and a positive policy environment created for SBCC programming across sectoral development initiatives.



Har Ghar Dastak campaign



The Har Ghar Dastak Abhiyan launched in 2021 is aimed at vaccinating and covering eligible populations groups (and those in hard-to-reach areas) for the first, second and precaution doses through house-to-house and door-to-door campaigns. The campaign is aimed at old age homes, schools and colleges, including the out-of-school children, and tribal rural communities and workers in brick kilns, etc. The campaign was relaunched as “Har Ghar Dastak campaign 2.0” to accelerate the coverage of COVID19 vaccination across States and districts, widely undertaken by NGOs. It is undertaken by utilizing the local influencers and community leaders to enhance the awareness among communities to take vaccine doses.

6.4. Sustainability

Evaluation of sustainability of the intervention involves exploring institutional, financial, social, and environmental capacity and capability. It provides crucial insights into the perpetuation or estimated perpetuation of the intervention's positive impacts in shorter and longer duration. During the evaluation of SBCC cells programme, probability of integration of SBCC programming in state department planning, government's capacity to carry out SBCC activities independently. Additionally, adequacy of the financial allocations and partnerships to carry out the activities of the SBCC Cells was also evaluated.

The sustainability of the cell was evaluated through insights received by interviewing state and district level officials of SBCC cells. It was corroborated through interactions with UNICEF SBCC specialists at national and state level.

KEQ 1. To what extent have the different state departments integrated SBCC programming into its planning and implementation of initiatives?

Category 1

Assam was one among the first states to have established an SBCC cell in 2007 brought under the NRHM for converging all health communications at on place. Under the DHSFW, the Cell focuses on communication related

to RI to address and eliminate the surrounding misconceptions that have so far constrained the state from attaining full immunization coverage. In this regard, the departments have integrated SBCC programming into its planning and implementation of SBCC initiatives, as it has comprehensively addressed a few critical health-related areas within the same programme such as flagship initiatives such as Mission Indradhanush and Mission Tejaswee, RMNCH+A, WASH, and MCH, etc., some key socially relevant issues related to child and adolescent SRH and women's SRH have not been integrated within the SBCC programmes. However, state departments need further integrate SBCC programming into its planning and implementation more holistically with focus and emphasis on behavior change.

Category 2

The transition from IC to SBCC has not fully happened, and has happened to varying degrees, for the cells in category 2. Planning, programming, and implementation of initiatives take into account communications activities only to the extent of IEC. For example, in UP, communications activities are mostly limited to printing of posters and banners. Even in Odisha, which is the best performing cell out of the three in this category, the integration of SBCC programming in planning and implementation of initiatives is restricted by factors such as budget. For example, for the implementation of SBCC activities within a program, a district is only given 2000 rupees to conduct the activities. With the budget being so small, majority of it gets spent on the tried and tested methods of IEC such as printing posters and banners, and almost no budget is left for conduction of SBCC methods such as natak, natak and IPC.

Though it is important to note that since the cells in category two are housed under the health department in the state government and their work is in alignment with state and national level programs, this allows for a certain level of integration of SBCC programming in the state's planning and implementation of initiatives, including allocation of funds for SBCC activities from program budget and designating of human resource for communications from within the workforce of the department.

Amongst the three cells in this category, the integration of SBCC programming in the states planning and implementation of initiatives was lowest in UP. This can be attributed to the lack of an active cell at the state level which would otherwise play the role of advocating for integration of SBCC in state programming.

Category 3

States' shift from IEC to SBCC process has been only partially fulfilled. Cells have not been able to bring the SBCC agenda to the forefront and hence, SBCC programming in state department planning has not been fully integrated.

In West Bengal, the visibility of the block volunteers at the ground level and regular interactions and updates on SBCC Cell's activities have widely helped in integration and collaboration, as the cell's members work with officials from AWW and CDPO related to the WCD, and also with the BMOH and MNB coordinators. The SBCC cell's engagement with District Social Welfare officers rather than with Zilla Parishad has impacted integration to a certain extent.

CRU in Telangana has been able to establish itself as a one stop shop for support and technical guidance provision on communications activities and through its work, has been able to help departments understand the importance of SBCC. Hence, various state departments reach out to the cell for support to develop communications strategy for programs such as SBM and JJM out of their own understanding of the significance of having a communications strategy for their respective programs. Hence, it can be said that CRU has been able to contribute to the integration of SBCC programming in the state department planning.

KEQ 2. Does the government (at the state and district-level) have the capacity to successfully carry out the activities of the SBCC Cells independently?

Category 1

The findings indicate that there is potential for the government to be able to successfully carry out the activities of the SBCC cells independently. The cells housed under the health department, are working on attaining comprehensive coverage of RI across rural and urban areas. However, the state's capacity building will include increasing the workforce strength of FLWs at the district, and down to the block level. The SBCC cells need a more socially aware and SBCC-oriented (not only IEC-oriented) personnel, inclusion in the programme budget for mainstreaming SBCC and gender equity, and leadership from senior government officials and other government resources. Only if the cells can actively endeavor to build capacity of the SBCC for officials within the departments, it can accelerate the integration of SBCC into its programming and carry out the activities.

However, greater accountability, coordination and convergence with SBCC-orientation and streamlining budget allocations can enable the government to take the SBCC activities forward. At this point, it was observed that the government both at the state and district level can independently carry out the commonplace activities in so far as the health and communication of SBCC mandates are concerned.

Category 2

For cells in category two, the findings indicate that there is potential for the government to be able to successfully carry out the activities of the SBCC cells independently. These cells are housed under the health department, which gives them a few advantages such as access to a large cadre of health workers positioned down to the block level, inclusion in the program budget, governance and leadership from senior government officials, and other government resources. With the cells actively advocating for and building capacity in SBCC for officials within the health department, the department has been able to develop its capacity to integrate SBCC into its programming and be able to successfully implement it at the field level.

Though in UP, there has been a lack of ownership and accountability from the government's side, especially at the state level, due to which the potential for the government to be able to successfully carry out the activities of the SBCC cells independently is not fulfilled and has remained limited.

"SBCC should be strengthened in the state and HEOs should be empowered. Further, such officials should be given regular training. The cell's utilization should be increased, and government should take up more ownership"

- Study Participant (U P)

Category 3

In both states, the government (at the state and district-level) have been reported to have limited capacity to adequately carry out the activities of the SBCC Cells independently. They have limited technical knowledge / expertise.

Though in West Bengal, the cells work very closely with district administration and are flexible to adopt to changes. The block volunteers and district coordinator are selected through district recruitment processes. And any requests from district administration for IEC support is well received by the department.

In CRU's case, its financial allocation comes solely from UNICEF. Hence, the government presently has limited opportunity to support adequate financial allocations and partnerships to successfully carry out activities independently. Though each department and programme have a budget for IEC/SBCC activities, this only covers the cost of development of material.

KEQ 3. Do the Cells and their implementation structures (at the state and district-level) have adequate financial allocations and partnerships to successfully carry out the activities of the SBCC Cells independently?

Category 1

The existing partnerships as was observed, are not yet strengthened for better communication strategies.; Additionally, a lack of convergence with other relevant stakeholders for collaboration, implementation, and sustainability of SBCC strategies is currently not robust in the state. The SBCC cell under NHM receive financial support through the State's PIP, as the NHM PIP is the main source of funding. IEC-related planning has been the aggregate of the budget of IEC under the umbrella of the NHM at the state level. Moreover, the funding for these cells comes through program budgets which are largely channelized for RI communications activities. The budget is managed through collaborative efforts of DMO, DCM, DEMO, etc. all work together on PIPs. Cell and system strengthening can be achieved through sufficient budgeting and availability of independent financial resources for the cell. There is thinning of budgets at the time of national review of the state PIPs, which leads to communication activities being placed under innovative schemes within their respective budget heads. There is a single-nodal agency where funds come in and which are then distributed from the source to other departments. streamlining budget allocations can enable the government to take the SBCC activities forward. Community engagement although supported strongly by community influencers, it is largely lacking due to shortage of human resources, coordinated and convergent efforts between departments, lack of intersectoral convergence, and community engagement are factors that can scale up and sustain through a more empowered community-based interventions and community-led initiatives with support from the state and district cells. There was a consensus among district officials that the Cell has not disseminated adequate funds to carry forward SBCC activities in

areas and among communities where urgently needed. However, whether it is a paucity of funds or a lack of optimal usage of funds at appropriate contexts, was difficult to ascertain from the evaluation. Partnerships too are limited and most definitely have not been utilized to the fullest.

Category 2

The funds for the cells in category two are allocated from within the budget of the health department. In most cases, the budget for SBCC/IECC activities is accounted for from the program budget itself. All three cells within this category spoke about experiencing limitations in working within the allocated funds. The cells also faced an issue in the way the fund expenditure is being monitored, which is through checking whether the cells have reached their spending target as allocated by the health department at the state level. This method was pointed out to be rigid and resulting in a top-down approach which did not consider the difference in the local contextual reality across the state.

The cells have adequate partnerships with other development agencies and other departments through the collaborations of the health department. Development agencies such as CHAI, JHPIEGO and UNDP also develop communications material for various aspects of health programming. Such material contributes to the larger objective to reach out to as many members of society especially those from vulnerable populations.

Category 3

The cell in West Bengal has inadequate financial allocation to effectively carry out SBCC activities. Delays in the budget cycle of SBCC affects the functioning of the cell wherein quarterly releases are delayed by months which affects the planning and programming. There is an instance where due to limited budget availability, the cell in Purulia had to choose only 20 schools for WASH in School initiative in an entire district. Though the SBCC consultant and budget available from UNICEF is a resource which the district cells are able to leverage in the functioning of the cell.

The cell in Telangana has adequate financial allocations as it has a say in the annual budget planning which is funded wholly by UNICEF. However, it was mentioned that during Covid-19 response, it has not been able to utilize the available budget and the relevant funds have been returned to UNICEF. Though, it is important to note that since the budget is funded completely by UNICEF, the sustainability of this source it's limited till the time UNICEF continues to extend this support. The department under which the cell is housed (NIRD PR), does not allocate any part of the funding for the cell which has been noted to be an important indicator of sustainability. In view of the lack of funding from the state department under which the cell is housed, increases the dependency on UNICEF supported resources and budget.

SBCC cells need to develop and/or strengthen partnerships with the department of health (in both states), WCD (in Telangana) and Panchayati Raj (in West Bengal) department for implementation of SBCC activities. It would allow increased leveraging of available IEC/ SBCC budgets with allied departments. Additionally, NGOs should be involved in implementation of SBCC activities.

Conclusion

Against sustainability as an evaluation criterion, there are initiatives being taken for achievement of all three outputs mentioned in the theory of change, namely, training of leadership and officials on SBCC programming reflecting the shift from IEC to SBCC, availability of structured plan for interactions with community members, and field functionaries, and availability of relevant job aids, tools, AV materials, prototypes, etc for SBCC. While progress is being made in each of these, they are yet to be fully achieved. In the case of Odisha as an example, government ownership is high and hence the cells are able to actively advocate for and build capacity in SBCC for officials within the health department, integrate SBCC into its programming, and be able to successfully implement it at the field level. Hence, with the achievement of these outputs, the cells will be able to contribute to the moving towards the longer term outcomes namely, institutionalization of SBCC programming through sustainable systemic reforms, and positive policy environment created for SBCC programming across sectoral development initiatives.

6.5. Coherence

Evaluation of Coherence of the intervention helps in gauging the degree of support, coordination or undermining, hinderance witnessed with other interventions and policies of the stakeholders. In current interdependent and interrelated world, it becomes essential to explore the impact of internal and external stakeholders, their interventions and policies, to ensure effective and efficient results. Here, evaluation of SBCC cells programme involved examining coherence based on partnerships and integration of SBCC in state programming.

We evaluated interlinkage of SBCC cells with other development partners and its alignment with interventions being carried out by state for improving health systems by interviewing state, district level officials of SBCC cells and consultants of various development partners and NGOs.

KEQ 1. How do the activities/interventions of the SBCC Cells interlink and synergize with the existing/other programme/initiatives by other relevant actors (NGOs and development partners and other sector players)?

Category 1

Partnerships with NGOs as was found have proved effective during Covid-19. However, the NGOs on a regular basis are not actively involved, nor exchange any knowledge or work on SBCC activities. This is despite the fact that NGOs are also working closely with communities on RI programmes, as do the SBCC Cells. However, at this point, there is no concrete linkage of NGOs with the SBCC cells especially related to more focus-driven and pointed work on women's health, gender and equity. NGOs such as the Voluntary Health Association of India (VHA) engage in issues such as communicable diseases, Reproductive Maternal Child and Adolescent Health, immunization, food and nutrition, and community participation and ownership through innovative programmes (egs: SALT, KHOJ, Arunoday), the organization does not work very closely with the state Cell. However, they too do not specifically work on gender and equity as a distinct or special attention aspect of their work. It was noted that NGOs are open to collaborating with the state cells to work on the ground level with communities (and FLWs) for wider outreach of SBCC and look forward to convergence. At the state level, there is no mapping of NGO partners is not yet carried out at the local levels, even as it has a limited number of partners and unskilled resources. Most importantly, the state faces challenges in reaching or directly working with communities in sensitive areas or hard-to-reach areas. In case of RI activities, greater institutional support will be further needed. Although the NGOs work with the support of the NHM in remote and hilly tribal and other socio-economically vulnerable areas, it was found that collaborations and engagements with development agencies and NGOs are limited. As was noted, that boat clinics and community IEC is not yet comprehensively covered and is only in its nascent stages (boat clinic concept is one of the good practices from Bangladesh which Assam intends to replicate in the state's char areas and LODOR). The State cells partner with bilateral and donor agencies such as WHO, UNDP, UNICEF, and JSI. Additionally, the NHM BCC Cell collaborates with a few NGOs and CBOs for communication outreach activities. The DHSFW has had a partnership with the Community Medicine Department of Assam Medical College.

"Convergence for SBCC activities can be seen to an extent on ground but it is not substantial at the state level"

- Study Participant (Assam)

Category 2

The cells engage with various development actors through the collaborations that the health departments have such as with NGOs, International Development Agencies like UNDP, CHAI, BMGF, etc. to ensure that they delve into specific areas for e.g., youth mobilization, immunization, etc. comprehensively. There are collaborative efforts between the cells and these development agencies to develop new material and seek technical expertise. The implementation aspect of the programs is carried out through government mechanisms already in place. The development agencies leverage these resources and mechanisms to aid and enhance the government work.

Category 3

Though CRU mostly works in the area of WASH, and hence has convergence with Swachh Bharat Mission and Jal Jeevan Mission. It also has interlinkage with other departments such as WCD and has contributed to programs such as Poshan Abhiyan. CRU does not have any ongoing interlinkage with other development actors and NGOs. Though during Covid-19 pandemic, they did leverage collaborations with NGOs for on file dissemination of Covid protocols.

In West Bengal, regarding interdepartmental convergence, the cell collaborates with the health and sanitation departments, and with district Information and Culture officer. The cell does not have any ongoing interlinkage with other development actors and NGOs in the state, despite the presence of multiple such development actors working in the field of health and other related fields such as nutrition. Since there is no convergence or collaboration amongst these actors including with the cell, there is duplication of effort on multiple fronts as each actor is working in silos.

KEQ 2. How do the SBCC Cells interventions align/synergize/fit with other interventions being carried out by the state for improving health systems?

Category 1

Overall, the SBCC Cell is striving to align to its interventions in health objectives. Programmes such as Mission Indradhanush and Mission Tejaswee, and Anemia Free India have been aligned to the objectives of immunization coverage strengthening in priority districts. This has also been largely possible due to the structure of the cell being housed in the NHM as well as the DHSFW. The Health Communication in Assam is implemented by the District Extension Media Officer (DEMO), (now the state Cold Chain officer under NHM). The other members include the DHSFW at the district level, Block Community Mobilizer (BCM), NHM and Health Educator (HE)/Block Extension Educator, DHSFW at the blocks. ASHA and ASHA Supervisors are the main frontline cadre involved in informing communities on available health services, disseminating health messages and assisting in utilization of health services. Service delivery at the state level have comprised various innovations towards reduction of mortality, such as through the Rural Health Practitioners (RHP), Boat Ambulance and Clinic (still in the discussion phases and was not heard of from participants in the study, except for one SBCC officer and participant), Mobile Medical Unit (MMU), ASHA Radio Program, Sarathi-104 (Health Information Helpline & Complaint Redressal system), etc. However, on ground it was found that accessibility of these in remote and hard-to-reach areas such as Chars, were difficult, and still a major concern. The gaps have been addressed and reduced in Reproductive Maternal Newborn Child and Adolescent Health (RMNCH+A), although services and knowledge and awareness by the health facilities and their utilization need a further scale up. However, health systems need to assess and address the existing communication gaps in priority districts and aspirational districts, in LODOR areas, tea gardens, Char areas, and minority settlements which have precarious, cross-cutting health and social behavior change issues such as early marriage, teenage pregnancy, and school dropout holistically and meticulously. These are factors which are also entrenched in mindsets, ignorance, and poverty of the communities.

Category 2

The cells in category two are highly aligned to the interventions being carried out by the state for improving health systems. Since the cells are housed under the health department, they are essential parts of the programming and implementation plan of various interventions brought about at state and national level. For example, the cells have contributed to spreading inclusive messaging on various state and national level health priorities such as Mission Indra Dhanush, Dastak Sanchari, Measles and rubella awareness, RI, postnatal care, home based care for young and new-born children and institutional delivery.

Since the cells' functioning occurs within the workings of the health department, the cells can leverage the department's key resources such as frontline health workers who act as the last mile connection with vulnerable populations on field. Further, the cells can enhance the functioning of the health department through introduction and advocacy for novel and inclusive methods of communication which lead to social behavior change and contribute to a significant development from the earlier practice of mere distribution of IEC material.

Category 3

Since the cell are not housed within the health department, they have limited alignment to core health programs. Though, they contribute to health systems strengthening by developing communications strategy for programs supporting health initiatives such as nutrition and sanitation programs. For example, CRU in Telangana has contributed to flagship programs such as SBM, JJM and Poshan Abhiyan. For the cell in West Bengal, the expected area of focus is ICDS for Nutrition and the four largest schemes viz., Kanyasree, Rupasree, Lakshmir Bhandar and Social Welfare schemes such as old age and widow pension.

Conclusion

For the program to have high coherence to the allied policy space and stakeholders, the theory of change suggests achievement of two outputs, namely, joint programmes planned and organised with allied sectors and meetings conducted with the same for SBCC, and availability of a network of CSOs/CBOs engaged in demand generation and behaviour change activities. It was found that the cells in Telangana, Assam and West Bengal have not been able to develop a steady network of CSOs/CBOs/NGOs due to which they have not been successful in initiating many joint ventures with allied sector actors. Whilst, in Odisha, U.P. and M.P., it was found that cells are able to leverage the network of other development actors and engage in joint planned programmes. Hence, it can be said for Odisha, U.P. and M.P. that they are closer to achieving the long term outcome goals.

mentioned in the theory of change namely, institutionalization of SBCC programming through sustainable systemic reforms, and positive policy environment created for SBCC programming across sectoral development initiatives.



SBCC programming with effective leadership



The ownership and leadership of SBCC activities at district level from the government's side in West Bengal has kept the SBCC cell relevant and active. The SBCC cell workers both at the district and block levels are well recognized by the relevant leadership who ensure that all stakeholders are working within a collaborative network. Further, the cell is housed within the Zilla Parishad under the leadership of the ADM and ICDS project officer. Receiving guidance and leadership from senior officials of two departments results in effective convergence at the district level. Convergence also exists between block level SBCC staff and frontline workers. The block volunteers of SBCC cell work very closely with Panchayat workers, ANM, ASHA and AWW at the village/GP level.

Further, availability of a senior SBC specialist from UNICEF (loaned staff) - The presence of a specialized staff is found to give technical guidance in leading conversations with district administration, forging partnerships (with NSS, with private partners during COVID to leverage masks, sanitizers etc.) and leveraging available IEC budgets with allied departments (like funds for COVID IEC available with BMOH

6.6. Gender and Equity

From the six evaluated states, it was found that although gender and equity are cross-cutting and a critical component of the SBCC, in practice, the Cells in the states have not yet mainstreamed gender equity. Neither are the Cells gender-cognizant while designing SBCC activities, as was noted under Relevance and Effectiveness across all the categories. Although, the SBCC programmes have principles of **gender and equity in theory**, at present, the gender and equity as a dimension and intersectional pillar of the SBCC is skewed across the states both in thought and practice. For instance, the evaluation in Telangana finds that gender and equity is institutionalized within the ambit of academics; whereas in other states, gender and equity are far from academic institutionalization; in fact, they are not even well-embedded within the SBCC practice. Most of the state cells have overlooked the gender and equity dimension, whereas, in some state level (and even district level cells), officials who were cognizant of the need for deeper enquiries and assessment of gender issues, simply do not have the necessary human resources (neither in workforce strength), nor mindset or evolving attitudes, and neither a dedicated (gender-responsive) budget. We gather that this gap has created a larger caveat at the district and block level where FLWs interactions with communities is narrow, and restricted to the prescribed SBCC activities. The SBCC Cells have given attention to WASH services and RMNCH+A, and hygiene and sanitation, nutrition, and women's health, but have not attended to more fundamental issues of child protection, safeguarding of the rights of adolescent girls, and women. In this context, the Rashtriya Kishor Swastha Abhijan and ARSH (national programmes for adolescent SRH and holistic development) are missing in any of the states during the consultations).

Relevance

Under relevance, it was found that across the 3 categories covering six states, Gender and equity are largely peripheral to most of the SBCC interventions. Nutrition and child survival with the support of community mobilisation have been focus areas, but not as such gender-centric or gender-aware. There is no advocacy at the state and district level on issues of child marriage and adolescent SRH, although the health department is overwhelmed at the number of cases of teen pregnancy and early (child) marriage being reported. Parallely, in some states, male child preference continues to be prevalent among communities in categories 1 and 2. In the states in category 3, as the cells are housed in departments which are other than the health department, the

programmes do not entirely include or focus on gender and equity in its programmatic planning. Whereas, in West Bengal, the state cell housed under WCD, have gender-focused interventions but do not have any specific programmes on equity for vulnerable groups such as tribal populations, people with disabilities, and other poor and marginal communities.

Effectiveness

Under effectiveness in category 1, programmes like Mission Tejaswee are anemia-related for women, girls and new mothers, these are only part of women's health which do not include a gender-inclusive dialogue on the household dynamics of food and nutrition, or women's and girls' need for timely eating or eating 'well'. In category 2, it was found that programmes such as the Adolescent Empowerment Programme (AEP) was implemented across 16 districts under the leadership of District Collectors and the Department of Women and Child Development (DWCD). The states do not yet have adequate and (gender-cognizant) field functionaries to focus on adolescent empowerment, particularly as there is a lack of gender-targeted interventions coupled with lack of literacy in parts of the regions. In Category 3, although there is a lack of stakeholder interventions, and the demand and need of the states is based on contextual requirements of diverse vulnerable populations. Although, West Bengal has ICDS for Nutrition and Social Welfare schemes such as old age and widow pension, there is no specific programmes that are centered on gender and equity. In Mayurbhanj district in Odisha, the Cell has an equity-focused approach wherein they focus on IPC to develop trust building relationships with community members. There is a lack of stakeholder interventions, and the demand and need of the states is based on contextual requirements of diverse vulnerable populations.

Efficiency

The overall shortage of resources in Category 1 as we gather, has not affected the efficiency of the SBCC objectives, but has largely hindered a gender-transformative approach to address and reduce gender gaps, such as social practices of early marriage or school dropouts. In category 2, there is no specific budget allocation for gender and equity, as the Cell is only part of programmatic planning. Hence, despite a programmatic focus on vulnerable including women and children, there are no specific interventions for the same. We gather that prevalent gender disparities will continue to perpetuate without timely SBCC intervention. In category 3, gender and equity in Telangana is constrained due to lack of separate monetary allocation which also impedes the staff from envisioning gender and equity-focused programmes. In West Bengal, although gender is identified as a key dimension of the Cell's activities, equity-based planning is missing, and resources are used only partially mainly on other SBCC activities.

Sustainability and Coherence

We gather from the study that Sustainability and Coherence are far from being attainable at this point, as we have in the aforementioned sections, concluded that gender and equity are the weak and missing links as was found under relevance and effectiveness. In this direction, largely the advantage of the states in Category 3 is that the programmatic focus is on women and children, which can serve as a good motivation for the state cell to have inclusion in its SBCC agenda—tribals, urban poor, and LODOR in its future activities. Reaching vulnerable groups through house-to-house surveys, a mix of communication approaches, including base line studies which are critical for equity, gender and social inclusion. With no gender-responsive budget but for states like West Bengal, advocacy and establishment of the state adolescent cell is of no added value if there is SBCC activity on equity and inclusion to assess the outcome of an adolescent cell on girls and women in tribal and economically vulnerable communities.

Conclusively, gender equity must be holistically as well as granularly integrated into the SBCC strategy to make health-related communication more effective, and to trigger a dialogue around equity and gender relations and roles. From budgeting to mainstreaming, gender and equity needs to be entrenched within the SBCC objectives more firmly, to increase the accessibility of SBCC interventions to large numbers of under-served and marginalized populations of women, girls and even men and adolescent youth.

Conclusion

Gender and inclusion focused integrated SBCC plans/programmes as an output has not been fully achieved by any of the cells across all three categories. Since, public health is the larger ambit within which the SBCC program functions, there is a programmatic focus on aspects of gender inclusion and equity such as maternal health and increasing health information access for LODOR communities. But, no specific focus was found on gender and equity while planning communications activities relating to SBCC programming. Hence, there is limited success of integrating a gender and equity approach in achievement of the longer term outputs namely, institutionalization

of SBCC programming through sustainable systemic reforms, and positive policy environment created for SBCC programming across sectoral development initiatives.



Equity focused SBCC programming



In Odisha SBCC programming is being done in a manner which focuses on equity and local contexts. For example, a large percentage of the population in Odisha are tribals. Hence SBCC programming is done in such a manner that it considers various socioeconomic realities of such vulnerable communities such as religious beliefs, social mores, difference in language and dialect and imagery that they associate with. This results in having to develop SBCC material which is slightly different in each district. The SBCC cells are responsible for this process of making the material relevant in the local context. For example, Mayurbhanj has a tribal population of more than 50% of its total population. The frontline health workers in this district have been given extensive training on how to reach out to hard to reach and vulnerable communities. Jaya Sahu, an ASHA worker, tells us how one community in the area assigned to her had no uptake of immunization services when she started out 10 years ago. This community had various misbeliefs regarding the effects of immunization and hence were weary towards it. With the introduction of SBCC methods, Jaya Understood the importance of strengthening interpersonal communication with the members of this community and to have an empathetic ear when they shared their fears regarding vaccination. She was able to build relationships of trust with these members and has managed to get 100% immunization within the said community.

7. Conclusion

The SBCC cells program was initiated to help state government departments to transition from IEC to SBCC. Though this transition is not yet complete for any of the six states evaluated in this study, the inception of the cells has got the ball rolling in the right direction. The state and district level officials are now familiar with the term 'SBCC' and are aware of its basic conceptual definition and how it differs from IEC. They are also aware of the novel methods being used in SBCC such as miking, interpersonal communication, leveraging key influencers, etc. The SBCC program has contributed to health systems strengthening in these six states by bringing about a change in the way health, ICDS and PRI officials approach and engage with community members.

It can be said that the program is operating in alignment with all four desired **outcomes** mentioned in the **theory of change**. Evidence suggests that the cells are contributing and moving towards outcomes one and four which are creation of sustainable government systems for SBCC programming and creation of positive policy environment for SBCC programming across sectoral development initiatives, respectively by achieving, to varying degree, some of the shorter term output goals such as having leadership and officials trained on SBCC methods, and establishing systems and processes to design and implement SBCC programs. The scope of this evaluation does not include assessing outcome two and three which are namely improved demand and outreach of government programs and improved uptake and community participation towards government sponsored programs/schemes.

It was found that the model of cell in Telangana (CRU) under category 3 was functioning in a manner **most efficient** to integrate SBCC programming in state planning and implementation of initiatives. As opposed to all the other cells, CRU has managed to establish itself as one stop shop for all communications activities in the state which allows the direction of pursuit for CRU to be such that the departments approach the cell for support out of their own understanding of the significance of having a communications strategy for their respective programs. In this way, CRU has been able to market itself as a center of technical expertise in communications which supports various departments in developing communications strategies. Further, it was found that CRU's mandate to only operate at the state level acts as an advantage as its resources can be focused on strategizing, whilst implementation remains within the ambit of the state departments. This also ensures that the cell is able to function effectively in the role of providing technical expertise on communication within a smaller pool of human resources and budget allocation. It is important to note that though CRU's model works best, it is still heavily dependent on UNICEF for its budget allocation. For the sustainability of the cell, CRU should advocate with NIRDPR for fund allocation from within the department's budget.

The SBCC cells' functioning was found to have high **relevance** and alignment with the national and state priorities by contributing to building the communications strategy for flagship programs of departments under which they are housed. The cells have been seen to be flexible and adaptable to evolving needs in the community such as the incidence of Covid-19 pandemic. Further, to varying degrees, the cells have been able to leverage existing systems and institutions of service delivery such as conduction of on field implementation through the cadres of frontline health workers; and have been able to have the presence and guidance of senior government officials in creating the **leadership and governance** structure of the cell. But, due to various limitations such as inadequate **finance**, challenging timelines, a **workforce** which does not specialize in communications or behavior change and lack of inter-departmental convergence, the **effectiveness** and **efficiency** of the **service delivery** by the cell has been negatively affected. The cells have been only partially successful in fulfilling their role as institutions of evidence generation, monitoring and evaluation, documentation and analysis. Although the cells were established to bring about a change in the way communications is planned, programmed and implemented, a majority of them ended up playing the role of a supportive unit to the state's IEC activities. The aspects that the cells are working to successfully deliver in are training and capacity building, strategic planning, knowledge sharing and increasing awareness of community members by disseminating health **information** and positive social practices.

Most of the cells have been able to foster an environment of convergence and collaboration with relevant departments and with other development actors such as IDAs, NGOs and CSOs. Though it is important to note that the need for even higher convergence (especially intersectoral convergence such as a collaboration between health and education department) was noted from the findings across all six states. Through these collaborations, the cells are able to have a higher contribution to integrating SBCC into state programming. Hence, it can be concluded that the cells have **coherence** with institutions and interventions of health and related fields, but not intersectoral.

The cells have a considerable scope of scaling up and being fully integrated in the public health system of the country. However, to ensure the program's **sustainability**, government ownership and accountability of the cells

is of utmost importance. The first step in this process would be for the government to have a complete shift from IEC to SBCC in its attitude and approach. Following which, the government must conduct regular capacity development workshops and training in communications and behavior change methods for its officials who also play the role of communications officers. This would improve the quality of SBCC material being developed, lead to better planning and programming resulting in more technically sound communications strategies.

It was found that although **gender and equity** are cross-cutting and a critical component of the SBCC, in practice, the Cells in the states have not yet mainstreamed gender and equity, and neither are there any gender focused programme beyond a basic programmatic focus on gender within health programs. For example, focus on gender in programs such as Poshan Abhiyan and RMNCHA+. A gender-transformative approach is thus a critical requirement to address social norms, practices in diverse social and economic contexts by the state and district cells. Though, through the programmatic focus on gender and equity within health programs, vulnerable populations such as young others from lower socio-economic strata and those in LODOR communities are included in the state programming.

In conclusion, it can be said that the SBCC cells program **kickstarted the transition of communications activities from IEC to SBCC**. The program's contribution has been significant and in many of the states been the first SBCC intervention initiated. Though the progress has been slow over the past programmatic cycle, it is important to understand that to bring about a change not just in methods but in the approach, understanding and attitude towards communications is a mammoth task. The very principles of SBCC can be applied here to see the slow progress not as a sign of inefficiency and ineffectiveness of the program, but rather as a **work in progress status for the cells**. The cells require guidance and support for at least one more programmatic cycle whilst they figure out their transition strategy to move from being UNICEF supported to completely government owned. **We strongly suggest UNICEF to continue its investment** and support in the program during this period.

Following are a few recommendations to increase the relevance, effectiveness, efficiency, sustainability, and coherence of the SBCC cells and to streamline the goals, efforts and resources being invested in the program.

8. Policy implications and recommendations

The objective of this evaluation exercise was to understand the SBCC Cells' performance towards maximizing utilization of allocated resources and augmenting the results of the health systems strengthening initiatives and other cross sectoral themes of social and health determinants at the state and district level. A qualitative research design was adopted for the same. Findings from this study have provided insights into the status of the existing SBCC cells and identified certain areas for improvement in its functioning.

The suggestive recommendations for strengthening the functioning of SBCC cells are **based on the findings from study, field observations, consultations with stakeholders.**

Confidence in the generalizability of the findings and recommendations is enhanced by the comprehensive strategy and design used for the **selection of the states and arena of stakeholders interviewed as a part of this evaluation.**

Recommendation 1:

Strengthen existing state level cells	Responsible Organization: Department under which cell is housed in states
Priority: High	SBC SS indicator: Leadership and Governance
Time frame: 6 – 12 months	Recommendation for category: I, II and III

Rationale:

As noted under relevance evaluation criteria (Relevance KEQs), among all the evaluated states, presence of an active state SBCC cell ensured high relevance of SBCC activities to flagship programs, and priorities of the national and state governments. It also enabled collaboration among departments, development partners and NGOs for various SBCC activities. However, the state governments have not taken full ownership of the cells. In states like UP and West Bengal, the state level cells are defunct and SBCC activities were being conducted mostly at the district level. Hence, it is recommended that government should strengthen the state level SBCC cells for better implementation of SBCC activities in the state and advocacy for adoption of SBCC approach in state government programming. The state SBCC cell would act as a one stop solution for all the SBCC activities in the state and converge the impact of district level cells.

Action points for State:

- Government should strengthen the state level SBCC cells by setting a clear mandate for its roles and responsibilities vis-à-vis the structure of the state department under which the cell is housed.
- The state cell should enable communication capacity building for health workforce and provide technical assistance to department it is housed under and other allied departments.
- The state cell must assign, but not be limited to, a nodal officer (preferably a BCC expert), a graphic designer, a M&E expert, a communication/ language expert for contextualization of SBCC material to local languages and cultural preference and imagery.
- The state cell should coordinate with the district level officials and FLWs and conduct SBCC activities effectively and efficiently at district and block levels.

Recommendation 2:

Prioritize key areas for provision of technical expertise	Responsible Organization: UNICEF
Priority: High	SBC SS indicator: Workforce and service delivery
Time frame: 6 – 12 months	Recommendation for category: I, II and III

Rationale:

The evaluation found that district level engagement by UNICEF in form of district level loaned consultant was limited to contribution towards capacity building and SBCC repository management. Their role at the district level can be taken over by a health / PRI official, already placed at the district. However, states are yet to make the transition from IEC to SBCC. In view of this, UNICEF should continue its assistance to the states, but with focused engagement. To ensure focused and effective delivery of SBCC, we strongly recommend that UNICEF should concentrate efforts and resources at the state level only through the continued placement of an UNICEF consultant. As evident in states such as Odisha, Telangana and Madhya Pradesh, the presence of a state level consultant who is able to advocate and provide technical expertise at the state level. In the case of Odisha and Madhya Pradesh, a cascade effect of the contribution of the state consultant resulting in increased effectiveness of service delivery was observed even at the district level.

Action points for UNICEF:

- Technical expertise – Support from UNICEF can be instrumental to reduce the gap that currently exists within SBCC programming, particularly related to gender disparities, prejudices, and ill-practices of early marriage, empowerment of girls and safeguarding sexual and reproductive health. UNICEF should continue to extend its support in SBCC activities across the cell through provision of technical expertise. To do so, UNICEF must continue to place a consultant in the state level cells in all six states whose main responsibility would be to set and implement the transition plan for at least one more programmatic cycle of five years. The consultant must have domain expertise in social behavior change and communication.
- During the next programmatic cycle, whilst the cells have the guidance of the UNICEF loaned SBCC expert, the focus of the cell should be to develop a clear transition strategy. This strategy should be developed to cover, but not be limited to, aspects of financial resource management human resource management, methods of evidence generation, monitoring and evaluation framework, community engagement and inclusion of gender and equity lens in SBCC programming (expanded in Recommendation 3). This would enable a smooth transition from SBCC activities being UNICEF supported to becoming fully owned by the state government.

At the end of the next programmatic cycle of five years, a similar evaluation should be conducted to assess the progress of the transition of the state and decide future course of action including the decision to continue stationing of the UNICEF consultant at the state level cell.

Recommendation 3:

Develop SOPs for SBCC cells at state and district level	Responsible Organization: UNICEF
Priority: Medium	SBC SS indicator: Service Delivery
Time frame: 12 - 18 months	Recommendation for category: I, II and III

Rationale:

Since the state departments under which the cell is housed have not fully transitioned from IEC to SBCC, they require technical support to effectively deliver SBCC activities (Sustainability KEQ 2). As suggested earlier, an UNICEF SBCC consultant who is a technical expert in SBCC programming should continue to provide support for another programmatic cycle of five years. In addition to the consultant, whose engagement is for a fixed period of time, the development of principle guiding documents in the form of clearly laid out SOPs would ensure a continued support and source of knowledge of methods of SBCC for the said departments. Hence, UNICEF is recommended to develop SOPs on various aspects of the functioning of cell to effectively deliver SBCC activities.

Action point for UNICEF:

- Development of SOPs - UNICEF should develop SOPs for the SBCC cells at state and district level which will work as principle guiding documents for state department communications officers to effectively carry out SBCC activities after the withdrawal of UNICEF support post the next programmatic cycle. The SOPs should cover, but not be limited to, the following topics-

1. Financial resource management
2. Human resource management
3. Methods of evidence generation
4. Monitoring and evaluation framework
5. Community engagement
6. Inclusion of gender and equity lens in SBCC programming

Recommendation 4:

Adopt a systematic approach to human resource management	Responsible Organization: Department under which cell is housed (primary role) and UNICEF (secondary role)
Priority: High	SBC SS indicator: Workforce
Time frame: 12-18 months	Recommendation for category: I, II and III

Rationale:

- It was highlighted that there was shortfall in availability of human resources (Relevance KEQ 2 and 3, except Telangana) corresponding to required positions due to vacancies at all three levels (state, district and block level). This hinders effective service delivery of SBCC activities and in engagement with community members.
- Further, it was also found that the capacity of human resources at all three levels in cells of category 1 and 2 to focus on communications activities is limited as priority is given to the governance and programmatic aspects of their professional role. (Efficiency KEQ 2)
- The human resources at all three levels (state, district and block level) across the six states were at differing levels of competency in regard to knowledge and skills relating to communications and social behavior change. There is a need to ensure uniform level of training and capacity development across all six states to ensure effective service delivery of SBCC activities.

Action points for State:

- The respective state government departments under which the cell is housed should increase advertising of vacant positions at state, district and block level and ensure active hiring to reduce vacancy of said positions.
- To ensure that the human resources at all three levels in cells of categories 1 and 2 give equal priority to communications activities and programmatic aspects of their professional role, the state government departments under which the cell is housed should clearly highlight responsibilities related to communications in the job description advertisement. This would decrease the likelihood of these human resources feeling that communications activities are additional responsibilities as compared to programmatic aspects being primary.
- Competency mapping - The respective state government departments under which the cell is housed across all six states should conduct a competency mapping exercise in which the current competencies of those human resource who are engaged in communications activities should be mapped against a cadre wise pre-selected list of required communications related competencies. This mapping would act as a needs assessment exercise from which the level of capacity development required can be determined for human resource who are engaged in communications activities within the state.
- Capacity development - The delivery of the capacity development modules and its regular refresher should be taken up by the respective state government departments under which the cell is housed. The conduction should be done by a communications expert hired by the state department. This person should also be trained in workshopping and facilitation to ensure an engaging delivery. This facilitator should work in tandem with the UNICEF consultant placed at the state level.

Action points for UNICEF:

- Development of capacity development framework - UNICEF has the advantage of having subject and technical expertise in methods of SBCC as compared to the state departments which have not fully

transitioned from IEC to SBCC. Hence, UNICEF should develop a capacity development framework for the state departments who in turn would be responsible of the conduction and delivery of the same. The framework should provide cadre wise lists of desired knowledge and skills to effectively delivery SBCC activities and meaningfully engage in SBCC programming. Further, the framework should indicate the level of capacity development required as per level of presence/absence of the desired knowledge and skills as assessed in the workforce.

- Development of training modules - Basis the capacity development framework; UNICEF should develop training modules which would be differentiated for cadres of human resource engaged in communications activities. Further, the modules can be differentiated on the basis of presence of desired knowledge and skills assessed in the workforce.

Recommendation 5:

Adopt a systematic approach for evidence based SBCC planning	Responsible Organization: UNICEF
Priority: Medium	SBC SS indicator: Information
Time frame: 12 - 24 months	Recommendation for category: I, II and III

Rationale:

- As noted in the effectiveness KEQ 1, the cells in category I and II, and III (WB) have not been able to focus on evidence generation to develop evidence-based communication initiatives due to lack of training and mentorship in required methodologies and HR constraints. The evaluation found instances of evidence-based planning in category III (Telangana) where the communication strategies developed by the CRU are based on secondary data sources.
- There is a need for planning, designing and implementation of the SBCC strategies, taking due account of local culture, practices and sensibilities
- As noted in the effectiveness KEQ 7.6, the cell in category I, no evidence of knowledge sharing with allied, collaborating departments and development agencies / NGOs was found in the state. Similarly, across other categories knowledge sharing was not seen as the priority and was need based as in case of COVID -19 pandemic.
- Similarly, it was found during the evaluation, effectiveness KEQ 7.5, there was little focus on documentation and analysis, and monitoring and evaluation of the SBCC activities beyond the general administrative documentation. This affects SBCC activities due to which strategic thinking and decision making regarding in SBCC programming are negatively affected.

Action points for UNICEF:

Evidence Generation

- UNICEF through its state level consultant with domain expertise in social behavior change and communication should provide technical expertise in designing appropriate implementation mechanisms for SBCC design and maintain social and behavioral sustainability by planning SBCC by consideration of changing social and behavioral norms and ensuring the needs of varied communities making it gender responsive and equity focused.
- To design socially relevant implementation mechanisms for SBCC activities, the state level consultant should work in close collaboration with district level officials through monthly meetings. The district levels officials should provide field level insights on the needs of community members and community representatives.
- Similarly, identification of key stakeholder groups (primary – those who are directly addressed, secondary- those who influence primary stakeholders and tertiary- directly or indirectly help or hinder the behaviors of other stakeholders) is crucial for SBCC planning. UNICEF consultant should work in coherence with the district officials to get insights about the social norms and needs of the community and in identification of key stakeholders

- Pre-testing of SBCC material – UNICEF through its state level consultant in collaboration with the department the cell is housed under should conduct the pre-testing of SBCC material to know if the intended audience understands the key messages before finalizing the SBCC material. UNICEF should develop a comprehensive guide for pretesting based on the measures – A) Comprehension – clarity of content, B) Attractiveness C) Acceptance – to the community based on socio-cultural norms, D) Involvement E) Call to action*. Based on the five criteria a tool to be developed for testing of SBCC material through FLWs in the districts where UNICEF has presence.
- Pre-testing (data collection) can be carried out by FLWs during the community meetings like VHND. The inputs from the community should be conveyed by the district to the state cell. Finally, by incorporation of the inputs, SBCC material needs to be modified.

Knowledge Management

- UNICEF to institutionalize knowledge management (documentation of good practices, learnings, knowledge sharing) into the overall SBCC planning.
- Good practices and lessons learned can be shared across the SBCC cells by publication of knowledge products on yearly basis. Documentation of case studies of successes and challenges and creation of documentation of good practices facilitate knowledge sharing among the state cells.
- The SBCC alliance can facilitate an annual conclave for innovative practices in SBCC with participation from MOHFW, state SBCC cells, development partners and other relevant stakeholders which will act as a platform for knowledge sharing and cross learning.

Recommendation 6:

Ensuring dedicated financial resources for SBCC cells	Responsible Organization: Department under which cell is housed in states and UNICEF
Priority: High	SBC SS indicator: Finance
Time frame: 6 – 12 months	Recommendation for category: I, II and III

Rationale:

During the evaluation, under evaluation criteria of efficiency KEQ 1, it was noted that:

- state cell of Assam (category 1) had sufficient funds for certain activities like, RI, a state priority, whereas there were limited financial resources for other SBCC activities beyond RI.
- Although, states under category 2 were housed health department, but the state, and district cells had to depend upon financial resources provided by various national programs who collaborated with the cell for SBCC activities. Cell's efficiency curtailed due to non-availability of earmarked funding for SBCC activities.
- CRU (state cell for Telangana) was able to make efficient and effective contribution towards SBCC activities of the state because it was involved in financial planning for the cell. The annual budget for CRU was based upon the strategic plan developed by CRU for the upcoming year, which led to optimum utilization of allocated monetary resources.
- Additionally, under evaluation criteria of relevance KEQ 3, it was highlighted in all three categories that there is a major limitation in the state cell's ability to respond to the needs of community members and representatives due to resource and budget constraints.
- Evaluation findings suggest that SBCC cells require independent financial resources for efficient functioning. Efficiency and effectiveness in state cells of states belonging to category 1 and 2 was affected by financial resource constraints as well. Whereas CRU's financial management system has positively contributed towards efficient functioning of the cell.

- It is proposed that the state cell should be given autonomy in deciding its financial resource requirement and utilization mechanism. State governments should be shouldered with the responsibility of providing desired financial resources to cell.

Action points for State:

- State governments should provide cells with opportunities to have an increased agency for conducting independent financial planning and spending the budget on SBCC activities.
- States should streamline financial management of the SBCC cells and invite annual planning from the state SBCC cell (in consultation with district cells) and include them in the overall departmental budget of the state to ensure sustainable availability of funds for SBCC activities.

Action point for UNICEF:

- UNICEF should advocate for provisioning of financial resources for SBCC activities in national programs and state interventions.

Recommendation 7:

Development of SBCC cell repository	Responsible Organization: Department under which cell is housed in states
Priority: Medium	SBC SS indicator: Information
Time frame: 6 – 12 months	Recommendation for category: I, II and III

Rationale:

- It was found that it was easier for human resources engaged in communications activities to implement IEC and SBCC activities when they had access to readily available SBCC material (Effectiveness KEQ 4, Category 2).
- For example, In UP, communication officials were able to reach out to the district SBCC cell of Balarampur, when in need to access any communication material across aspects of health messaging. The cell had a large reserve of various communication material (posters, pamphlets, flipbooks, videos, etc.) developed by center, state and other development partners. Another example is the district level cells in Mayurbhanj and Puri (Category 2) which served a similar role of a repository for all communications material developed for the district.
- The repository should include material which has been contextualized to ensure higher relevance of the material to local needs and norms. For example, in Telangana (Category 3), the CRU adapts national level IEC material into regionally contextual material which includes translation into local languages and changing of imagery.

Action points for State:

- One stop repository of SBCC material: The state SBCC cell should be developed as a one stop repository of SBCC material for accessing and use by programme officials and frontline health workers. At the district level the repository should be physical, whilst at the state level, the repository should be physical and online to ensure greater accessibility to district level officials and frontline health workers.
- The repository should contain SBCC material including, but not limited to leaflets, posters, banners, written content on various common topics of public health, flipbooks, charts, designs for wall painting, audio-visual material and other campaign material which are arranged under thematic rubrics. The knowledge resources should be available in English, Hindi and regional languages.
- The state instated graphic designer and translator should focus on contextualization of SBCC material to local languages and dialects, culturally relevant imagery and social norms. This will enrich quality of the material stored in the repository.

Recommendation 8:

Ensuring monitoring and evaluation of SBCC cells	Responsible Organization: Department under which cell is housed in states and UNICEF
Priority: High	SBC SS indicator: Leadership and Governance, Workforce, Service Delivery and Information
Time frame: 6 – 12 months	Recommendation for category: I, II and III

Rationale:

During the evaluation, under evaluation criteria of effectiveness KEQ 6, it was noted that:

- Across the categories, monitoring and evaluation of the SBCC cell's activities was not conducted because there was no mandate and framework for M&E. The cells also lacked either financial resources or human resources (in terms of both people as well as required skill set) to conduct M&E activities.
- Additionally, inconsistency in effectiveness of the cell in regard to training, documentation etc. could also have been reduced, if there had been a monitoring and evaluation mandate in the programming of the cells.
- A system for monitoring and evaluation of SBCC activities is critical to ensure regular information flow on performance of activities, deviation of the cell from objectives and overall performance. It is recommended that UNICEF should advocate and provide technical support to states for investing in development of monitoring and evaluation framework.

Action points for State:

- The state SBCC cell should include monitoring and evaluation, as crucial pillar of the SBCC cell for the state. For which the state should instate a domain expert for M and E who can guide other officials for ensuring regular monitoring of SBCC activities in the state.
- The state should conduct periodic monitoring and evaluation of the activities of SBCC activities through field visits, community observation etc.
- The state should invest in leveraging technology (dashboard, apps etc.) to monitor SBCC activities.

Action points for UNICEF:

- UNICEF should design a sustainable monitoring and evaluation framework involving input and process indicators, like availability of SBCC strategy, percentage of fund utilization, number of training, trainee/trainer feedback etc., for states to follow and adopt in long run.
- UNICEF should also act an independent evaluator of the performance of the cells across the states.
- UNICEF should train and build capacity of the state officials to conduct monitoring and evaluation activities independently.

Recommendation 9:

Gender and Equity - SBCC interventions should be more explicit and strategically integrated to gender equity	Responsible Organization: Respective State Governments with UNICEF's support
Priority: High	SBC SS indicator: Across all Indicators
Time frame: 6 – 12 months	Recommendations for Category: I, II, and III

Rationale:

- From the six evaluated states, it was found that although gender and equity are cross-cutting and a critical component of the SBCC, in practice, the Cells in the states have not yet mainstreamed gender equity, and neither are they gender-cognizant while designing SBCC activities, as was noted under relevance and effectiveness across all categories. WASH services and RMNCH+A has given attention to hygiene and sanitation, nutrition and women's health.
- However, SBCC activities are largely limited to comprehensive health strategy under RMNCH+A with priority intervention for nutrition, IFA supplementation, and so on. Wide gaps persist in fundamental areas of adolescent sexual and reproductive health (SRH), information and counseling, and in early marriage, teen pregnancy, school dropout of girls, and even male child preference in some states. In this context, the Rashtriya Kishor Swastha Abhijan and ARSH (national programmes for adolescent sexual and reproductive health (SRH) and holistic development) were not featured, neither mentioned by the states during the consultations).
- As was found across the six evaluated states, there are no (in-depth) gender equity integrated SBCC activities for health for women and men, and adolescent boys and girls. There are no participatory studies related to sexual and reproductive health. Therefore, SBCC activities should emphasize the importance of health priorities but more comprehensively that intersects with related fundamental social issues, central to gender and equity.
- Furthermore, it was found that the departments are not yet gender-focused, and largely do not have the necessary skills or level of knowledge to initiate gender and equity programmes, except for Odisha which has a relatively better equitable approach to SBCC planning.
- The evaluation further found that a comprehensive Gender and Equity approach was missing at the state level SBCC agenda, while no specific gender-focused programmes are part of the district cell's SBCC activities. A gender-transformative approach is thus a critical requirement to address social norms, practices in diverse social and economic contexts by the state and district cells, and assessing areas that need gender-based intervention, such as those in LODOR and other vulnerable communities. In view of this, UNICEF should support to the states in creating an enabling environment that is gender responsive on priority within its next programmatic cycle/ as a part of its transition strategy. Presence of a 'Gender and Equity aware' program staff will enable assessment of contexts and consciously address existing gaps, gender constraints, and so on within SBCC.
- We gather that UNICEF's support and interventions will be imperative both at state and district levels to initiate a dialogue and consolidate the SBCC agenda on mainstreaming Gender and Equity more thematically within the Cells. To ensure relevant, focused and effective delivery of the SBCC objectives, UNICEF should support state and district cells on research-based inputs on gender responsiveness and inclusivity and create the initial dialogue between states and districts for a Gender Equality Continuum.

Gender and Equity	Action points for State	Action points for UNICEF
Integrate and mainstream gender into the SBCC agenda	• States should design more gender responsive SBCC programs and enhance dialogue of men and women to ensure their active participation in discussions on family planning, mindsets, and gender roles • Conduction of needs assessment with inputs from FLWs on prevalent social and SRH-related issues • Development of differentiated district-level plans based on needs of communities is a critical need.	• UNICEF should engage with states in order to give impetus to the assessment • UNICEF should extend support to each state in the research inputs with frameworks, while the Cells can conduct the needs assessments based on socio-economic and cultural contexts in their respective states
Social Mobilization Committee (or Group) for gender-inclusive and gender-responsive	• States to Increase gender-inclusive and gender-responsive participatory studies as a core aspect for SBCC Cells for improved social behaviour outcomes	• UNICEF should design the methodologies and framework, and extend support by sharing inputs on the method of such studies for optimal

participatory studies	• States must undertake the responsibility of conducting the studies and allocating human and financial resources • A Social Mobilization Group for each state is crucial for initiating the study, as the state does not have a coordinating team for carrying out such studies, and moreover, this can eventually increase gender equitable demand generation related activities.	• impact on the SBCC's gender and equity outcomes • UNICEF should support states to create a social mobilization group that is built on socio-cultural and economic contexts of population groups, and settlements, need-specific gender-responsive issues
Identify and map stakeholders for focus-driven gender and equity partnerships within SBCC	• States must Leverage partnerships with key (established) partners and stakeholders focused on gender equality NGOs and development agencies must be involved with the state and district cells • States should identify relevant stakeholders both geographically and thematically to gauge the most relevant organizations	• UNICEF should support the mapping exercise and appraise identified stakeholders. • It may be useful for states to receive feedback from UNICEF on the mapping and the stakeholders identified
Design communication materials that are gender-sensitive to mainstream gender equity	• States to Increase effectiveness and inclusivity through communication on gender sensitization for adolescent youth, men and women health and hygiene, SRH • Mainstream health information communication to a more nuanced communication-strategy that will address issues taboos around gender-related topics	• UNICEF must support (at least in the initial phase) to help states with a blueprint for the communication materials that must be gender-cognizant and emphasizes on the need for equity (as this requires a carefully planned, gender-sensitive and gender-inclusive agenda that UNICEF can provide) • UNICEF can appraise the communication materials and share their inputs with the states
Leverage UNICEF's capacity development framework for capacity building of the States	• Respective departments under which the Cells are housed, must leverage UNICEF's capacity development framework, to strengthen capacity • States would be responsible for the conduction and delivery of the SBCC activities • These will progressively expedite the gender and equity dimension in the transition from IEC to SBCC • A refresher's course needed for the Cell, trainers, and FLWs officers	• Augment UNICEF staff capacity on gender and other equality and inclusion-related concepts and programming strategies • UNICEF having the advantage of expertise in both social issues and technical expertise, should support states in formulating the research inputs and guide in building capacity, as this requires thorough research and extensive baseline studies • UNICEF should appraise (at least in the initial phase) the refresher's course as this would determine the quality and

**Training of SBCC
officers and FLWs
in Gender
sensitization and
gender
responsiveness**

- States can conduct SBCC through oral presentations, and interactive plenary sessions for SBCC Cells on the issues (but not limited to):
 - Social status, gender roles, and role of men and women in household decisions, employment, education and age during marriage, childcare, family planning and health, access to employment, education and healthcare
 - Attitudes of adolescent youths, men, and women toward Gender based Violence (GBV) (dominance, power dynamics, etc)
 - Status of women and girls in physically work household responsibilities and reproduction
- UNICEF to support the Cells in orienting SBCC officers, trainers and FLWs on the issues mentioned (under actions for state)
- UNICEF should support states by sharing their inputs (or designing) the oral presentations that will eventually be taken over by state cells to be carried forward
- UNICEF must also support actively in the interactive plenary sessions during the initial phase that state cells would eventually carry out

Appendix

A.1. Theory of Change (ToC)

Categorization of themes for revised ToC development						
Sample States	Category 1		Category 2		State color code	
	SBCC Cell with state level institutional set-up	SBCC Cell with state and district level institutional set-up	SBCC Cell housed under health dept.	SBCC Cell housed under 'Other' departments		
Assam					Assam	
West Bengal					West Bengal	
Odisha					Odisha	
Uttar Pradesh (UP)					Uttar Pradesh	
Telangana					Telangana	
Madhya Pradesh (MP)					Madhya Pradesh	
Rationale for the above state wise categorization						
Category 1: In this categorization, SBCC Cells have been grouped as per their existent institutional structure and presence across state and district levels. The two layers explored are one where SBCC Cells are structured with a state level institutional set-up, and the second where the SBCC Cells are structured at the state+ district level institutional set-up. We have mapped Assam and Telangana the former layer and MP West Bengal, Odisha and UP in the latter layer. Please note that while West Bengal has been mapped in the sub-category of SBCC Cells structured at the state+ district level institutional set-up, we understand that in West Bengal, SBCC cells are located with a district level institutional set-up only.						
Category 2: In this categorization, SBCC Cells have been grouped as per the location of where SBCC Cell(s) are housed. The two layers explored are one, where SBCC Cells are housed under health department and the second where the SBCC Cells are housed under 'Other' departments (i.e., line depts or institutional set ups beyond the state health dept and aligned structures). We have mapped Assam, Odisha, UP and MP in the former layer and West Bengal and Telangana in the latter layer.						

Mapping of common characteristics of state SBCC Cells basis category 1 and category 2			
Sample states	SBCC cells housed under the health dept. and based at the state level	SBCC cells housed under the health department, based at state and district level	SBCC cells housed outside the Health Department
Assam			

West Bengal			(West Bengal with district level cells)
Odisha			
Uttar Pradesh			
Telangana			Telangana (without district level cells)
Madhya Pradesh			

In consideration of the review of the common structures mapped across SBCC Cells, the sample states have been clubbed across three layers found below.

1) Assam: Only state among the six sample states where the SBCC Cell is housed under health dept and institutionalized at the state level.

2) MP, Uttar Pradesh, and Odisha: All three SBCC Cells focused on health and cross-sectoral themes and institutionalized at state and district levels.

3) West Bengal and Telangana: Both SBCC Cells are housed outside the health dept and have similar characteristics of SBCC Cell activities focused on health and cross-sectoral themes.

A.2. Evaluation framework

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
RELEVANCE							
1	How relevant and appropriate are the SBCC Cells to the flagship programmes and priorities of the national and state governments?	1.1.1	To what extent is SBCC programming aligned to global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, SDG)?	- Level of alignment SBCC programme plan with national priorities - Number of orders/ policy documents covering SBCC programmes addressing global or national goals	Document review KII Focus group with beneficiaries at the sub-district and community level	1. Desk review of: * Guidelines/ implementation plan/ annual reports/ updates/ etc. of Flagship programmes at state and national level (2017-21)	* Desk review framework * Interview guide
		1.1.2	To what extent is SBCC programming aligned to flagship programmes/ schemes at the national level?	- Number of SBCC programmes aligned to national flagship programmes - Nature of flagship programmes guidelines/ operational SOPs (Standardized Operating Procedures) covering SBCC as a component - Percentage budget allocation on SBCC for implementation of national programmes	Web search	2. National and state level data such as NFHS V 3. Annual Programme Plans for SBCC Cells 4. State sectoral programme guidelines/ documentation / etc. 5. Communication action plan on Covid (if any)	
		1.1.3	To what extent is SBCC programming aligned to flagship programmes/ schemes at the state level?	- Number of SBCC programmes integrated with state schemes or flagship programmes - Nature of SBCC programmes designed to cover state flagship programmes and schemes		* Process documentation * Budget allocation and budget line items to flagship	

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
1 2	How relevant, appropriate, and aligned is the design of the SBCC Cell to the State's established systems and institutions of service delivery?			- Percentage allocation of state flagship programme budgets - Expenditure percentage on SBCC within state flagship programmes		programmes (2017-21) 6. Expenditure statement of flagship programmes * Website on flagship programmes/ thematic departments * Grey literature, particularly covering BCC campaigns and programmes undertaken in the state * MoU with the collaborating departments * IEC/ communication material developed/ prototype * Concept note/ strategy note 7. Organization structure and staff organograms 8. Annual Reports 9. JDs and ToRs of SBC Team 10. Minutes of the meeting/meeting notification Consultation with: * Nodal head of the SBCC Cell * UNICEF state representative for C4D/ SBCC * UNICEF state specific representative for health/ WASH/ child protection * UNICEF state C4D officer and Specialist (Officer in charge when the Cell was pitched to the collaborating department/ MoU signed/ supported to be set-up) IF FEASIBLE * UNICEF ICO C4D officer * Officials from allied state departments Group discussion	
		1.1.4	To what extent did the SBCC Cell support states integrate SBCC programming in health or survival related issues/ programmes?	- Nature of SBCC programmes within the health programmes at the state level - Number of SBCC programmes designed on a range of health and survival at the state level - Nature of health focused SBCC strategies integrated in the SBCC plan since 2017			
		1.2.1	To what extent was the introduction of the SBCC Cell relevant within the existing state-specific institutional set-ups and structures?	- Level of engagement of the SBCC Cell with the existing institutional structure (e.g., state depts, state level institutions) - Number of programmes organized in collaboration with the existing institutional structure by the SBCC Cell - Roles and responsibilities of SBC Staff complementing the roles of State specific institutional structure - Nature of support that the Cell provides to the existing institutional set up at the state level			
		1.2.2	To what extent did the SBCC Cell complement the existing state-specific institutional set-ups and structures?	- Nature of programmes undertaken by the SBCC Cell with the existing Departments - Roles and responsibility sharing between the Cell and the other Departments or platforms\ - Change in the role of the Cell to the existing Departments compared to earlier structure before 2017			
		1.2.3	To what extent and how did UNICEF support existing state-specific communication structures strategies and strengthen SBCC programming/	- Difference on the nature of communication activities before and after the UNICEF support - Nature of support (HR, capacity building, strategies, and tools)			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
			implementation/ outreach?	extended by UNICEF to the concerned Department/s on SBCC between 2017-22 Allocation of budget/resources to the state by UNICEF for the SBCC programming support.		with beneficiaries (FLWs/ CBOs/ community influencers)	
		1.2.4	To what extent did the Cell align with the key state specific influencers for SBCC programming?	- Frequency of meetings with the state influencers and members of SBCC Cell - The role of the state specific influencers in SBCC Cell Structure - Nature of participation of SBCC Cell members in state level programming discussions			
		1.3.1	To what extent did the SBCC cell's programming supported in reaching out equitably to women and men, girls and boys, migrants, tribal, vulnerable groups?	- Number of programmes designed by SBCC Cell specifically targeting women and girls, tribal and other local vulnerable communities - Number and nature of programmes targeting specific geographies comprising of tribal or other vulnerable population between 2017 and 2022 - Nature of messaging on equity in access of service/ programme delivery			
		1.3.2	To what extent were the inherent social equity gaps to meet the health and social services needs and priorities of vulnerable and marginalized communities integrated in the designing of the SBCC Cell's programming and operationalization plan?	- Number of SBCC programmes from 2017 highlighting equity gaps in the state wrt to health and other social services - Elements in the SBCC Operational plan that guides programming to target vulnerable and marginalized communities - Number of studies that has referred to the equity gaps in accessing social services to inform the SBCC programming - View of the officials at the Cell, the state department on the role of the Cell in bridging equity gaps in the state for accessing key services			
	1.3		To what extent does the SBCC Cell respond to the needs of community members and community representatives (FLWs, trainees etc.)				

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
1 . 4	How adaptable is the SBCC Cell to evolving concerns/ areas of focus/ evidence for realignment (including Covid)	1.4.1	To what extent was UNICEF able to support the SBCC cell in adapting/ responding to the evolving needs of the government especially in the covid times?	- Nature of support provided by UNICEF to SBCC Cell: - Financial - volunteers - Technical guidance - Programme design and delivery - Perspectives of the state government officials and the leads of SBCC cell on nature and promptness of the response			
		1.4.2	To what extent did the SBCC Cell's programmes adapt to changes in external environment due to new legislations/ new schemes/ evidence of community behavior/ unrest/etc.?	- Number of changes incorporated in the existing programmes due to COVID or any other legislation/ scheme since 201 - Nature of changes introduced in the programme due to COVID - Perspective of stakeholders in the government on the ability of the			
		1.4.3	To what extent did the SBCC Cell support states integrate SBCC programming in emergency response and preparedness? (Including Covid)	- Nature of BCC strategy embedded wrt emergency response/ preparedness in existing schemes - Nature of programmes organized by the SBCC Cell on emergency response and preparedness for functionaries at the state and district level - Perspectives of state officials and other stakeholders on the support			
		1.4.4	Have the recommendations from the earlier assessments been incorporated in the functioning of the SBCC Cells?	- Examples of changes introduced in the functioning of the SBCC Cell with reference to recommendations of a study / assessment - Views of UNICEF state team and SBCC Cell staff on being able to incorporate earlier suggested recommendations for functioning of SBCC Cell			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
EFFECTIVENESS							
2.1	To what extent has setting up of SBCC Cells strengthened government systems in bringing about more effective and evidence-based communication initiatives?	2.1.1	To what extent have the programmes and initiatives (supported by SBCC Cells) incorporated evidence-based communication programming for behavioral change and demand generation?	- Indicators on behavior change and demand integrated in the existing programme MIS - Number of studies/evidence generation activity covered SBCC programmes	Field observation from FGD with trainees and community members FGD KII	Desk Review of: - Programme document - PIP - Previous years' assessment reports *Budget Review * Expenditure reports *Training and capacity development reports * Training modules *Annual Reports *M&E Reports/ programme MIS *Budget Review on percentage of training and capacity development invested for SBC - Staff and team * Expenditure/ utilization reports * M&E report * MIS reports In-depth interviews with: heads of SBCC Cells, staff members, UNICEF consultants, UNICEF C4D team what are and staff members of collaborating govt line departments SEPIO, DHEIO, DMEIO, BEE, State and District staff members FGD with FHWs In-depth Interview with trainers	Desk review framework In-depth Interview schedule FDG guide KII questionnaire Observation checklist
		2.1.2	To what extent and ways did the SBCC Cells support integration of a <u>gendered</u> approach for evidence generation and SBCC programming?	- Indicators that incorporate gender dimensions - Nature of gender disaggregated data collected through the programme MIS - Perspectives of stakeholders on integration of gender in programme			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		2.1.3	To what extent and ways did the SBCC Cells support integration of an <u>equity and inclusion</u> lens for evidence generation and SBCC programming?	- Nature of data captured by social groups (e.g., SC, ST, OBC, PwD) and other vulnerable categories (e.g., BPL) for SBCC programmes - Equity and inclusion issues in programme, research, and evaluation reports.			
		2.1.4	How effective has Cell and UNICEF been in transforming the state government's people and systems from IEC to BCC?	- Nature of human resource engaged in the state programme for communication activities - Role's responsibilities of the human resource that communication and related activities - Key platforms that have been integrated in the government to focus on BCC - Examples of strategies before and after 2017 that reflect the shift from IEC to BCC Perspective of stakeholders on UNICEF and the SBCC Cell's role in the transformation			
	2. To what extent is evidence-based programming effective in identifying and addressing the challenges to demand generation for RI, health, nutrition and WASH services?	2.2.1	To what extent did evidence-based programming improve targeting focused beneficiary groups including vulnerable/marginalized communities?	- Number of SBCC programmes designed based on demand generation data across programmes - Examples of changes in strategies to focus on a specific target group after 2017 with reference to			
		2.2.2	To what extent did evidence-based programming improve planning and implementing localized communication campaigns that includes focused behavioral triggers for demand generation and service uptake?	- Changes in the planning process after 2017 on SBCC - Number of programme designs that include SBCC strategies targeted towards demand generation in areas with low service uptake - Implementation framework that includes target group specific strategies and messages or focused on a community, target group of geography			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		2.2.3	Did evidence-based programming reduce time towards the desired change vis-a-vis that observed through IEC?	- Percentage of population accessing key services (e.g., ODF, toilet usage, SNP, immunization, institutional delivery) before 2017 and after until 2022 - Changes and accelerated results shared by stakeholders at state level			
		2.2.4	Did evidence-based programming impact the use and diversification of communication channels engaged?	- Nature of communication channels used by themes and messaging - Reports highlighting indicating the change in the strategies and communication channels - Programme materials using media mix, inclusion of diverse target groups			
		2.2.5	Did evidence-based programming improve inclusion and diversity in planning communication campaigns particularly towards women and vulnerable/marginalized communities?	- Number of programmes across themes focusing on women, tribal, and other disadvantaged communities in the state - Programmes/campaigns that integrate the gender and inclusivity messages compared to programmes before 2017 - Perspective of the internal and external stakeholders on the improvement in gender and inclusivity messaging			
		2.2.6	What are some of the recommendations to promote gender mainstreaming in the monitoring of SBCC plans?	- Reports of evaluation and assessment highlighting recommendations on gender mainstreaming - The gaps in the M&E framework/ existing MIS in missing capturing gender disaggregated data - Stakeholder perspectives on the gaps in gender integration and necessary improvements			
	2.3 To what extent has the training equipped stakeholders to plan, implement, monitor behavioral change, and demand	2.3.1	To what extent have the capacities of state level stakeholders been built to integrate, plan, design, implement, and monitor SBCC initiatives?	- Number of capacity building programmes organized for the state level stakeholders on SBCC programming - Percentage of budget allocation and spent on capacity			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
	generation initiatives?			building of state functionaries, - Research or evaluation reports that assess the effectiveness (pre-post 2017) of capacity building programmes undertaken at the community - Insights and perspectives on nature of training provided and feedback received from stakeholders			
		2.3.2	To what extent have the capacities of the different stakeholders at the community interface been built to deliver key messages to the community?	- Number of capacity building programmes organized for the community level stakeholders on BCC, delivery of messages - Percentage of budget allocation and spent on capacity building of community level functionaries - Research or evaluation reports that assess the effectiveness (pre-post 2017) of capacity building programmes undertaken at the community - Insights and perspectives on nature of training provided, and feedback received from stakeholders particularly at the district level and below, such as the trainee and trainer level			
		2.3.3	Have the SBCC cells allocated funds to train its staff members in incorporating a gender-lens to SBCC planning, and to what extent?	- Training/ capacity building modules that cover gender in its design - Percentage of utilization of programme budget using capacity building using a gender lens			
		2.3.4	To what extent has training translated into gender transformative social, and behavior change campaigns?	- Messages integrated in SBCC campaigns/ programmes that challenge gender norms, power imbalances and stereotypes and aims to change behavior, attitudes, and beliefs about gender - Stakeholders' views on the translating gender training translated in the gender transformative BCC campaigns			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		2.3.5	To what extent has the implementation of the SBCC strategy influenced the frontline workers' knowledge, attitudes, and practices towards RMNCH+A, routine immunization, health, nutrition, and WASH services?	- Insights of trainers on adequate integration of gender and social equity as part of SBCC activities and communications. - Examples of changes integrated by the frontline workers message delivery to influence knowledge, behavior and practices of the community - Reports indicating the change in the frontline workers attitudes, behaviors, and practices			
		2.3.6	How effective has the training of frontline workers been in challenging their own biases and social norms?	- Examples of changes integrated by the frontline workers message delivery to influence knowledge, behavior and practices of the community - Reports indicating the change in the frontline workers attitudes, behaviors, and practices - Views of the changes experienced by frontline workers before and after the training.			
	2.4 How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives?	2.4.1	To what extent has the SBCC strategy and associated content contributed to an increased ability for frontline workers to provide quality services?	- Percentage allocation and utilization of budget on frontline worker training on SBCC - Views of frontline workers in improvement of the ability to deliver messages (with ref to tools, aids, and other supportive content)			
		2.4.2	How effective has the communication content been to challenge discriminatory gender and social norms which impede access to developmental services/schemes?	- Number of messages and content that challenge discriminatory behavior, gender, and social norms in the communication programmes - Nature of content and messaging targeting to improve access to schemes by challenging social and gender norms - Feedback of trainers and trainees in facilitating improved access to services by the community through			
		2.4.3	How has information on COVID-19 been incorporated in the communication	Nature of messages incorporated post COVID in communication			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
			strategies of SBCC Cells?	programmes and strategies Number of programmes where SBCC Cell has been in relief initiatives for Covid management			
		2.4.4	How effective was the SBCC Cell in supporting states for Covid-related communication and adoption of desired behaviors?	- Number of programmes undertaken by the SBCC Cell along with the state (various departments at the state level) to share COVID related messages - Change in human resource and responsibilities of staff in SBCC Cell in implementing COVID related communication at the cell post covid 19 - Change budget allocation and utilization in implementing COVID related communication at the cell post covid 19 - Stakeholder perspectives at the state and district level on the effectiveness of SBCC Cell in COVID related communication			
		2.4.5	How effective was the SBCC Cell in knowledge management and knowledge sharing that a larger number of stakeholders (at state/ national/ global levels) are sensitized on the need for SBCC programming?	- Change in proportion of knowledge management products created by SBCC Cells between 2017-21 - Perspectives and insights of stakeholders on the role of SBCC Cell in highlighting the need for SBCC a - Number of opportunities for knowledge sharing such as meetings, webinars, workshops on SBCC at various levels			
	2.5	2.5.1	How effective was the SBCC Cells in intra-departmental and cross-departmental collaboration?	- Number of programmes undertaken by the SBCC Cell with other Cells/ units within the Department (parent department) between 2017-22 - Number of programmes undertaken with other departments, international agencies, and NGOs at the state level - Perspective of allied departments and agencies in collaborating with SBCC			
	To what extent have the SBCC Cells improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates?						

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		2.5.2	How effective was the SBCC Cell in developing partnerships and convergences for joint initiatives and resource augmentation? (With allied state departments and development agencies)	- Number of MoUs with other departments and SBCC Cell for implementing SBCC programmes in the evaluation period - Reports highlighting the collaborative effort between agencies on various SBCC programmes and - Percentage of budget and expenditure of SBCC Cell contributed by other departments - Views of stakeholders (allied depts and SBCC Cell members) on effectiveness of collaborative SBCC programmes			
		2.5.3	How effective was SBCC Cell in partnering with collaborating institutions and like-minded stakeholders for setting up the SBCC Cells at the state level?	- Availability of platforms for allied institutions to contribute to the SBCC Cell - Perspective of SBCC Cell staff the state level and other stakeholders on the role of other agencies in setting up of SBCC Cell			
26	Have the SBCC Cells successfully fulfilled their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships?	2.6.1	To what extent have the SBCC Cells supported evidence generation, monitoring, and analysis?	- Availability of M&E framework or MIS covering SBCC activities and results - Number of periodic reports and documents developed by the state on the results of the SBCC intervention, citing demand or sectoral services - Decisions taken by stakeholders based on evidence generated by the SBCC Cell			
		2.6.2	To what extent have the SBCC Cells supported strategic planning?	- Number of meetings held with the state departments in programme planning for the state - Contribution of the SBCC Cell in state PIP preparation, especially for the nodal Department			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		2.6.3	To what extent have the SBCC Cells supported documentation, knowledge sharing, and built partnerships?	- Number of research reports, documents, publications released annually and periodically by the nodal Dept with support of SBCC Cell - Number of documents, reports, knowledge materials developed in collaboration with allied depts and agencies			
		2.6.4	What modifications can be made to improve the process further?	- Gaps and challenges in facilitating collaboration - Views and suggestions about strengthening the collaboration			
	2. How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system?	2.7.1	To what extent and ways have SBCC Cell supported in strengthening service delivery of health and other allied developmental services?	- Responsibilities of the SBC Staff aligned to support the health department and other allied department as appropriate - Nature of programmes designed and executed by SBCC cell to support health and other departments in the state - Views of SBC Staff at state and district level			
		2.7.2	To what extent and ways have the SBCC Cell supported in strengthening the responsiveness and capability of the workforce for health and other social programmes?	- Percentage of budget allocation and utilization of PIP for SBCC training of workforce in health and other social development areas - Number and themes of training programmes undertaken for workforce at various departments - Nature of programmes undertaken by the SBCC Cell to support the health/ allied dept			
		2.7.3	Have interventions by the SBCC Cell improved service uptake?	- Change in results for uptake of government sponsored programmes - Views and perspectives of SBC Staff at various levels on improvement of service uptake			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				Stakeholder views on SBCC Cells in improving service uptake			
		2.7.4	Did equity and inclusion in beneficiary targeting/outreach improve?	- Change in targeting and programme plan focusing on women and other disadvantaged communities - Reports and existing analysis of programme report on improvement in equity and inclusion in programme delivery			
		2.7.5	What are some of the recommendations to further strengthen the functioning of SBCC Cells to improve health systems delivery at the state/district/block/village levels?	- Recommendations highlighted by reports published during the evaluation period - Perspectives of the staff in ways to improve the functioning of SBCC Cell			
EFFICIENCY							
3.1	Did the SBCC Cells efficiently deliver/implement its activities and function within the planned/allocated monetary resources?	3.1.1	To what extent were the allocated funds to the Cell utilized efficiently as per the proposed budget plan?	- Annual budget/ expenditure (2017-2021) - Percentage allocation of budget for SBCC Cell and break up by themes and programmes in PIPs - Proportion of utilization of funds against dedicated budget components of SBCC programmes	Document Review KII Budget Assessment Focus group	In-depth Interview with: Nodal head SBCC Cells, SBC Staff, UNICEF SBCC consultant, SEPIOs, DIOs, State and District Finance Managers (wherever applicable) Desk review of: •MIS/RAM •Annual Reports •Regional evaluations and studies •M&E Reports; •Budget Review	Desk review framework FGD guide KII questionnaire
		3.1.2	Have the SBCC Cells utilized any funds to train its staff members in incorporating a gender and equity-lens to SBCC planning?	- Percentage allocation and utilization of budget for SBCC Cell training on gender, equity, and inclusion - Change in trend of funding invested on gender sensitization and equity training during 2017-21 - Break-up (amount by components) of training budget for gender, equity and inclusion at the state, district, block, and community level			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
		3.1.3	What are some good implementation practices that have benefited the programme and increased its efficiency?	4. Reports from research, evaluation and process documentation that highlighting increased effectiveness and efficiency 5. Documentation of case studies and success stories 6. Perspective of some of the lessons learnt (what works/ what can work better and how/ what does not work in the gen context)			
		3.1.4	What were the primary challenges and how were they addressed? What are key recommendations/ considerations to overcome the challenge(s) in future?	7. The gaps in implementation of the SBCC programmes at the state level 8. Rationale behind the emerging gaps 9. The research reports that highlight the suggested future course of action for the SBCC Cell			
		3.1.5	What other processes have been incorporated to increase the efficiency of SBCC Cells functioning on monetary resource mobilization/ utilization/ planning?	10. Participation of stakeholders in the planning process 11. Existing reporting and review mechanism of the SBCC programme and resource utilization 12. Perspectives on planning and resource management followed from SBCC Cell staff 13. The bottlenecks that impede the effectiveness of the programme and allocated resources			
		3.2.1	To what extent could the allocated human resources and person-hours adequately implement the functioning and programmes of the Cell?	14. Percentage allocation of HR budget for SBCC Cell and break up by themes and programmes in PIPs 15. Person-hours budgeted and consolidated at the end of the programme cycle 16. Increase or decrease HR budget revisions undertaken at various levels 17. Overlapping and complementary roles and responsibilities allocated to HR on SBCC 18. Perspectives of stakeholders on efficiency of the HR in implementing SBCC activities			
3 2	Did the SBCC Cells efficiently deliver/implement its activities and function within the planned/allocated <u>human resources</u> ?						

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				19. Activities negatively impacted due to absence of human resource			
		3.2.2	To what extent and how were additional human resources mobilized to prevent inefficiencies due to implementation challenges?	20. Number of HR inducted into the SBCC Cell by year 21. Roles and responsibilities of staff engaged compared to the earlier structure 22. Reduced implementation challenges reported by functionaries			
		3.2.3	To what extent have the SBCC Cell's functionaries and leadership oriented and trained for incorporating a gender and equity - lens to SBCC planning?	23. Budget reports highlighting details on fund utilization across programme components such as gender, equity, and inclusion 24. Number of capacity building programmes for SBCC Cell functionaries and leadership 25. The themes covered in the capacity building initiatives			
		3.2.4	To what extent have the expertise and commitment of the allocated UNICEF's HR utilized to deliver the SBCC activities/interventions?	26. The structure of the SBCC Cell and the roles and responsibilities of UNICEF staff at various levels 27. Role of UNICEF staff in conceptualizing and technically supporting programme design and implementation plan undertaken by SBCC Cell 28. Perspective of the government officials in drawing benefits from the UNICEF staff continuing to work on SNCC			
		3.2.5	What strategies and plans have been developed to meet the key challenge of unavailability of trained human	29. Gaps in sanctioned allocation of human resource and currently positioned in SBCC Cells 30. Human resource strategies and plan indicated in Annual			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
			resources for SBCC Cells?	Plan documents of the state 31. SBCC Cell staff and the nodal government departments to address gaps of trained human resource in SBCC 32. Number of SBCC training organized every year to train the staff at various levels from government depts or allied agencies			
		3.2.6	What were the primary challenges and how were they addressed? What are key recommendations/ considerations to overcome the challenge(s) in future?	33. Gaps identified in programme delivery due to absence of trained human resource at various level 34. The research reports that highlight the key recommendations to enhance the efficiency of the human resource			
		3.2.7	What other processes have been incorporated to increase the efficiency of SBCC Cells functioning on human resource mobilization/ utilization/ planning?	35. Number of planning meetings organized for planning and programme design for SBCC Cell 36. Participants representing a range of stakeholders in the planning process 37. Periodic mechanism to review the status of human resource, planning and utilization 38. Budget reports highlighting process of fund utilization adopted for human resource, planning and implementation during the project years 39. Perspective on mechanism of financial review and management across programme verticals			
		3.3.1	To what extent have the SBCC cell activities been delivered in a timely manner as planned?	40. Number of programme activities which could not be completed as envisaged 41. Annual Reports indicating the timeline achievement of the planned activities 42. Utilization percentage annually of SBCC budget 43. Perspective on challenges faced and			
3.3	Did the SBCC Cell sufficiently deliver/implement its activities and function within the planned/allocated <u>timelines</u> ?						

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				mitigation mechanism undertaken at various levels to support efficient HR fund utilization and resource management			
		3.3.2	To what extent were the planned timelines for SBCC Cell's programme implementation realistic and contextualized to the local scenario?	44. Proportion of SBCC programme activities completed as per the planned time and budget highlighted in PIPs 45. Stakeholder views on the rationale for timely or delayed implementation of programmes 46. Perspective of the staff on ability to manage the plan and timelines, challenges of timely implementation and localization			
		3.3.3	How efficiently did the SBCC cells respond to the equity-based challenges to implement activities within the time frame set in the plan?	47. Number of equity-based activities completed in the desired time 48. Barriers in executing such activities 49. Stakeholder views on the rationale for timely or delayed implementation of programmes			
		3.3.4	What were the primary challenges and how were they addressed? What are key recommendations/ considerations to overcome the challenge(s) in future?	50. Annual reports of programme and SBCC Cell indicate the challenges faced during the year in implementing programmes 51. Stakeholder view on the mechanism to address the challenges			
		3.3.5	What other processes have been incorporated to increase the efficiency of SBCC Cells functioning on time management/ planning/ monitoring?	52. Existing programme management framework or MIS 53. Number of review meetings held periodically 54. Agenda discussed in the review meetings 55. Staff and government officials' perspective on managing efficiency of the SBCC Cell			
SUSTAINABILITY							

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
4.1	To what extent have the different state departments integrated SBCC programming into its planning and implementation of initiatives?	4.1.1	To what extent and ways have SBCC programming been Integrated within state planning for health and other social development programmes?	56. The dedicated components on SBCC within the key sectoral PIP 57. Presence of a dedicated cadre of resource persons for the functioning of SBCC in the government department at the state level	Document Review KII Focus groups	In-depth Interview with: heads of the SBCC Cells, SEPIOs, DIOs, State and District Finance Managers, staff members and UNICEF consultants (wherever applicable) KRPs	Desk Review framework Consultation guide
		4.1.2	To what extent and ways has SBCC programming been Integrated within the budgeting cycle of the state/district/ block?	58. Allocation of budget for SBCC from health and other Depts. at the state, district and below 59. Expenditure of the SBCC Budget periodically 60. Annual reports of the state department reflect the activities of SBCC		Desk review of: • MIS/RAM • Annual Reports • Regional evaluations and studies • M&E Reports; • Budget Review * PIP	
4.2	Does the government (at the state and district-level) have the capacity to successfully carry out the activities of the SBCC Cells independently?	4.2.1	To what extent are human resources with requisite skills deployed to the SBCC Cell as per the proposed plans of implementation?	61. The HR structure of SBCC Cell staff and the allocated roles and responsibilities 62. ToR of the staff at each level and their actual qualification of the SBCC Cell staff at various levels 63. The gaps in the existing human resource impacting programme delivery 64. Number of training at staff at each level has experienced			
		4.2.2	To what extent is a governance mechanism put into place to oversee and guide the functions of the SBCC Cell?	1. Documented governance structure, reporting and Quality assurance mechanism of the SBCC Cell 2. Availability of platform/structure/ scheduled activity for coordination between implementing stakeholders of SBCC Cell. 3. Availability of grievance redressal mechanism 4. Roles and responsibilities of staff of SBCC Cell at various levels (As applicable)			
		4.2.3	To what extent is an ownership and sense of purpose (for the Cell) built among the leaders of the institute/ collaborating organization that houses the SBCC Cell?	5. MoU or Concept note or MoM of collaborative roles and responsibility of SBCC Cell members within the nodal Dept (collaborating organization that house the cell) and otherwise 6. Vision articulated for the SBCC Cell by the			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				leaders (senior government staff responsible for SBCC Cell) 7. Stakeholder view (internal within SBCC Cell and outside) on the sense of purpose and ownership amongst the leaders			
4.3	Do the Cells and their implementation structures (at the state and district-level) have adequate financial allocations and partnerships to successfully carry out the activities of the SBCC Cells independently?	4.3.1	Does the Cell receive an allocated budget? How much percentage of the Cell's functions are adequately covered under the budget received?	8. Percentage allocation of budget for SBCC Cell and break up by themes and programmes in PIPs by project years 9. Proportion of utilization of funds against dedicated budget components of SBCC programmes from 2017 10. Perspectives on the adequacy of the allocated to SBCC Cell aligned to its vision and plan			
		4.3.2	To what extent does the SBCC Cell mobilize additional monetary resources for its programmes through convergence with inter and intra departments?	11. Proportion of funds for SBCC Cell from within the Dept it is housed and from other departments 12. Budget reports highlighting the overall budget of the Cell (since 2017) process of fund utilization adopted to ensure achievement of programme implementation requirements 13. Documentation of process followed for fundraising across convergence platforms, other agencies across Depts. 14. Number of events that had full or partial contribution of other agency/ dept for undertaking SBCC activities (other than where it is housed).			
		4.3.3	To what extent does the SBCC Cell mobilize additional monetary resources for its programmes through partnerships with developmental agencies and corporate organizations?	15. Proportion of funds for SBCC Cell from other development agencies, or Corporate and other Depts 16. Budget reports highlighting process of fund utilization adopted to ensure achievement of programme implementation 17. Process followed for fundraising			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				across joint platforms with other development agencies and corporate			
	What are some of the recommendations to further strengthen the functioning of SBCC Cells to improve health systems delivery at the state/district/block /village levels?	4.4.1	How can the Cell strengthen its support to the programme teams (health and other allied department) for planning improved and inclusive outreach	18. Existing research and evaluation reports that indicate the need for enhanced support from the cell 19. Gaps in the demand for services by specific target groups, social groups, and gender 20. Reports highlighting gaps in attitude, behavior, and practices in accessing services as input to future strategies 21. Stakeholder perspective on future roles and responsibilities with respect to improved inter departmental coordination and partnership to improve services	Observation from FGD conducted with trainees and community members FGD KII	Community FLWs State District and block level programme functionaries SBCC Cell's staff	Field observation Checklist FGD guide KII tool
		4.4.2	How can the Cell further strengthen quality of service delivery and convincing power of the FLWs for improved demand generation	22. Reports and data highlighting gaps in service delivery and recommendations to improve the same 23. Stakeholder views on improving quality of service at various levels 24. Areas identified by the workforce at various level for capacity enhancement for improved service delivery and demand generation 25. Stakeholder views from allied departments and other agencies on mechanism to improve demand generation through SBCC Cell			
COHERENCE							
5.1	How do the activities/interventions of the SBCC Cells interlink and synergize with the existing/other programme/initiatives by other relevant actors	5.1.1	Have the SBCC Cells leveraged partnerships with the external actors/stakeholders to improve the demand of health/ RI/ WASH/ nutrition services for vulnerable communities?	26. Number and nature of programme developed and/ or implemented in synergy with departments of state govt. 27. Number of new flagship programmes (launched between 2017-21) with specific focus on behavior change or demand generation in	Document Review KII FGD	Desk Review: Annual Reports Regional/global evaluations/studies Evaluations/studies Programme document Baseline studies/assessments for programme design, the annual and strategic Plan of UNICEF ICO and	Desk Review framework KII schedule

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
	(NGOs and development partners and other sector players)?			approach/strategy/activities 28. Number of training and the themes undertaken by SBCC Cell for functionaries to deliver improved services/ govt programmes at the state level 29. Number of sensitization programmes or workshops and the themes undertaken by SBCC Cell for functionaries to deliver improved services/ govt programmes at the state level 30. Number of partnership and convergence initiative supported by SBCC Cell 31. Budget allocated by other Departments for SBCC		Sector Specific plans ToR and JD of SBC Staff Interviews with: Heads of SBCC Cells, SEPIOs, DIOs; officials from different state departments (Health, Rural Development, Women and Child Development, WASH, PHED) and development agencies; SBC Staff members; strategic partners	
		5.1.2	How well have these leverage partnerships (joint interventions/programmes) undertaken between SBCC Cell and other development agencies in the states?	32. Number of joint programmes between the Cell and govt. Departments at the national and state level 33. Nature of partnership (themes and community covered) and role of the SBCC Cell and any other government and private agencies			
		5.1.3	Is there evidence of SBCC Cells having a comparative advantage vis-à-vis these external stakeholders for these interventions/programmes?	34. Number of joint programmes between the Cell and other development agencies (other UN agencies, INGOs, bilateral orgs, NGOs, etc.) 35. Nature of partnership (themes and community covered) and role of the SBCC Cell and development agencies (govt, NGO/ INGO, private)			
		5.1.4	Are there any critical gaps to this approach to collaborate/synergize with these external stakeholders?	36. Examples of effectiveness of SBCC Cell's intervention through other research reports 37. Stakeholders' views on the comparative advantage of SBCC Cell in demand generation as compared to other programmes			

#	KEQ	#	SEQ	Indicators	Means of verification	Data sources/ stakeholders	Data collection tool
				38. Skill set and services identified as unique to SBCC Cell's offering			
5.2	How do the SBCC Cells interventions align/synergize/fit with other interventions being carried out by the state for improving health systems?	5.2.1	In what ways and to what extent has the state government been able to effectively collaborate and coordinate internally the activities of the SBCC cells with the other state government interventions?	39. Participation of various units in the planning meetings/ discussions 40. Number of collaborative meetings held in year along with other functions and units of the Dept (Health/ other anchoring Dept)			
		5.2.2	Are there any critical gaps to this approach?	41. Programmes implemented by the units/ cells within the health department (As applicable) annually 42. Roles and responsibilities indicated of the units/ cells in the Annual Report of the Department			

A.3. List of Documents reviewed

Programme Guiding Documents
- UNICEF C4D interventions: Presentation - State Case Study: West Bengal - State Case Study: Odisha - State Case Study: Madhya Pradesh - State Case Study: Assam - West Bengal: District HR involvement in IEC / SBCC - West Bengal: Kannyashree Parkalpa, a brand to leverage for dengue prevention - UNICEF Report on Prevention of Child Marriage-2019 Purulia - SBCC Cell Malda Action Plan - SBCC in Purulia - Village Information Centers: Bottom-up approach for SBCC in an Integrated District: Purulia - TARANG SBCC Training Report- Social and Behavior Change Communication (SBCC) Training for Demand Generation on Routine Immunization - Report of State Level Communication Orientation & Planning Workshop On Measles & Rubella and Japanese Encephalitis Vaccination Campaign - CoE Guideline
Reports
Good Practices for Health Systems Strengthening: Shaping winning strategies for changing RMNCHA+A behaviors

Programme Guiding Documents	
•	C4D Progress Report 2018
•	C4D Progress Report 2020
•	National C4D Report of UNICEF 2013-2017 Country Programme Cycle
•	Report on UNICEF internal evaluation of SBCC cells
•	C4D Case Study Compendium
•	CoE Inauguration report
•	BRIDGE training report
IEC material	

A.4. List of stakeholders interviewed

A.5. Status of stakeholder consultations completed in Uttar Pradesh

Level	#	Stakeholder Category		Uttar Pradesh		# Interactions completed on field	Notes
				# Interactions as per IR			
State		Nodal Leadership, SBCC Cell		1	KII	0/1 Not completed	The stakeholder was unavailable at the time of the scheduled date of consultation.
		Functionaries of the SBCC Cell		1	KII	1/1 completed	-
		UNICEF Consultant (loaned staff)		-			
		Nodal government Department		-			
		Collaborating government official		-			
		Allied department officials		2	KII	1/2 completed	One of the two allied dept officials identified was found to be better fitted in the category of nodal leadership as they currently head the state IEC cell
		UNICEF state officials: SBC Specialists		1	KII	1/1 virtual interaction planned	Stakeholder was unavailable on field
		Government officials responsible for the setting up (if different from current officials)		-		1/0 One completed	The person identified as the nodal leadership was found to be better fitted within this category
		UNICEF officials responsible for the setting up (if different from current officials)		-			
		Development agencies with which the SBCC	IDA/ UN allied agency	1	KII	Stakeholder unavailable	Unavailability confirmed by state

		Cell has collaborated					team and recorded on email
		NGO/ research agency/ medical institute/ etc.	1	KII	1/1 completed	-	
Divisional		Divisional SBCC Cell	1	GI	Stakeholder unavailable		Unavailability confirmed by state team and recorded on email
		BCC Nodal Officers	1	KII	1/1 completed	-	
District 2 districts to be covered per state		Head of District SBCC Cell	2	KII	1/2 completed		In one district, stakeholder unavailability confirmed by state team and recorded on email
		SBCC Cell District level 2nd Line Leadership	-				
		Allied department officials	-				
		UNICEF Consultant (loaned staff)	-				
		Officials supporting communication planning at district level	-				
		SBC Coordinator	1	GI	1/1 completed 2/0 KII completed		We were able to conduct GI in one district and two KIIs in the second district hence number of interactions has tripled
		SBC Cadre at district level (Master trainers)	-				
		Communication Nodal Officer	-				
Block 2 blocks per district (4 blocks total)		SBC Cadre at sub-district level (Master trainers) ⁵⁰	2		0/2 FGD		Unavailability confirmed by state team and recorded on email
		IEC resource centre	-				
		SBCC Coordinator	1	GI	1/1 completed	KII	Only one stakeholder
		Trainees of SBCC Cells	2	FGD	2/2 completed		-
		ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO					
		Field Level Observation of Trainers with Trainees	1	Observation	1/1 completed		-

⁵⁰ At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

	Community Members	2	FGD	2/2 completed	-
Total interaction per state (proposed)		20		15 interactions completed 1 virtual interaction planned 1 training observed = 17 interactions completed	
Total interaction sample in ToR		12			

A.6. Status of stakeholder consultations completed in Madhya Pradesh

Level	#	Stakeholder Category	Madhya Pradesh			Notes
			# Interactions as per IR		# Interactions completed on field	
State		Nodal Leadership, SBCC Cell	2	KII	1	Owing to the busy schedule leading to unavailability of the MD, NHM the consultation could not be completed.
		Functionaries of the SBCC Cell	1	GI	1	
		UNICEF Consultant (loaned staff)	2	KII	2	
		Nodal government Department	2	KII	1	Only one of the two proposed stakeholders was available for consultation and the consultation was completed with the same.
		Collaborating government official	1	KII	2	
		Allied department officials	2	KII	1	Only one of the two proposed stakeholders was available for consultation from the relevant allied dept identified and the consultation was completed with the same.
		UNICEF state officials: SBC Specialists	1	KII	1	
		Government officials responsible for the setting up (if different from current officials)	1	KII	1	
		UNICEF officials responsible for the setting up (if different from current officials)	-		-	

Level	#	Stakeholder Category		Madhya Pradesh			
				# Interactions as per IR		# Interactions completed on field	Notes
		Development agencies with which the SBCC Cell has collaborated	IDA/ UN allied agency	2	GI	3	In addition to the identified stakeholder, two more relevant partner organizations were identified, and the consultation was conducted.
			NGO/ research agency/ medical institute/ etc.				
Divisional		Divisional SBCC Cell		NA		NA	
		BCC Nodal Officers		NA		NA	
District	2 districts to be covered per state	Head of District SBCC Cell		2	KII	2	
		SBCC Cell District level 2nd Line Leadership				-	
		Allied department officials		-			
		UNICEF Consultant (loaned staff)		-			
		Officials supporting communication planning at district level		4	KII	2	There is at present one stakeholder available at the district level across both districts for this category.
		SBC Coordinator		-		1	There was a relevant stakeholder identified on field and the consultation was undertaken.
		SBC Cadre at district level (Master trainers)		2	FGD	2	
Block	2 blocks per district	Communication Nodal Officer		-		2	There were relevant stakeholders identified on field and the consultation was undertaken.

Level	#	Stakeholder Category	Madhya Pradesh			
			# Interactions as per IR		# Interactions completed on field	Notes
(4 blocks total)		SBC Cadre at sub-district level (Master trainers) ⁵¹	-			
		IEC resource centre	-			
		SBCC Coordinator	-			
		Trainees of SBCC Cells	2	FGD	2	
		ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO				
		Field Level Observation of Trainers with Trainees	1	Observation	1	
		Community Members	2	FGD	2	
Total interaction per state (proposed)			26		26 interactions completed	
Total interaction sample in ToR			12			

A.7. Status of stakeholder consultations completed in Telangana

Level	#	Stakeholder Category	Telangana			
			# Interactions as per IR		# Interactions completed on field	Notes
State	1.	Nodal Leadership, SBCC Cell	2	GI	1/2 completed	One of the two stakeholders is only available post 22nd
	2.	Functionaries of the SBCC Cell	-		1/0 completed	We were able to identify stakeholder on field and hence have added their interaction
	3.	UNICEF Consultant (loaned staff)	1	KII	1/1 completed	-
	4.	Nodal government Department	1	KII	0/1 completed	Not been able to get an

⁵¹ At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

Level	#	Stakeholder Category		Telangana		# Interactions completed on field	Notes
				# Interactions as per IR			
							appointment To be completed this week
	5.	Collaborating government official		-		1/0 completed	We were able to identify a stakeholder on field and hence have added their interaction
	6.	Allied department officials		2	KII	0/2 completed	Only one stakeholder has been identified within this stakeholder category. Verified by state team and recorded on email Not been able to get an appointment yet. To be completed this week
	7.	UNICEF state officials: SBC Specialists		1	KII	2 completed	We were able to identify two relevant stakeholders within this category and hence the number has doubled
	8.	Government officials responsible for the setting up (if different from current officials)		-			-
	9.	UNICEF officials responsible for the setting up (if different from current officials)		2	GI	1 completed	Only one stakeholder has been identified within this category. Verified by state team and recorded on email
	10.	Development agencies with which the SBCC Cell has collaborated	IDA/ UN allied agency	-			-
			NGO/ research agency/ medical institute/ etc.	-			-
	Divisional	11.	Divisional SBCC Cell		-		

Level	#	Stakeholder Category	Telangana		# Interactions completed on field	Notes
			# Interactions as per IR			
	12.	BCC Nodal Officers				-
District 2 districts to be covered per state	13.	Head of District SBCC Cell	-			-
	14.	SBCC Cell District level 2nd Line Leadership	-			-
	15.	Allied department officials	-			-
	16.	UNICEF Consultant (loaned staff)	-			-
	17.	Officials supporting communication planning at district level	-			-
	18.	SBC Coordinator	-			-
	19.	SBC Cadre at district level (Master trainers)	1	FGD	1/1 Completed	-
Block 2 blocks per district (4 blocks total)	20.	Communication Nodal Officer	-			-
	21.	SBC Cadre at sub-district level (Master trainers) ⁵²	-			-
	22.	IEC resource centre	-			-
	23.	SBCC Coordinator	-			-
	24.	Trainees of SBCC Cells	2	FGD	2/2 Completed	
		ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO				
	25.	Field Level Observation of Trainers with Trainees	1	Observation	1/1 Completed	
26.	Community Members	2	FGD	2/2 Completed		
Total interaction per state (proposed)			15		13 interactions completed	
Total interaction sample in ToR			8			

⁵² At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

A.8. Status of stakeholder consultations completed in Assam

Level	#	Stakeholder Category	Assam			
			# Interactions as per IR		# Interactions completed on field	Notes
State	1.	Nodal Leadership, SBCC Cell	2	KII	1	Relevant stakeholder was not available for consultation on the respective day
	2.	Functionaries of the SBCC Cell	3	KII	1	Relevant stakeholder was not available for consultation on the respective day
	3.	UNICEF Consultant (loaned staff)	2	KII	1	Only one consultation could be completed, the second stakeholder was not available for consultation on the respective day
	4.	Nodal government Department	-		1	Relevant stakeholder was identified on field and hence the interaction has been included
	5.	Collaborating government official	2	KII	1	Only one consultation could be completed, the second stakeholder was not available for consultation on the respective day
	6.	Allied department officials	2	KII		Relevant resource persons were not available and the same has been documented on email
	7.	UNICEF state officials: SBC Specialists	2	KII	1	There is at present only one SBC specialist in the state. This consultation has been completed

Level	#	Stakeholder Category		Assam		
				# Interactions as per IR	# Interactions completed on field	Notes
						with the present position holder
	8.	Government officials responsible for the setting up (if different from current officials)		-		
	9.	UNICEF officials responsible for the setting up (if different from current officials)		-		
		Development agencies with which the SBCC Cell has collaborated	IDA/ UN allied agency	1	KII	One consultation could be completed, the second stakeholder was not available for consultation on the respective day
			NGO/ research agency/ medical institute/ etc.	1	1	
Divisional	10.	Divisional SBCC Cell		-		
	11.	BCC Nodal Officers				
District	12.	Head of District SBCC Cell		-		
2 districts to be covered per state	13.	SBCC Cell District level 2nd Line Leadership		-		The relevant stakeholder was identified on field and the consultation was conducted
		Allied department officials		-		
	14.	UNICEF Consultant (loaned staff)		-		
	15.	Officials supporting communication planning at district level		3	KII	Two out of the total three stakeholder consultations could be completed. The remaining stakeholder is

Level	#	Stakeholder Category	Assam			
			# Interactions as per IR		# Interactions completed on field	Notes
						retired and was unavailable
	16.	SBC Coordinator	-		1	
	17.	SBC Cadre at district level (Master trainers)	2	FGD	2	
Block 2 blocks per district (4 blocks total)	18.	Communication Nodal Officer	-		1	Stakeholders were identified on field and the consultation was conducted
	19.	SBC Cadre at sub-district level (Master trainers) ⁵³	-		1	Stakeholders were identified on field and the consultation was conducted
	20.	IEC resource centre	-			
	21.	SBCC Coordinator	-		1	Stakeholders were identified on field and the consultation was conducted
	22.	Trainees of SBCC Cells	2	FGD	2	
		ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO				
	23.	Field Level Observation of Trainers with Trainees	1	Observation	1	
	24.	Community Members	2	FGD	2	
Total interaction per state (proposed)			25		19 interactions completed	
Total interaction sample in ToR			10			

⁵³ At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

A.9. Status of stakeholder consultations completed in West Bengal

Level	#	Stakeholder Category	West Bengal			
			# Interactions as per IR		# Interactions completed on field	Notes
State	1.	Nodal Leadership, SBCC Cell	-		1	
	2.	Functionaries of the SBCC Cell	2	KII	-	Stakeholders were unresponsive for consultation. Two reminder emails were shared and the evaluation team along with the UNICEF state SBC Specialist also attempted to follow up on phone.
	3.	UNICEF Consultant (loaned staff)	-			
	4.	Nodal government Department	-			
	5.	Collaborating government official	-			
	6.	Allied department officials	-			
	7.	UNICEF state officials: SBC Specialists	1	GI	1	
	8.	Government officials responsible for the setting up (if different from current officials)				
	9.	UNICEF officials responsible for the setting up (if different from current officials)	1	KII	-	Relevant resource person is not available and the same is documented on email
	10.	Development agencies with which the SBCC Cell has collaborated	1	KII	1	virtual consultation
Divisional	11	Divisional SBCC Cell	-			
	12	BCC Nodal Officers	-			
District	13	Head of District SBCC Cell	2	KII	-	
	14	SBCC Cell District level 2nd Line Leadership	2	KII	2	

Level	#	Stakeholder Category	West Bengal			
			# Interactions as per IR		# Interactions completed on field	Notes
2 districts to be covered per state	15	Allied department officials	2	KII	2	
	16	UNICEF Consultant (loaned staff)	2	KII	1	
	17	Officials supporting communication planning at district level	-			
	18	SBC Coordinator	2	KII	2	
	19	SBC Cadre at district level (Master trainers)	2	FGD	2	
Block 2 blocks per district (4 blocks total)	20	Communication Nodal Officer	-			
	21	SBC Cadre at sub-district level (Master trainers) ⁵⁴	-			
	22	IEC resource centre	-			
	23	SBCC Coordinator	-			
	24	Trainees of SBCC Cells	2	FGD	2	
		ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO			1	
	25	Field Level Observation of Trainers with Trainees	1	Observation	1	
	26	Community Members	2	FGD	3	
Total interaction per state (proposed)			22		19 interactions completed on field	
Total interaction sample in ToR			20			

⁵⁴ At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

A.10. Status of stakeholder consultations completed in Odisha

Level	#	Stakeholder Category		Odisha			
				# Interactions as per IR		# Interactions completed on field	Notes
State	•	Nodal Leadership, SBCC Cell		3	KII	3	At the field level, there were additional relevant representatives that were identified and were available to provide inputs. The number of interactions completed is thus additional to the number mapped in IR
	•	Functionaries of the SBCC Cell		1	GI	1	
	•	UNICEF Consultant (loaned staff)		1	KII	1	
	•	Nodal government Department		-		-	
	•	Collaborating government official		2	GI	5	At the field level, there were additional officials identified and were available to provide inputs. The number of interactions completed is thus additional to the number mapped in IR
	•	Allied department officials		2	KII	2	
	•	UNICEF state officials: SBC Specialists		1	GI	1	
	•	Government officials responsible for the setting up (if different from current officials)		1	KII	1	
	•	UNICEF officials responsible for the setting up (if different from current officials)		1	KII	1	Proposed to be conducted virtually post 08 January as the relevant stakeholder is on leave until the mentioned timeline
•	Development agencies with which the SBCC Cell has collaborated	IDA/ UN allied agency	1	KII	3	At the field level, there were additional partner organizations were identified and were available to provide inputs. The number of interactions completed is thus additional to the number mapped in IR	

Level	#	Stakeholder Category		Odisha			
				# Interactions as per IR		# Interactions completed on field	Notes
			NGO/ research agency/ medical institute/ etc.	1	KII		
Divisional	•	Divisional SBCC Cell		-		-	
	•	BCC Nodal Officers		-		-	
District 2 districts to be covered per state	•	Head of District SBCC Cell		2	KII	2	
	•	SBCC Cell District level 2nd Line Leadership		-		-	
	•	Allied department officials		-		-	
	•	UNICEF Consultant (loaned staff)		-		-	
	•	Officials supporting communication planning at district level		3	GI	1 KII	At the field level, there were only one stakeholder available against this category to provide inputs. The number of interactions completed is thus less than the number mapped in IR
	•	SBC Coordinator		-		-	
	•	SBC Cadre at district level (Master trainers)		2	FGD	2	
	•	Communication Nodal Officer		-	-	1 KII	At the field level, there were relevant stakeholders identified against this category and the same were available to provide inputs.
Block 2 blocks per district	•	Communication Nodal Officer		-		2 KII	At the field level, there were relevant stakeholders identified against this category and the same were available to provide inputs.

Level	#	Stakeholder Category	Odisha		
			# Interactions as per IR		Notes
(4 blocks total)	•	SBC Cadre at sub-district level (Master trainers) ⁵⁵	-		-
	•	IEC resource centre	1	GI	1
	•	SBCC Coordinator	2	KII	-
	•	Trainees of SBCC Cells (ASHA/ ANM/ AWW/ LHV/ MO/ ASHA Sahyogini/ CSO/ CBO)	2	FGD	2
	•	Field Level Observation of Trainers with Trainees	1	Observation	1
	•	Community Members	2	FGD	2
Total interaction per state (proposed)			30		32 Interactions completed on field
Total interaction sample in ToR			42		

⁵⁵ At the time of mapping the stakeholder interactions in the inception phase, from the orientation meeting with the respective UNICEF state SBC officials, it was understood that for UP, block level SBCC Coordinators to be considered as master trainers

A.11. Interview checklists/protocols

A.12. Checklist for field enumerators

#	Focused Sections	Agenda to be covered	Completed completed	/Not
	Introduction of the interviewer/moderator & setting the context and purpose of the consultation/discussion	Interviewer/Moderator to provide self-introduction		
		Interviewer/Moderator to provide introduction to the project context and purpose of the consultation		
		Interviewer/Moderator to provide some ice-breaking context with participants		
	Consent protocols	Interviewer/Moderator to narrate the complete oral consent statement and cover all aspects highlighted in the consent statement		
		Interviewer/Moderator to document consent agreement form		
	General instructions on navigating the data collection tools	Interviewer/Moderator to be cognizant of the scheduled time to complete the consultation		
		Interviewer/Moderator to refrain use of any leading questions		
		Interviewer/Moderator to refer and/or utilize all relevant probe areas included in the data collection tool		
		Interviewer/Moderator to cover all questions in the KII and/or all discussion pointers in the FGD guide		
	Specific instructions for conducting FGDs	Moderators to navigate the discussion basis the discussion pointers noted in the tool, observe and remain impartial to the discussion		
		Moderators to provide support to ensure there is participation noted from all attendees of the discussion		
		Moderators to ensure that there is a circular seating arrangement followed to ensure visibility of all members. Moderators to refrain support to seating arrangements that reflects a hierarchical representation of members		
		Moderators to refrain from making any promises or any commitments to the attendees		
		Moderators to thank all attendees of the FGDs and request for feedback on the discussion		

A.13. Study tool – KII

KII with Nodal Leadership, State SBCC Cell

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Nodal Leadership, State SBCC Cell to understand the aspects of the SBCC program that has worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing

community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator”.

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the ‘do no harm principle’ in conducting the discussion**

Before posing the specific questions, the facilitator allows participant to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

Relevance of SBCC Cells-

- Can you please share the key milestones in the evolution of the SBCC Cells at the state level? What was the critical support or guidance that you received from UNICEF to be able to operationalize it? What were most critical activities undertaken by the Cell (2017-20)
- Do you think SBCC Cells are aligned to the flagship programmes and priorities of the national and state governments? Some of the specific areas that we would request you to highlight include its contribution to
 - Global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, SDG).
 - Your state's flagship programmes / schemes and the nature of support to integrate SBCC programming in health or development programmes.
 - The support received in integrating SBCC in emergency response and preparedness (including COVID-19 pandemic)
- Can you please share the structure of the SBCC Cell and how it is embedded in the district's Departmental structure? Please share the roles and responsibilities that the SBCC cell has that complement's the Department's activities.
- How does the SBCC Cell respond to the needs of community members and community representatives (FLWs, trainees etc.) in your state? Does it address the local issues with regards to gender and equity? If yes, please share how this is possible.
- According to you, how has UNICEF been able to support the SBCC cell in adapting/ responding to the evolving needs (legislations/ schemes / emerging evidence) of the government especially in the covid times? Please share examples of the same

Effectiveness of SBCC cells

- IEC has been a more traditional method used in social communication. According to you, how effective has the cell been in transforming the dialogue towards SBCC in your state and district? Please share some examples of this shift with reference to initiatives before 2017 and after.
- Have the SBCC Cells successfully fulfilled their role as institutions for identifying and addressing challenges, supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing?
- How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system (please refer to the six pillars)? Has it improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates?

- How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives? Please share examples of strategies which reflected improved demand generation in the last five years. Is there any evidence or document to support the same?

Efficiency of the SBCC cells:

- Did the SBCC Cells efficiently deliver/implement its activities (resource planning including human resources, allocation, spending, monitoring, and mobilization of resources) and function within the planned/allocated monetary resources?
- Are there any special initiatives taken to improve efficiency of operations of SBCC Cells? Please mention the same
- In your opinion, what were the desired results from the Cell? Any specific constraints that have led to the disruption of the Cell to function as planned?

Sustainability of SBCC cells:

1. In which ways do you think state departments have been able to integrate SBCC in the planning, implementation, and monitoring of key programmes? Please share a few examples.
2. Does the government (at the state and district-level) have the capacity (with respect to human resources, governance mechanism and ownership within the system) to successfully carry out the activities of the SBCC Cells independently?
3. According to you, has the Cell established partnerships to successfully carry out the activities of the SBCC Cells independently?

Coherence of SBCC cells:

1. How do the activities/interventions of the SBCC Cells synergies and/ or complement the existing programme/initiatives of national/state government, non-governmental partners, and other sector players? Please share if there are joint programmes to reflect this. Are there MoUs to support collaboration and detailing the roles of the agencies. Please share the same.
2. What are some of the challenges in integrating SBCC cells in state programmes in enhancing the programme uptake of government schemes across sectors and in achieving improved outcomes at the programme level? Please share your recommendations to improve the integration of SBCC in state programmes.

Way Forward

1. In your opinion, what has worked the best and what can work better?
2. What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
3. What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with Officials supporting communication planning

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Officials supporting communication planning to understand the aspects of the SBCC program that has worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator must allow the participant to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

Programming, learning and evidence:

1. What is the structure of team supporting communication programming (both from UNICEF and the govt) for the SBCC Cell at the state and district level?
2. Are the allocated resources adequate to ensure the effective programming functions supported by the SBCC Cell? (*Probe for perspectives of adequacy of budgets, resources and time allocated to services and activities*)
3. Any specific challenges faced in funding to implement the communication strategies adopted between 2017-21?
4. What are the challenges faced by the SBCC cells around community knowledge dissemination while conducting COVID-19 response tasks and activities? (Mention examples of strategic focus areas undertaken by the SBCC Cell in the period both pre-COVID and present times)
5. How has information on COVID-19 been incorporated in the communication strategy of SBCC Cells? How have the capacities of the different stakeholders been built to deliver key messages to the community?
6. What are the implementation constraints faced in while supporting/conducting the activities at the community level?
7. Any specific challenges in training and capacity building of resource persons in the communicating and outreach of SBCC programs with respect to gender, inclusion, and social equity?
8. What efforts have been invested to augment the capacity of trainers and community staff in leveraging innovative methods and technology to address issues related to social norms and barriers?
9. What kind of technological support has been utilized to implement SBCC functions and community training? (Mention any instance of innovative use of technology adopted)
10. Have the SBCC Cells developed and implemented alternate work plans/strategies with special focus on vulnerable communities including women?
11. What have been (if any) specific community-led approaches that have gained focus as a recommended action?
12. How has the SBCC cells improved access of media enabled social messaging across communities and how have hard to reach areas been included for the same?

Communication Strategy and engagement with community target group

1. What has been the strategic approach in engaging SBCC Cell in areas where interventions were required for ending child marriage or related domestic violence, etc. What are the various communication strategies that may have been used and have helped in addressing issues?
1. To what extent has the training support and skill development initiatives from the SBCC cell enhanced quality of work and understanding on the importance of ending child marriage for relevant gender and social issues in the community?

- What are the present challenges faced by the SBCC Cell in supporting training and capacity-building and what are the priority action areas? (Probe for insights and understanding of the diverse range of themes of work pursued by the SBCC Cells and what are the future areas of action envisaged)

Way Forward

- In your opinion, what worked the best and what can work better?
- What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
- What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with Allied Department Official

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Allied Department Official to understand the aspects of the SBCC program that have worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participant to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

1. How effective has Cell and UNICEF been in transforming state government's approach from IEC to SBCC?
2. How, in your opinion, are the SBCC Cells aligned to the flagship programmes and priorities of the national and state governments? (Probe for: alignment of SBCC Cells with to global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, marginalized communities, SDGs)
3. How do the activities/interventions of the SBCC Cells interlink and synergize with the existing programme/initiatives by national/state government, non-governmental partners, and other sector players? How in your view, can SBCC cells complement government programmes/initiatives at state level?
4. In what ways has the SBCC Cell supported states in integrating SBCC programming in its range of development programmes? (Probe for: SBCC Cell support extended to integrate SBCC programming in emergency response and preparedness? e.g., health such as Covid-19, education, nutrition, tribal and other marginal communities, etc.)

5. How have the SBCC Cell's programmes adapted to changes due to possible new legislation/ new schemes/ evidence of community behaviour/ unrest/etc.? How has UNICEF been able to support the SBCC cell in adapting/ responding to the evolving needs of the government especially in Covid times?
6. How have the SBCC Cells **strengthened government initiatives** in bringing about more effective and evidence-based communication strategies? What are the ways in which the SBCC Cells supported integration of an equity and inclusion lens for evidence generation and SBCC programming?
7. If and how evidence-based programming has been effective in identifying and **addressing the challenges to demand generation** such as for education, RI, health, nutrition, and WASH services, and community development?
8. Did evidence-based programming improve targeting focused beneficiary groups including vulnerable/marginalized communities? Did evidence-based programming reduce time towards the desired change vis-a-vis that observed through IEC? (*Probe for: Impact of evidence-based programming on the use and diversified communication channels, and Impact on improvement of inclusion and diversity in planning communication campaigns particularly towards women and vulnerable/marginalized communities*)
9. In what ways have the **training equipped stakeholders** to plan, implement, and monitor behavioral change, and demand generation initiatives? How have the capacities of the different stakeholders at community interface been built to integrate, plan, design, implement, monitor SBCC initiatives and deliver key messages to the community?
10. Have the SBCC cells allocated funds to train its staff members in incorporating a gender-lens to SBCC planning? Evidence on whether training conducted has been translated into gender transformative social and behavior change campaigns?
11. How have the SBCC Cells **improved collaborations, partnerships, and convergence** with allied departments like yours at the state level? How effective was the SBCC Cells in intra-departmental and cross-departmental collaboration? (Within the institution wherein the Cell is established)
12. How effective was the SBCC Cell in developing partnerships and convergences for joint initiatives and resource augmentation with other departments, like that of yours? How, in your view, have the different departments at the state level integrated SBCC programming into its planning and implementation of initiatives? In what ways have SBCC programming been integrated within state planning for education, health, and other social development programmes?
13. How useful is the **communication strategy, material and content developed by the SBCC Cells towards demand generation** for development schemes and initiatives? How effective has the communication content been to challenge discriminatory gender and social norms which impede access to developmental services/schemes for women and for marginalized communities? How has information on COVID-19 been incorporated in the communication strategies of SBCC Cells? How effective was the SBCC Cell in supporting states across education, health (Covid and other challenges), nutrition, social equity, and awareness?
14. How has the institutionalization of SBCC-based programming through the **setting up of SBCC Cells particularly strengthened the health system**? What are the ways in which the SBCC Cell supported in strengthening service delivery of health and other allied developmental services?
15. What role did SBCC Cell play in knowledge management and knowledge sharing such that a larger number of stakeholders (at state) are sensitized to the need for SBCC programming?
16. How is the SBCC programming integrated within budgeting cycle of the state/district/ block?
17. How in your view, have the SBCC Cells been progressing in their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships? What modifications can be made to improve the process further?
18. How do the programme activities of SBCC Cells interlink and synergize with the existing activities/interventions by national/state government, non-governmental partners, and other sector players?
19. (e.g., health, nutrition, education, WASH)?

20. In your opinion, what worked the best and what can work better to integrate SBCC in state level programme implementation?
21. What is the vision and goals ahead aimed for the SBCC Cell for the coming years?
22. What would be your recommendations in replicating this model of SBCC Programming for other states and regions

KII with Collaborating govt officials

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Collaborating Govt Officials to understand the aspects of the SBCC program that has worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participant to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

- In what ways has the SBCC Cell supported states in integrating SBCC programming in its range of development programmes? (*Probe for: SBCC Cell support extended to integrate SBCC programming in emergency response and preparedness? e.g., health such as Covid-19, education, nutrition, tribal and other marginal communities, etc.*)
- How have the SBCC Cell's programmes adapted to changes due to possible new legislation/ new schemes/ evidence of community behavior/ unrest/etc.? How has UNICEF been able to support the SBCC cell in adapting/ responding to the evolving needs of the government especially in Covid times?
- How have the SBCC Cells **strengthened government initiatives** in bringing about more effective and evidence-based communication strategies? What are the ways in which the SBCC Cells supported integration of an equity and inclusion lens for evidence generation and SBCC programming?
- If and how evidence-based programming has been effective in identifying and **addressing the challenges to demand generation** such as for education, RI, health, nutrition, and WASH services, and community development?
- How in your view, have the SBCC Cells are progressing in their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships? What modifications can be made to improve the process further?
- How do the programme activities of SBCC Cells interlink and synergize with the existing activities/interventions by national/state government, non-governmental partners, and other sector players? (e.g., health, nutrition, education, WASH)?

- What have been (if any) specific community-led approaches that have gained focus as a recommended action?

Way Forward

- In your opinion, what worked the best and what can work better?
- What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
- What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with UNICEF ICO, SBC Specialist

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the UNICEF ICO, SBC Specialist to understand the aspects of the SBCC program that have worked or may have not worked for a future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator to allow participant to introduce themselves; particularly their role and department or programme they are associated with and for how long

Questions:

- How were the different models of SBCC Cell rationalized and how did it evolve? Please discuss the critical milestones in this journey before 2017 and after?
- What was the initial strategic approach followed for the development of SBCC Cells Programming? How has this strategy changed the course since? (*Probe for any specific change or shift btw.2017-21*) ?
- How relevant and appropriate are the SBCC Cells to the flagship programmes and priorities of the national and state governments?
- How do you think UNICEF was able to support the SBCC cell in adapting/ responding to the evolving needs (legislations/ schemes / emerging evidence) of the government especially in the covid times?
- IEC has been a more traditional method used in social communication. What was the strategy approach followed in transforming the dialogue towards SBCC? Please share some examples to illustrate the shift.
- What is the support extended by UNICEF to the SBCC Cells? How have the ICO supported SBCC Cells in fulfilling their role as institutions for visioning and conceptualizing the SBCC approach and further in identifying and addressing challenges, supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing?
- Has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system and how, indicate in the context of the six pillars? Has it improved collaborations,

partnerships, and convergence with allied government departments, Developmental agencies/ corporates at the national and state level? Please share examples

- Has the ICO supported in improvement of efficiency of operations of SBCC Cells at the state level and how? Was it through HR, training/ capacity building, technical assistance in planning to monitoring?
- In your opinion, what were the expected results desired from the Cell? What is the desired change envisaged from institutionalizing the Cells at the state and sub-state levels? What have been the enabling factors in the journey for the Cell so far?
- How has the ICO supported state departments in integrating SBCC in the planning, implementation, and monitoring of key programmes?

Way Forward

- In your opinion, what worked the best and what can work better?
- What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
- What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with UNICEF state official, SBC Specialist

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the UNICEF State SBC Specialist to understand the aspects of the SBCC program that have worked or may have not worked for a future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

Relevance of SBCC Cells-

- Can you please share the key milestones in the evolution of the SBCC Cells at the state level? What were the critical support or guidance that you received from UNICEF ICO or from the nodal Department at the state level to be able to operationalize it? What were the key activities undertaken in the last 5 years (2017-20)?
- Do you think SBCC Cells are aligned to the flagship programmes and priorities of the national and state governments? Some of the specific areas that we would request you to highlight include its contribution to
 - Global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, SDG).

- Your state's flagship programmes/ schemes the nature of support to integrate SBCC programming in health or development programmes.
- The support received in integrating SBCC in emergency response and preparedness. (Including COVID-19 pandemic)
- Can you please share the structure of the SBCC Cell and how it is embedded in the state's Departmental structure (In case of West Bengal it will be focused on understanding the district structure and effort made to integrate the structure at the state level). Please share the roles and responsibilities that the SBCC cell has that complement's the Department's activities.
- How has the SBCC programming aligned to global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, SDG)?
- How does the SBCC Cell respond to the needs of community members and community representatives (FLWs, trainees etc.) in your state? Does it address the local issues with regards to gender and equity? If yes, please share how this is possible?
- According to you, how has UNICEF been able to support the SBCC cell in adapting/ responding to the evolving needs (legislations/ schemes / emerging evidence) of the government especially in the covid times? Please share examples of the same

Effectiveness of SBCC cells

1. IEC has been a more traditional method used in social communication. How effective has the cell been in transforming the dialogue towards SBCC?
2. Have the SBCC Cells successfully fulfilled their role as institutions for identifying and addressing challenges, supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships?
3. How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system? Has it improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates?
4. How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives?

Efficiency of the SBCC cells:

1. Did the SBCC Cells efficiently deliver/implement its activities (resource planning including human resources, allocation, spending, monitoring, and mobilization of resources) and function within the planned/allocated monetary resources?

Are there any special initiatives taken to improve efficiency of operations of SBCC Cells?

Sustainability of SBCC cells:

1. Do you think the other state departments have been able to integrate SBCC in the planning, implementation, and monitoring of key programmes? Has this been integrated in their budgets and is it adequate? Please share a few examples.
2. Does the government (at the state and district-level) have the capacity (with respect to human resources, governance mechanism and ownership within the system) to successfully carry out the activities of the SBCC Cells independently?
3. Do you think the Cell has established partnerships to successfully carry out the activities of the SBCC Cells independently?

Coherence of SBCC cells:

1. How do the activities/interventions of the SBCC Cells interlink and synergize and/ or complement with the existing programme/initiatives by national/state government, non-governmental partners, and other sector players? Please share if there are joint programmes to reflect this.

What are some of the challenges in integrating SBCC cells in state programmes in enhancing the programme uptake of government schemes across sectors and in achieving improved outcomes at the programme level? Please share your recommendations to improve the integration of SBCC in state programmes.

Way Forward

1. In your opinion, what worked the best and what can work better?
2. What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
3. What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with UNICEF Consultant (loaned staff)

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the UNICEF Consultant to understand the aspects of the SBCC program that have worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

Relevance of SBCC Cells-

1. Can you please share the ways in which SBCC Cells have been relevant to the flagship programmes and priorities of the national and state governments? Some of the specific areas that we would request you to highlight include its contribution to:
 - Global and national areas of socio-developmental focus (e.g., aspirational districts, high burden districts, SDG).
 - Your state's flagship programmes/ schemes at the national and state level.
 - The nature of support to integrate SBCC programming in health or development programmes.
 - The support received in integrating SBCC in emergency response and preparedness. (Including COVID-19 pandemic)
2. How relevant, appropriate, and aligned is the design of the SBCC Cell to the State's established systems and institutions of service delivery? Particularly with respect to the existing state-specific institutional set-ups and structures?
3. In what ways does the SBCC Cell respond to the needs of community members and community representatives (FLWs, trainees etc.) Does it adequately address the local issues with regards to gender and equity? Please share how this has been possible.

4. How has UNICEF been able to support the SBCC cell in adapting/ responding to the evolving needs (legislations/ schemes / emerging evidence) of the government especially during Covid?

Effectiveness of SBCC cells

1. According to you, what are the ways in which the setting up of SBCC Cells has strengthened government systems in bringing about more effective and evidence-based communication initiatives? How have the programmes and initiatives (supported by SBCC Cells) incorporated evidence-based communication programming for behavioral change and demand generation?
2. Are stakeholders equipped with skills to plan, implement, monitor behavioral change, and demand generation initiatives through capacity building?
3. Have SBCC Cells successfully supported integration of a social and gender equity and inclusion approach for evidence generation and SBCC programming?
4. IEC has been a more traditional method used in social communication. How effective has the cell been in transforming the dialogue towards SBCC?
5. Have the SBCC Cells successfully fulfilled their role as institutions for identifying and addressing challenges, supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships?
6. How has the institutionalization of SBCC-based programming through the setting up of SBCC Cells strengthened the health system? Has it improved collaborations, partnerships, and convergence with allied departments at the national and state level/ Developmental agencies/ corporates?
7. How effective is the communication strategy, material and content developed by the SBCC Cells towards demand generation for development schemes and initiatives?

Efficiency of the SBCC cells:

1. Did the SBCC Cells efficiently deliver/implement its activities (resource planning including human resources, allocation, mobilization. spending, monitoring, and mobilization of resources) and function within the planned/allocated funding?
2. Are there any special initiatives taken to improve efficiency in the operations of SBCC Cells?
3. How are timelines planned for SBCC activities? Are they contextualized to the local scenario and consider dependencies?

Sustainability of SBCC cells:

1. Do you think the other state departments have been able to integrate SBCC in the planning, implementation, and monitoring of key programmes? Has this been integrated in their budgets and is it adequate? Please share a few examples.
2. Does the government (at the state and district-level) have the capacity (with respect to human resources, governance mechanism and ownership within the system) to successfully carry out the activities of the SBCC Cells independently?
3. Do you think the Cell has established partnerships to successfully carry out the activities of the SBCC Cells independently?
4. Has SBCC programming been integrated within the budgeting cycle at the state/district/ block level?
5. Does the Cell receive an allocated budget? How much percentage of the Cell functions are adequately covered under the budget received?

Way Forward

1. In your opinion, what worked the best and what can work better?
2. What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
3. What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with Leadership, Nodal govt dept

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Leadership, Nodal govt dept to understand the aspects of the SBCC program that has worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

1. What are your views on the need for the SBCC cell at the state level or the district level (as applicable) in generating demand and enhancing uptake of basic services such as health, nutrition, WASH, and other social development services?
2. What have been the key benefits of institutionalizing SBCC Cell at the state level? Indicate how
 - a. It changed the course from earlier communication initiatives managed by the department.
 - b. It supported achieving programme results. If yes, then how?
 - c. if there is any change in the state's health programmes/ schemes/ policies?

Probe for enhanced focus on gendered programming? Equity/ inclusion in programme implementation/ monitoring of demand generation in hard-to-reach areas/ etc.

3. How does the Cell engage in cross-departmental coordination?
 - a. Is it need based?
 - b. Is there a strategy to integrate SBCC in programmatic planning? If yes, how? If not, how can it be supported?
1. How is the budget for the cell planned?
 - c. What is the perception about the cell within inter and intra departments related to its functions and activities? Has the perception changed over time? If yes, then how? If not, then why?
4. In your opinion, what would the desired results from the Cell? Has the cell achieved the desired results from its institutionalization? Please elaborate
 - a. What factors have helped the journey for the Cell?
 - b. What factors were the key barriers for the Cell to function as planned?
5. Did Covid 19 influence any change in the Cell's strategy/ operations/ monitoring? Please elaborate
6. What are the next steps/ future strategies for the Cell? With HSS 3, what changes are envisaged in communication planning and demand generation?
 - a. How can UNICEF support the Cells in meeting/ actualizing the strategies?
7. What are the three ways in which UNICEF can improve its support to the state SBCC Cells?

- a. Probe to understand perception on UNICEF's role in programme design/ staffing/ orientation/ capacity building (To T)/ process standardization/ advocacy/ etc.

Way Forward

1. In your opinion, what worked the best and what can work better?
2. What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
3. What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with Development agencies with which the SBCC Cell has collaborated

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Development agencies with which the SBCC Cell has collaborated to understand the aspects of the SBCC program that has worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

1. How long is your association or acquaintance with the SBCC Cell at the state or district level? What has been the nature of collaboration/partnership undertaken with the SBCC Cell so far? Is there any MoU or meeting of the minutes that documents the complementary role of the two bodies? Please share in that case.
2. Has there been any joint programme undertaken with the SBCC Cell in the last five years? Please share the nature of programmes and themes. Also share supportive documentation if any.
3. What have been the key benefits of the collaboration with SBCC Cell? Please refer to the following areas:
 - o Has it supported programmatic results? If yes, then how?

- Has it supported any changes in the design of states' health programmes/ WASH, nutrition, education, protection schemes/ policies so far?
- Has it focused on enhanced attention to gendered programming, equity/ inclusion in programme implementation/ monitoring of demand generation in hard-to-reach areas/ etc.?
- 4. Have the SBCC Cells worked with development partners to develop and implemented alternate work plans/strategies with special focus on vulnerable communities including women?
- 5. How does the Cell engage in cross-sectoral coordination with development partners?
 - Is there a strategy to integrate SBCC in programmatic planning? If yes, how? If not, how can it be supported?
 - What are the various challenges (if any) in departmental coordination? What has been the way forward to mitigate the challenges for continued efficient operations of the SBCC cell?
- What is the view of development partners about the present functioning of the Cell? Has there been any change in their perspective over time? If yes, then how? If not, then why?
- In your opinion, what were the desired results from the association with the Cell? What factors have helped build the association so far? Any specific constraints or challenges faced in the partnership or collaboration undertaken?
- Has the partnership with the SBCC Cell improved collaborations, partnerships, and convergence with allied departments at the state level other Developmental agencies/ corporates?
- How successful was UNICEF in partnering with collaborating institutions and like-minded stakeholders for setting up the SBCC Cells at the state level? How effective was the SBCC Cell in developing partnerships and convergences for joint initiatives and resource augmentation? (With allied state departments and development agencies)
- Have the SBCC Cells successfully fulfilled their role as institutions for supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing, including convening partnerships? What modifications can be made to improve the process further?
- Is there a greater need for sensitivity in approaching training modules and while delivering training , in addition to responding to gender and social equity concerns in capacity-building programs, monitoring and reporting and creating and managing knowledge?

Way Forward

- In your opinion, what worked the best and what can work better for strengthening collaboration with other agencies?
- What are the forward-looking visions and goals aimed for the collaboration with SBCC Cell for the coming years?
- What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with Nodal Leadership, District SBCC Cell

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the Nodal Leadership, District SBCC Cell to understand the aspects of the SBCC program that have worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in

increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participant to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

1. What according to you is the core purpose and need of the existence of the SBCC Cell in the state or district?
2. How relevant and appropriate are the SBCC Cells to the flagship programmes and policy priorities of the national and state governments? *(Probe for: alignment of SBCC Cells national or state goals covering (e.g., aspirational districts, high burden districts, SDGs, HWCs)*
3. How do the activities/interventions of the SBCC Cells interlink and synergize with the existing programme/initiatives by national/state government, non-governmental partners, and other sector players? In what ways and how do the SBCC cells complement government programmes/initiatives in the state, please share examples from the last five years and if similar initiatives were undertaken before 2017?
4. In what ways according to you, the SBCC Cell supported the state in integration of SBCC programming in health or other development programmes? *(Probe for: SBCC Cell support extended to integrate SBCC programming in emergency response and preparedness e.g., Covid, other statewide initiatives)*
5. How have the SBCC Cell's programmes adapted to changes in external environment due to new legislations/ new schemes/ evidence on community behavior/ unrest/etc.? How was UNICEF able to support the adaptation process, especially with reference to the COVID situation?
6. How did UNICEF support existing state-specific communication strategies and strengthen SBCC programming/ implementation/ outreach? *(Probe for: How and in what ways did the Cell align the key state specific influencers for SBCC programming?)*

Do you think there is a mechanism for the SBCC Cell to understand the needs of community members and community representatives (FLWs, trainees etc.) and respond to it? Is there a process to integrate issues related to social equity gaps to meet the health and social services needs and priorities of vulnerable and marginalized communities in SBCC Cell's programming and operational plan?

(Probe for: How adequately does the SBCC cell's programming support in reaching out to women and men, girls and boys, migrants, tribal, vulnerable groups; In what ways were the inherent gender equity gaps to meet the health and social services needs and priorities integrated in the designing of the SBCC Cell's programming and operationalization plan?)

Programme Implementation Challenges Faced

1. What are the comparative advantages seen in implementation of district focused SBCC Cell model? Are there any inherent challenges faced owing to the limited state level support received in this model?
2. Do you think SBCC Cells strengthened government initiatives for more effective and evidence-based communication strategies for SBCC, please indicate how? Please indicate ways in which the SBCC Cells

support the integration and inclusivity in evidence generation? Has this resulted in a wider reach of government initiatives?

3. In which ways do you think the SBCC Cell was able to transform the state government's approach from IEC to SBCC? Please share experiences of the past (before SBCC Cells were established) and as it is now.
4. Did the SBCC Cell sufficiently deliver/implement its activities and function within the planned/allocated timelines? In what ways were the planned timelines for SBCC Cell's programme implementation realistic and contextualized to the local scenario? (Probe for: instances where there were delays and reasons for the same and also programmes those were managed in a timely manner)
5. Has the presence of SBCC cells contributed to better planning, implementation, and timely roll out of SBCC activities as per state's needs? Are there any challenges and is there any mechanism to mitigate them?
6. How have the different state departments integrated SBCC programming into its planning and implementation of initiatives? Has SBCC programming been integrated within state planning for health and other social development programmes and within budgeting cycle of the state/district/ block? Please elaborate.
7. Does the government (at the district-level) have the capacity to successfully carry out the activities of the SBCC Cells independently? What kind of human resources and nature of requisite skills required for SBCC Cell as per the proposed plans of implementation?
8. How do the activities/interventions of the SBCC Cells interlink and synergize with the existing programme/initiatives by national/state government, non-governmental partners, and other sector players? Have the SBCC Cells leveraged government and private partnerships to improve the demand and supply of health/RI/ WASH/ nutrition services for vulnerable communities? Please share examples of joint programmes undertaken in the last five years.

Finance and Budgeting

- Did the SBCC Cells deliver/implement its activities and function within the planned/allocated human resources? In what ways could the allocated human resources and person-hours adequately implement the functioning and programmes of the Cell? How much of the expertise and commitment of the allocated UNICEF's HR utilized to deliver the SBCC activities/interventions?
- How were additional human resources mobilized to prevent inefficiencies due to implementation challenges, please share examples? What other processes have been incorporated to increase the efficiency of SBCC Cells functioning on human resource mobilization/ utilization/ planning? (*Probe for: Strategies and plans developed to meet the key challenge of unavailability of trained human resources for SBCC Cells*)
- Did the SBCC Cells implement its activities and function within the planned/allocated funds/budget? How and for what purpose were the allocated funds to the Cell utilized as per the plan and timeline? (*Probe for: Estimated percentage of the funds allocated and subsequently spent towards different activities like training on gender equality and mainstreaming in SBCC planning/ programming/ implementation/ monitoring etc.*)
- What are some good practices that have benefited the programme and increased its outreach and programme results achieved at a faster pace? What other processes have been incorporated to increase the efficiency of SBCC Cells functioning on monetary resource mobilization/ utilization/ planning?
- What are the challenges faced in ensuring availability of adequate resources in terms of HR, funds etc.?

Sustainability

1. What are the governance mechanisms in place to oversee and guide the functions of SBCC Cell (*Probe for: the ownership and sense of purpose [for the Cell] built among the leaders of the institute/ collaborating organization that houses the SBCC Cell*)?
2. Are the available resources, partnerships etc. adequate in sustaining the activities of the SBCC cell and its implementation machinery at the state and district level?

3. How does SBCC Cell mobilize additional monetary resources for its activities through convergence with inter and intra departments and partnerships with developmental agencies and corporate?

Way Forward

1. In your opinion, what worked the best and what can work better? Any specific recommendations for improving the effectiveness and efficiency of the cell in overall management of SBCC roll out?
2. What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
3. What would be your recommendations in replicating this model of SBCC Programming for other states and regions?

KII with SBCC District official

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the SBCC District official to understand the aspects of the SBCC program that have worked or may have not worked for future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your engagement with local govt stakeholders and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that **no personal name will be reflected in the report.**

Before posing the specific questions, the facilitator to allow participant Before posing the specific questions, the facilitator to allow participants to introduce themselves including their role, the time spent in current role, and overall understanding of the UNICEF SBCC Cell evolution and implementation, strategies, and your views w.r.t. the same. It is important to give the participant the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Questions:

Relevance of SBCC Cells

1. How according to you are the SBCC Cells aligned to flagship programmes and priorities being implemented in your district? How is the SBCC programming aligned to regional areas of socio-developmental focus (e.g., aspirational districts, high burden districts)?
2. How does the SBCC Cell support state's integrated SBCC programming in health or other development programmes?
3. Has there been any training and capacity building efforts undertaken to support capacity needs and gaps to carry out activities under its flagship programmes at the district levels and below?
4. Do you think SBCC has been able to implement and/or monitor equity and inclusion in remote areas and how, please share examples?
5. According to you, are the allocated resources adequate to ensure the efficient functioning of the SBCC cells at the district level and below?

6. Has the SBCC Cell been able to incorporate a gender-lens in its planning? How has the SBCC cell's programming supported in reaching out equitably to women and men, girls and boys, migrants, tribal, vulnerable groups? Did the initiative focus on remote and hard-to-reach areas?
7. Please share the trend of fund utilization over the last five years on SBCC training for staff members? What were the challenges for SBCC cell to coordinate with other departments? How was it mitigated?
8. How were the inherent social equity gaps in accessing health and social service integrated in the designing of the SBCC Cell's programming and operationalization plan? Please share a plan and examples. How were the needs and priorities wrt. equity and inclusion integrated in the designing of the SBCC Cell's programming and operationalization plan?

Effectiveness

- How and in which ways do you think SBCC Cells strengthened government systems in bringing about more effective and evidence-based communication initiatives?
- Do you think SBCC cells have been able to fulfil their role in supporting evidence generation, strategic planning, operationalization, monitoring, and analysis? Please share instances for each area.
- Were the training programmes implemented for stakeholders for engaging with community important to achieve results?
- Has the implementation of the SBCC strategy influenced the frontline workers' knowledge, attitudes, and practices towards RMNCH+A, routine immunization, health, nutrition, and WASH services?
- How and in which ways did the SBCC Cell made the shift from IEC to SBCC?

Efficiency

1. Did the SBCC Cells deliver/implement its intended activities and function within the planned/allocated human resources?
2. Has the SBCC Cell utilized funds to train its staff in incorporating a gender-lens into SBCC planning?
3. Have the funds which have been allocated to SBCC utilized as per plan and timeline to focus on vulnerable communities?
4. Do you think that the activities as per the plan for the district level have been developed in cognizance of the unavailability of trained human resources for SBCC Cells?
5. What are the key activities that the SBC Staff most invest in? Are they always aligned to SBCC activities? Do you think their effort is effectively invested? Share rationale for your response .
6. Has there been any fund allocation for the gender related activities?

Sustainability of SBCC cells:

1. Do you think that the capacity building targets on SBCC at the district levels have been achieved in the last five years?
2. Do you think at the district level SBCC programming is integrated into its planning and implementation of initiatives? Did the SBCC Cell mobilize additional monetary resources for its programmes through convergence with inter and intra departments at the district level, or is the allocation coordinated only at the state level?
3. Does the government (at the district-level) influence activities of the SBCC Cells? Share examples
4. Is there a district level governance mechanism put into place to oversee and guide the functions of the SBCC Cell?

Coherence of SBCC cells:

5. Which activities/interventions of the SBCC Cells interlink and synergize with the existing programme/ and other interventions (across health, nutrition, education, WASH sections, child protection) in your district by government departments, NGOs, and other sector players in your district?
6. Is there any evidence of SBCC cells having a comparative advantage vis-à-vis the various programmes in your district?

Way Forward

- In your opinion, what worked the best and what can work better to make the work of SBCC Cell more effective at the district level?
- What are the forward-looking vision and goals aimed for the SBCC Cell for the coming years?
- What would be your recommendations in replicating this model of SBCC Programming in uncovered districts or regions?

B. Study tool - FGD

FGD with District SBCC Coordinator

Introduction (Can be modified as per interviewer):

PricewaterhouseCoopers Private Limited, has been selected as the evaluation team for the 'Evaluation of UNICEF's SBCC Cells Programme from 2017-2021'. The evaluation process is divided into three inter-related phases: Inception; data collection, data analysis and reporting; and report writing and dissemination. As a part of the data collection exercise, it is important to have the views of the District SBCC Coordinators to understand the aspects of the SBCC strategy that has worked or may have not worked for a future course of action. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your communication with local stakeholders particularly AWW/ ASHA and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that this is meant for collective discussion and not to capture individual views. Thus, **no personal names will be reflected in the report**. It is also important to indicate that the FGD would take about one hour of their time. As a process, the facilitator will raise the issue of discussion and the participants would share their knowledge, views, perceptions, and thoughts. It is important to give the participants the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion.**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Pointers for discussion:

1. What is your view about the SBCC Cell and how has it evolved over the years? Are there any specific examples that describe or explain the evolution of BCC activities at the district level?
2. What according to you are the most important roles of SBCC Cell at the district level or SBCC activities undertaken at the district level? (As applicable depending on the categorization of the state). Please indicate if the District Cells are focused on the activities indicated in supporting evidence generation, strategic planning, operationalization, monitoring, analysis, training, documentation, knowledge sharing and building partnerships).
3. Please share the specific roles and responsibilities that you have in executing the SBCC activities at the district level. The reporting framework, any changes in the last five years as compared to the past.
4. The complementary role with respect to the Departmental positions at the district level. Are there any overlaps or what is the mechanism of coordinating with stakeholders at various levels e.g., state SBCC Cell, District functionaries and with other functionaries below? Also indicate if there is a structure in place for the same.
5. What are the various campaigns and SBCC activities undertaken at the state level (such as demand generation for RI, health, nutrition, and WASH services)? What were the themes taken up in the last five years (2017-22), has there been any shift in those in the recent times? Are they also aligned to the needs of the community at the local level?

6. Please share the mechanism that is undertaken to decide on the theme and the content of the BCC strategy or activities, if evidence or research or MIS data is referred to decide. Please indicate if the planners consider evidence for incremental increase in proportion of women and vulnerable groups targeted/reached through mix-media communication
7. Has there been any collaboration, partnerships and convergence with other departments, NGOs, and other agencies to implement the SBCC activities at the district or sub district level? Have such collaborations helped in thematic exchange, resource sharing, coverage, and any other benefits?
8. What are the challenges you face in actualizing communication strategies in the field through various departmental workforce? How do you mitigate the same?
9. What supervisory or technical support do you require to design and deliver SBCC activities? What additional support would help you?
10. Request insights to make the SBCC campaigns and strategies more localized and equitable (Suggested areas: domestic violence; child marriage)

FGD with UNICEF SBCC Cell Trainers/Master Trainers Introduction (Can be modified as per interviewer):

With this group facilitator/s may refer to the Department with which the SBCC Cell is associated with (in West Bengal we may refer to the District Administration too) and the intent of the team to understand the elements of the programme that has worked or not worked in the context of the communication. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your communication with local stakeholders particularly AWW/ ASHA and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that this is meant for collective discussion and not to capture individual views. Thus, **no personal names will be reflected in the report**. It is also important to indicate that the FGD would take about one hour of their time. As a process, the facilitator will raise the issue of discussion and the participants would share their knowledge, views, perceptions, and thoughts. It is important to give the participants the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion.**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Pointers for discussion:

1. What kind and number of training courses were provided by the SBCC cells to the trainers in the last five years? Please elucidate with examples covering the topics and themes covered in the training (e.g., understanding of SBCC, facilitation and training skills, community mobilization, developing a training module and material and facilitating community-based communication activities. The themes for example may cover the issues related to RI, usage of clean water and sanitation and so on, depending on the state categorization).
2. Could you please give an overview of the training process? Please emphasize the themes, pedagogy, method of training and overall experience. Please indicate if there was any support from the state, other district/s, technical expert/s, and other departmental functionaries in the district?
3. How critical do you think that communication is to ensure behavior change to enhance uptake of flagship programmes and basic services around health, education, nutrition, child protection?

4. If there are any change in the ways/methods of generating awareness or facilitating change in behavior of the community or beneficiaries visible in the last five years, please explain
5. What is the role of the trainers (all of you) in ensuring there is improved demand and uptake of services?
6. If any training or orientation on understanding of gender equality, equity and inclusion in BCC have been organized for trainers?
7. Was there any structured exercise for understanding the training and capacity building needs of trainers? The average number of training courses undertaken by this group annually in the last five years on SBCC for the trainees at various levels.
8. Has the training support and skill development initiatives from the SBCC cell enhanced the quality of work and understanding on the importance of social and behavior change communication of those interacted with at the district level and below. If yes, please explain by sharing examples of such experiences in which it has helped you to execute training for the frontline workers and local functionaries for improved demand for basic services?
9. How are the TARANG modules used in the training process of trainees? Can you please share an overview of the broad components of the training?
10. Is there any process in place to gather inputs and field insights taken of master trainers in developing BCC material, training modules, programme or strategy for the state and district? If yes, how? If not, share your views?
11. What on-field support is made available to aid your training delivery? What additional support would help you?
12. Does the training delivered by Master Trainers have a gender sensitive and social equity lens? How has this lens been incorporated in the delivery and content, especially in the last five years?
13. Did Covid 19 influence any change in the training process? Was the training switched to an online mode?
14. In your opinion how can the training content and delivery be strengthened? In your opinion how can the SBCC Cell be better supported in the strengthening of training content and delivery?

FGD Guide for Trainees of SBCC Cells

Introduction (Can be modified as per interviewer):

With this group facilitator/s may refer to the Department with which the SBCC Cell is associated with (in West Bengal we may refer to the District Administration too) and that they are here to assess certain aspects of the programme delivery and its effectiveness. The facilitator can introduce themselves, "I, _____, am from the evaluation team would like to discuss with you about your communication with local stakeholders particularly AWW/ASHA and other local workers/leaders in increasing community's demand and access to basic services such as health, WASH, nutrition, education, protection or otherwise. I am accompanied by _____, my co-facilitator".

Thank the participants for joining the discussion and share that this is meant for collective discussion and not to capture individual views. Thus, **no personal names will be reflected in the report**. It is also important to indicate that the FGD would take about one hour of their time. As a process, the facilitator will raise the issue of discussion and the participants would share their knowledge, views, perceptions, and thoughts. It is important to give the participants the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the 'do no harm principle' in conducting the discussion.**

Before posing the specific questions, the facilitator allows participants to introduce themselves; particularly their role and department or programme they are associated with and for how long.

Pointers for discussion:

1. Please share your thoughts on the necessity of communication and community mobilization with the community in providing services. (An ice breaking question)

2. Their exposure to nature of training that they have received from district/ circle/ block level on SBCC (facilitator should explain what SBCC all about). Allow the participants to recall the various training received in the last five years. Share the number and themes under each year, as per recall
3. What are the themes and topics that are covered in the discussions conducted with trainers (Probe on topics covered in the dissemination meetings, e.g., home visits, community education, interpersonal communication, conducting meetings, other community mobilization techniques and on themes related ANC, immunization, COVID related issues, nutrition, childcare, child marriage and so on)
4. Discuss if you feel the training that you received was relevant to the issues and challenges in supporting community access services available? Please probe specifically on issues related to discrimination and practices due to gender (men/ women/boys/ girls/others), income (poor and marginalize), social groups (SC, ST and others) and inclusion of people with disability on access to services.
5. How often do you meet the target group and disseminate the message?
 - a. Probe on how the target group is identified and prioritized. (People and groups with local access to services, marginalized groups, women, mother in laws, local leaders, and other age groups and so on).
 - b. Probe on key messages they deliver from the point of view of if they understand the criticality of the key issues being addressed through the messages (Probe areas on existing knowledge and understanding, and the approach of trainees in delivering information, and their own biases and practices, their attitude to behavioral change such as biases around gender inequality and domestic violence)
6. Indicate if there has been any change in communicating or interacting with the community before five years and now (specifically for the ones in the system for more than five years). Share examples of the same

Are there community groups who you communicate with on a regular basis and the ones that you find challenging to share messages?

7. Are you provided with any aid and materials to support your communication with the community and the beneficiaries? (Probe if they are also trained to prepare the communication materials such as posters, flip books themselves)
8. How aligned do you think the training programmes and job aids/ materials provided are to your capabilities, skill sets, knowledge, and ability to deliver services? In this context enquire if any training that needs assessment for communication has been done.
9. How receptive are your target groups to the messages shared through meetings or other mechanisms? (*Probe on target groups – women, men, couples, marginalized/vulnerable groups*) Who are the target priority groups for such meetings/gatherings (if there are any specifics to be mentioned)?
10. Share your insights on if and how community mobilization and awareness programmes have enabled identifying and addressing the challenges to increase demand for RI, health services, nutrition programs and WASH services. Has this impacted the availability and accessibility of such services in your area?
11. Could you share the challenges that you face in your day-to-day work and training? (For instance: community's unwillingness to attend meetings, group sessions, training, etc., resistance from communities' in talking with you on certain topic etc.) Please probe if challenges are around infrastructure or lack of relevant materials, human resources, deep rooted culture/ tradition, or biases and so on
12. Is there any supervision or guidance while executing the communication activities? In case there is, how and what is the nature of this supervision or quality assurance
13. In your opinion, who are the key influencers that support and increase possibilities of community mobilization?
14. Please share if you have received any feedback from the target group on the effectiveness of key community mobilization activities done by you. Have target beneficiaries acknowledged any positive change in their behavior which resulted from your inputs? (Probe if this has increased participation in meetings, adoption of positive behavior around health practices, social biases etc.).
15. Share your insights on the support mechanisms received (if any) from the government in improving the effectiveness and efficiency of Covid response? (Probe for support received to improve access to basic support services including vaccine roll-out?)

16. Do you think that there are any specific facilities that you would need as trainees to make the training received more relevant to specific community-based contexts? For instance: what are the relevant tools and IEC materials received and used to deliver training and activities in the community?
17. Please share your thoughts and insights on the following:
 - a. Identification of priority messages, target groups and delivery of the message through the various training content received
 - b.
 - c. Inputs received to support an environment where communities and key influencers are engaged in developing and designing messages on health and other aspects through various platforms.?

FGD with Community members

Introduction (Can be modified as per facilitator):

With this group, facilitator/s may refer to the Department with which the SBCC Cell is associated with (in case of West Bengal we may refer to the District Administration too) and that they are here to understand what has worked and not worked with respect to communication related intervention/s. The facilitator can introduce themselves, “I, _____, am from the evaluation team and would like to discuss with you about your communication with local stakeholders particularly AWW/ ASHA and other local workers/leaders in increasing community’s demand and access to basic services such as health, WASH, nutrition, education, or others. I am accompanied by _____, my co-facilitator”.

Thank the participants for joining the discussion and share that this is meant for collective discussion and not to capture individual views. Thus, **no personal names will be reflected in the report**. It is also important to indicate that the FGD would take about one hour of their time. As a process, the facilitator will raise the issue of discussion and the participants would share their knowledge, views, perceptions, and thoughts. It is important to give the participants the confidence that **they can express their views without fear, and they would not be quoted or forced to participate at any point of time and their participation is entirely voluntary. Request permission to record the interaction for documentation purposes. All information and responses shared during the discussion would remain confidential and anonymous. All the evaluators would follow the ‘do no harm principle’ in conducting the discussion.**

Pointers for discussion:

1. Have any kind of campaigns/ meetings/sessions on health issues, vaccination, WASH, nutrition, education etc. happened in your village? What were these? Can I give you some details? Who conducted these meetings/campaigns? (Probe for interactions with refer to examples such as AWW/ ASHA, local leaders or SHG members)
2. Are there any specific days or occasion when you have interactions with the FLWs (ASHA, AWWs, etc.) (e.g., during nutrition days, immunization week, mothers meeting at Anganwadi, home visits, workshops, or others) (Probe on any other ways that they have access to the messages related to the above-mentioned sectors such as street plays, group meetings, gram sabhas, local TV channel, mobile messages, posters SHG meetings and so on).
3. Please share the specific themes and messages that they cover in these sessions
4. Please indicate the most covered themes (Probe on social issues, behavioral change around health and hygiene, prejudices against gender, specific groups such as economically disadvantaged, marginalized groups, etc.) Ensure that facilitator allows participants to add more to the list and cover as many topics and themes as they can recall.
5. Which groups/ target groups in your community do they mostly target (mothers, women, men, local community). If there is a trend in the last five years to focus on a certain group or community.
6. Your views on the alignment of the topics discussed or communicated with the lives and experiences of the community (please refer to local priorities, the COVID situation as an example). Please probe into the usefulness of the top is in the community’s context.

7. If there was any scope to share your/ community's thoughts on how and what messages and themes to cover as a part of discussions or communication activities. (wrt. The communication with the representatives of the government, workers, frontline functionaries from various departments)
8. Do you think these messages have brought about any change in the community's use/ uptake for the government services (please refer to services linked to the state categorization, immunization, other pre- and post-natal services, nutrition services, WASH facilities, child protection and cover themes such as stereotypes defined by gender, economic status or social groups, existing biases, and barriers on the same)
9. Please share some examples of some campaigns, messages or other programmes that were useful or not so useful for your needs.
10. Based on the communication and messages received do you think that more girls from the village are continuing school, girl child is equally taken care as boy child, girls are not seen as burden etc.
11. Any area as a community you think should be taken up by health workers, community leaders, SHG groups etc. to enable you to access public goods and services?

Field-Level Observation Checklist

On Resources:

1. Availability of training material
2. Availability of training modules
3. Availability of training modules in local language(s)
4. Availability of training records and attendance sheets
5. Availability of training protocols pertaining to interpersonal interactions

On Physical environment:

6. Physical environment of training location(s) is well lit
7. Physical environment of training location(s) is ventilated
8. Availability of washroom facilities in training location(s)
9. Availability of potable water in training location(s)
10. Availability of seating area in the training location(s)

On Subjective Experience: (Subjective experiences of SBCC related training conducted by Master Trainers with Trainees of SBCC Cell is noted for observation)

11. Do the trainees seem interested and engaged in the training?
12. Do the trainees and trainers seem comfortable with each other?
13. Do the trainees and trainers seem comfortable in the physical environment of the training location?
14. Is the training interactive? Are there participatory training methodologies adopted for conducting training with trainees?
15. Was the ratio of trainer to trainees appropriate for increased knowledge acquisition and training delivery?

On Trainers' training:

16. Have the trainers received training on how to conduct training?
17. Have the trainers received training on the training modules to be used for the training?
18. Have the trainers been sensitized in social and gender inequities?
19. Have mixed-media tools been incorporated in training conducted?

On Reporting and monitoring:

20. Are the training details such as the number of participants documented?
21. On Training effectiveness, how is the training effectiveness measured and monitored?
22. How are the trainees' learning levels and knowledge acquisition monitored?

A.14. Terms of Reference:



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